





PRACTICE MATERIALS

LISTENING

READING

OET 2.0

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LISTENING

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Where do I write my answers for the Listening sub-test?

For Part A, you must write your answers in the space provided in the question booklet.
For Part B and Part C, you must shade the lozenge next to the appropriate answer. Answers written elsewhere in your booklet will not be marked.
It is a good idea to use the sample tests to familiarise yourself with the different task formats you will find in the test

Do I have time to check my answers for the Listening sub-test?

Yes, you will have two minutes at the end of the sub-test to check your answers for all three parts of the sub-test. You will listen to each part of the test once only, so you must write your answers in the question booklet as you listen.

Can I use abbreviations in the Listening sub-test?

You can use abbreviations that are commonly accepted in your profession and which are clear to other professionals, for example "BP" for blood pressure. However, you should avoid abbreviations that are specific to a particular workplace or specialism, because these might not be commonly understood. OET assessors are trained to accept a reasonable range of abbreviations, but OET does not refer to any specific dictionaries or lists.

Do I lose marks if I give incorrect or extra information in the Listening sub-test?

There is no penalty for including information that is not in the marking guide. However, you will lose marks if you contradict yourself or make your meaning unclear.

Do I lose marks for spelling mistakes in the Listening sub-test?

In the Listening sub-test, you will not be penalised for misspelling, provided the meaning is clear to other healthcare professionals. Any reasonable attempt at spelling the correct answer has a good chance of being accepted.
Names for conditions and medications are often difficult to spell, and we try to ensure that candidates are not disadvantaged by this. Where possible, reference is made in the audio recording to both the generic and brand names for medications, and to both medical and lay terms discussed during the consultations. The marking guide gives assessors extensive guidance on the range of misspellings which are to be accepted.
Please note that the Listening sub-test is different from the Reading and Writing sub-tests in the way misspellings are treated.

How many questions do I need to get correct for the Listening sub-test?

New listening tests are written for each test session incorporating new material and the grade boundaries are adjusted slightly for each test to allow for minor differences in the difficulty of items included in that particular version. The number of marks needed to secure grade B will therefore vary. However, test-takers awarded grade B (a scale score of 350) will typically have a score of at least 30 marks.

About the Listening sub-test

The Listening sub-test consists of three parts, and a total of 42 question items. The topics are of generic healthcare interest and accessible to candidates across all professions. The total length of the Listening audio is about 40 minutes, including recorded speech and pauses to allow you time to write your answers. You will hear each recording once and are expected to write your answers while listening.

The Listening sub-test structure

Part A – consultation extracts (about 5 minutes each)

Part A assesses your ability to identify specific information during a consultation. You will listen to two recorded health professional-patient consultations and you will complete the health professional's notes using the information you hear. **Note: the health professionals may be any one of the 12 professions who can take OET.**

Part B – short workplace extracts (about 1 minute each)

Part B assesses your ability to identify the detail, gist, opinion or purpose of short extracts from the healthcare workplace. You will listen to six recorded extracts (e.g. team briefings, handovers, or health professional-patient dialogues) and you will answer one multiple-choice question for each extract.

Part C – presentation extracts (about 5 minutes each)

Part C assesses your ability to follow a recorded presentation or interview on a range of accessible healthcare topics. You will listen to two different extracts and you will answer six multiple-choice questions for each extract.

How is listening ability assessed in OET?

The Listening sub-test is designed to assess a range of listening skills, such as identifying specific information, detail, gist, opinion or the speaker's purpose. These skills are assessed through note-completion tasks and multiple-choice questions.

Assessors who mark the Listening sub-test are qualified and highly trained. Candidate responses are assessed against an established marking guide. During the marking session, problematic or unforeseen answers are referred to a sub-group of senior assessors for guidance and all papers are double-marked to ensure fairness and consistency.

How is the listening test scored?

Your answers for Part A are double-marked by trained OET assessors.

These answers are randomly assigned to assessors to avoid any conflict of interest.

Your answers for Part B and Part C are computer scanned and automatically scored.

For Part A, Listening assessors use a detailed marking guide which sets out which answers receive marks. Assessors use this guide to decide whether you have provided enough correct information to be given the mark. Assessors are monitored for accuracy and consistency.

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

In this part of the test, you will hear Dr. Steve interviewing Janet Cooper, grandmother of Riley Cooper with a recent problem. For questions 1-12 complete the notes with word or a short phrase.

You have 30 seconds to look at the notes.

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Patient	:	Riley Cooper
Background	:	He has got red (1)_____ and they are always itchy and got some on (2)_____ and (3)_____ as well
Family History	:	Her (4)_____, is suffering from a condition known as Eczema. Eczema is also known as (5)_____ that occurs in early childhood.
Triggering Factors	:	Triggering Factors such as dairy products, (6) _____ and eggs. He is very fond of (7)_____ eggs. Certain products such as(8)_____ which can be quite abrasive. He has a pet

(9)_____ that he loves. Dust (10)_____ are another contributing factor that can be found in stuffed

(11)_____ and his (12)_____

Extract 2: Questions 13-24

In this part of the test, you will hear a Doctor interviewing Alfred Jenkins, who recently suffered a heart attack and has just been transferred from a intensive care ward.
.For questions 13-24 complete the notes with word or a short phrase.

Patient	:	Alfred Jenkins
Background	:	Severe pain when lifted 13)_____ and his friend drove him to 14)_____ hospital. He was admitted in 15)_____ four days ago.
Medications	:	During hospitalization, he was prescribed with 16)_____
Work Condition	:	Most of the time he has to 17) _____or 18)_____ Around in van.
Personal Life	:	He was 19)_____ 18 months ago. At present he has girl Friend were both interested in 20)_____ or 21) _____ . He was concerned about to maintain active play with (22)_____.
Family History	:	Father died due to (23)_____ and his mother has history of (24)_____

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Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now you will hear a talk about ambulatory nursing care:

25.Ambulatory nursing care provides care :

A. only for one year

B. for a life span

C. for specific communities

Now you will hear about cleaning patient care equipment:

26.cleaning is practiced by using:

A. only with water

B. Wter and enzymatic products

C. detergents and enzymatic products

Now you will hear about nasal foreign body removal

27.Nasal foreign body removal method which does not need restrains:

A. wax hook method

B. Bag-valve-mask apparatus method

C. Parent's kiss method

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Now you will hear about vaccination:

28.Parents should inform the clinic before bringing the unvaccinated child in order to :

A. take possible precautions to avoid contact with other patients

B. Take possible arrangements to provide vaccines

C. Take possible precautions to prevent pertussis and measles

Now you will hear a nurse talking about the role of nurse in wound care:

29. A nurse should observe common wound sites of bedridden patients such as:

A. surgical incision site

B. Back and hip bones

C. ulcered areas

Now you will hear about the usage of peak flow meter:

30. Choose the correct way of using a peak flow meter

A. sit comfortably

B. place the tongue on the hole of the meter

C. Blow out hardly and fastly

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Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

31). Most estimates suggest that among primary school age children

A) 2% of children have ADHD

B) 80% Of children have ADHD

C) 5-10% of children have ADHD

32). Regarding gender and ADHD

A) ADHD is much more common in boys

B) ADHD is equally common in boys and girls

C) ADHD is slightly more common in boys

33). Which of the following is not correct?

A) Smoking during pregnancy can increase the risk of ADHD

B) 50% of male parents who had ADHD may have a child with ADHD

C) Exposure to lead in early childhood may increase the risk in ADHD

34). Which of the following is more likely to trigger out of control behavior?

A) Too much food containing sugar and food colorings

B) Too much excitement

C) A combination of both A & B

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35). Which of the following statements is most correct?

A) About 1/3 of children with ADHD suffer from sleep apnoea

B) Most children with ADHD suffer from sleep apnoea

c) About1/3 children who suffer from sleep apnoea also have ADHD

36). Which of the following statements is most correct?

A) After 40 years of research the reason why some children suffer from ADHD is becoming clear

B) Recent theories suggest problems with left side of the brain

C) A child with ADHD may interpret feelings and information differently to other Children

Extract 2: Questions 37-42
In this part of the test you will hear a lecture based on the subject of Cardiac Investigation. For questions 37 to 42, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes

37). Symptoms of patient with heart problem include

A) Chest pain, Palpitation

B) Edema of hands

C) Tachypnea, Bradycardia

38). Examination often not helpful,but it is important to check

A) Relationship to exertion

B) Risk factors of cardiac

C) Cardiac murmers and blood pressure

39). Functional tests assess the effect of ------ on the heart

A) Exertion

B) Trauma

C) Drugs

40). During an exercise ECG, Ishemia will be signaled by a drop in

A) Blood pressure

B) Heart rate

C) ST.waves

41). One drawback and exercise ECGs is that they are

A) Prohibitively expensive

B) Lacking in precision

C) Less effective with males

42). According to the speaker when an exercise ECG test is negative

A) Angina should still not be ruled out

B) Chest pain is not related to angina

C) The patient is more at risk of angina.

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ANSWERS

Part A

1) Scaly patches

2) Knees

3) Tummy

4) Grandson

5) Atopic dermatitis

6) Wheat

7) Screambled

8) Wool

9) Dog

10) Mites

11) Toys

12) Bedroom

13) Heavy battery

14) Boxhill

15) Intensive care

16) Anginine

17) Sit

18) Drive

19) Divorced

20) Gardening

21) Fishing

22) Son

23) Heart attack

24) Arthritics

PART B

25) B

26) B

27) C

28) A

29) B

30) C

PART C

31) C

32) A

33) B

34) B

35) A

36) C

37) A

38) C

39) A

40) C 41) B 42) A

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OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

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PAUSE: 5 SECONDS

Extract 1: Questions 1-12

For questions 1-12 complete the notes with word or a short phrase.

You have 30 seconds to look at the notes.

Patient name	:	Michael fisher
Reason for presenting	:	Not (1) _____ well. Has fever and also (2) _____. Not hungry and unable to (3) _____ meals. (4) _____ noticed yellowish skin color.
Personal details	:	Age is (5)_____. Married, and has (6)_____ children. He is a carpenter and works on (7) _____ sites. Worked in east (8) _____. Drinks in (9)_____

Family history	:	Father is (10) _____ and has good health. 3 sisters all well.
Diagnosis	:	(11)_____ hepatitis A.
Possible causes	:	Picked up on (12)_____.

Extract 2: Questions 13-24

In this part of the test, you will hear an anesthetist talking to a patient called Mary Wilcox prior to an operation. For questions 13-24 complete the notes with word or a short phrase.

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Patient name	:	Boy aged 10
Case history and current symptoms	:	Asthmatic since age (13)_____. Had severe (14) _____ attacks.
Symptoms according to mother	:	Had a (15) _____night. Hardly had. any (16)_____.
Past history	:	Use (17) _____ inhaler, But it is (18) _____. Asthma since 4 years, able to (19) _____. Had (20) _____ in the past, which made

Condition (21)_____. Now uses

(22) _____ inhaler.

Further details of condition : Uses (23) _____ or peak flow meter.

Today's peak flow meter was

(24)_____.

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

NOW YOU WILL HEAR A NURSE EXPLAINING ABOUT SPILLAGE MANAGEMENT

25. Where should keep the contaminate wools?

A. Waste container

B. Sharp container

C. Mercury containers

NOW YOU WILL HEAR ABOUT VENOUS CATHETER CARE

26. Storage of cvc supplies should be at

A. Clean and cool place

B, Clean and dry place

C. Wet and clean area

NOW YOU WILL HEAR ABOUT DRAINAGE PROCEDURE OF LEFT LEG BAG

27. How often the leg bag should be emptied

A. whenever necessary

B. Every 3 Hours

C. Every 3 -4 Hours

NOW YOU WILL HEAR ABOUT IABP THERAPY

28. IABP safety is contradicted for which patients?

A. Patients with Leaky aortic valve

B. Patients with Leaky valve

C. Patients with Valve displacement

NOW YOU WILL HEAR ABOUT POST OPERATIVE COMPLICATIONS

29.The most common causes of post-operative vomiting are

A. Drug induced and gut atony

B. Drug overload

C. Anesthesia

NOW YOU WILL HEAR ABOUT ABG SAMPLE COLLECTION

30. How you hold ABG syringe?

A. Like a dart

B. At right angle

C. Like a cart

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Part C

In this part of the test, you hear a talk about global affordability of fluoride tooth paste.

For questions 31-42, choose the answer (A, B or C) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

31. The cariogenic potential of diet emerges in area such as

A. Many developed countries

B. Many developing regions and countries in the world

C. many under developed regions

32. One third of the global population do not access required medicine because of

A. Location

B. Affordability

C. Education

33. According to the speaker the optimal use of fluoride is important to

A. Treat dental caries

B. Monitor dental caries

C. Manage dental caries

34. Which of the following is not mentioned as a way of administering fluoride to the public

A. Milk

B. Toothpaste

C. Bottled water

35. According to the idea of equity marketing, the price of medication should be

A. Free for the poor

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B. Based on consumer income

C. Equal in all countries

36. The major cost in toothpaste production comes from

A. Contents

B. Labour

C. Packaging

Extract 2: Questions 37-42

In this part of the test you will hear a presentation by a consultant regarding Tourette syndrome.

For questions 37 to 42, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes

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37. which age group is commonly affected by Tourette syndrome?

A. 2-20 years

B. 2-21 years

C. 12-20 years.

38. One of the possible cause of Tourette syndrome is

A. brain disorders

B. viral infection

c. abnormal catabolism of brain chemicals.

39. Name one of the factor which intensify the symptom

A. Tension

B. Concentration

C. Relaxation

40. which is an example if tics/

A. Swearing

B. Grunting

C. Grimacing

41. Which is an example of vocalization?

A. Sifting

B. Clearing the throat

C. Shrugging

42. Which of the following statement is most correct regarding the Tourette syndrome?

A. The condition is rarely inherited.

B. The condition is less common in boys.

C. People with the condition appear to metabolize brain chemicals differently.

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ANSWERS

1. FEELINGG'
2. NAUSEOUS
3. COMPLETE
4. WIFE
5. 38
6. 2
7. BUILDING
8. TIMOR/NUIGINI
9. MODERATION

10. RETIRED
11. POSSIBLE
12. OVERSEAS TRAVEL
13. FOUR
14. ASTHMATIC
15. BAD
16. SLEEP
17. BLUE
18. NOT EFFECTIVE
19. MANAGE
20. CAT
21. WORSE
22. BROWN
23. NEBULIZER
24. 100

PART-B

25. A
26. B
27. C
28. A
29. A
30. A

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PART-C

31. B
32. B
33. C
34. C
35. B
36. C
37. B
38. C
39. A
40. C
41. B
42. C

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

You hear a nurse talking to a patient's husband called David. For questions 1-12 complete the notes with word or a short phrase.

You have 30 seconds to look at the notes.

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Patient : Jill.

Background : Jill had a (1) _____ night, and I was giving (2)_____, but felt that it wasn't effective as before.

Eating Habits : I have tried to give different things like (3)_____ and easy to eat but she has less appetite. I have tried some (4) _____.

Family Support : We had help from my (5) _____but Jill does not want to be a (6) _____.

Benefits of Palliative care: In case of breakthrough pain, doctors can be (7) ____ and (8) _____ at how they can really relieve that. Palliative hospitals do have (9) _____ and another thing is you told me you have (10) _____.

Other Support : There is opportunity to have spiritual support. The (11) _____from a number of churches visit regularly. And for the boys there is (12) _____ counselling service.

Extract 2: Questions 13-24

You hear Dr. Jane Hope Interviewing Henry Bunyan a patient with recent problem.

For questions 13-24 complete the notes with word or a short phrase.

Patient	: Henry Bunyan
Lab reports	: Your cholesterol was 13)_____,Your Triglycerides is 14)_____, and your 15) _____ was 4.656.
Background	:Cholesterol is carried in the blood by 16)_____. Higher 17)_____ in your diet, 18)_____ in stress may be you are not exercising.
Family History	: Father died of 19) _____
Diet	: Enjoy 20) _____ at lunch but doesn't like 21) _____
Exercise	:Less exercise due to 22) _____ and family commitment
Life Style	: Smokes about 23) _____ cigarettes in a day and 24) _____ at weekends.

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Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

Now you will hear a talk about ECG

25.Treatment for patients who are identified with STEM1 is:

a) Possible cardiac ischemia

b) Screening STEM1

c) Is based on initial readings

Now you will hear a talk about Cannulation and venipuncture

26. The main difference between venupuncture and cannulation is :

a) Venupuncture is restricted to venous and cannula can have access to vein or artery

b) Cannula is used to administer medicines and venupuncture us used to administer intrinsic factors.

c) Venupuncture insertion depth is twice as compared to cannulation.

Now you will hear a talk about subcutaneous injections

27. Which type of equipment is of single or multi use.

a) Vials of liquid medication

b) Syringes

c) Insulin springs of 25 or 27 gauge

Now you will hear a nurse explaining about lumbar puncture

28. The hollow needle is introduced by:

a) Pressure point

b) Subarachnoid space

c) Spinal Fluid

Now you will hear a instructions about asciting tapping

29.What indicate the appropriate warfarin dosage:

a) Introduction of alternate blood test.

b) Chucking INR as a blood test

c) Stopping of aspirin.

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Now you will hear a talk about instruction about colonoscopy

30. The client should abstain from which diet to prevent uncertainty of colon

a) Red liquids

b) Solid foods

c) Meat

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

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31. Health literacy means

a. Ability to read

b. requires a complex group of skills

c. is related to years of education

32. One of the aim of consumer health core system is

a. Health core environment

b. Overall health quality

c. Assess healthcare cost

33. According to recent studies stronger predictor of persons health

a. for health literacy

b. Standard measured

c. Health information

34. One of consequence of poor health literacy is

a. Population face vulnerability

b. Don't understand simple instruction

c. Low income

35. Poor decisions are often made in rejecting treatment because of :

a. Informed consent

b. Complex forms of decisions

c. Low understanding ability due to complex consent

36. Consumer health librarians have partnership with

a. Material providing committees

b. Community based organisations

c. Rewriting health material organization.

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Extract 2: Questions 37-42
You will hear an eye specialist talking about color blindness. For questions 37 to 42, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes

37. The main reason that most of the people are not aware of being color blind is because:

a. They are blind

b. They don't know about other people who are color blind

c. They are color blind from birth

38. The main characteristics or function of rod cell

a. React to only sky light

b. React to pole light

c. React to different light

39. We get wide range of colors from

a. by combining light

b. From a type of cone cell

c. Three types of cones cells

40. The child will not have red-green color vision if

a. One gene is normal and other defected

b. If the gene are carrier type

c. If the carrier genes are red and green in color

41. Color blindness might be because of

a. Different chromosome

b. Abnormal chromosomal development

c. Chromosome 7

42. Unparalleled way to test color blindness is

a. Internet

b. Face-to-face

c. Printed Charts

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ANSWERS

PART A

1) Restless

2) Medication

3) Soups

4) Green teas

5) Sister

6) Burden

7) Called

8) Look out

9) Individual rooms

10) Full health insurance

11) Local ministers

12) Bereavement

13) 6-6

14) 1.6

15) LDL

16) Lipoprotiens

17) Fat content

18) Increase

19) Stroke

20) Macdonolds

21) Vegtables

22) Busy work

23) 5-10

24) Glass of wine

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PART B

25) A

26) C

27) A

28) B

29) B

30) A

PART C

31) B

32) B

33) A

34) B

35) C

36) B

37) C

38) B

39) C

40) A

41) B

42) B

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OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

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At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

In this part of the test, you will hear Nurse Melissa Sweet interviewing Jim Fisher an elderly Patient. For questions 1-12 complete the notes

with word or a short phrase.

You have 30 seconds to look at the notes.

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Patient	:	Jim Fisher
Medication History	:	Taking (1)_____ for arthritis. I take treatment every (2)_____ for removing sun spots.
Hobbies	:	Likes gardening and apart from that I enjoy (3)_____, but do not really (4) _____ that much
Present Concern	:	Doctor has taken out a (5) _____ but he said he's got it all.
Blue care	:	It is a(6) _____ They will come every (7)_____ to check on your wind
Benefit of Blue Care	:	Services are (8)_____ by government and no one is excluded from receiving a service because of (9)_____

Provide social support such as (10)_____ for spiritual support.

Ambulance service dropping is provided to make sure that you are

(11)_____

The appointment has been made to the outpatient department

(12)_____

Extract 2: Questions 13-24

In this part of the test, you will hear Doctor Brian Fleming interviewing Mr Murray a parent Of a child presenting today .For questions 13-24 complete the notes with word or a short phrase.

Patient	:	Kate
Assessment	:	Kate has 13)_____ and looks 14)_____
Background	:	She had a fall at 15)_____ class and she might have landed awkwardly in one of the 16)_____.
History	:	Has occasional asthma for which she takes 17) _____
Observation	:	On observation, nasty 18) _____ and 19)_____ was noticed on her right leg. X- ray revealed 20) _____ fracture of 21)_____. Even though Mr. Murray was expecting it as 22)_____
Management	:	Prescribed painkiller which contain both paracetamol and 23)_____ Can get some 24) _____ from the pharmacy across the road.

MANJORAN Connect

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

You hear a discussion between a nurse and a physician explaining about Nipah virus.

25. The genus of Nipah virus is;

A. Paramyxoviridae

B. Henipavirus

C. Pteropus

You hear a monologue of a physician explaining about the use of Thimerosal in vaccines.

26. Which of the following statements is correct?

A. Thimerosal contains methylmercury

B. Ethylmercury causes severe side effects to humans

C. Ethylmercury is the most widely used form of organic mercury

You hear a discussion between two doctors on adjuvants used in vaccines.

27. Which of the following statements is correct?

A. Aluminum gels are used in Cervarix vaccine

B. Monophosphoryl lipid A has been used as an ingredient in vaccine since 1930

C. Cervarix doesn't contain aluminum salts

MANJOORAN Connect

You hear a monologue of a physician explaining about Typhus Fevers.

28. Sylvatic typhus is also known as;

A. Scrub typhus

B. Louse-borne typhus

C. Murine typhus

Now you will hear a nurse talking about the role of nurse in wound care:

You hear a discussion between two doctors about VX and its effects.

29 Which of the following statements is correct?

A. Sarin is much more toxic by entry through the skin.

B. Symptoms will start appearing within few seconds of exposure to the liquid form of VX.

C. The least volatility makes the VX as short-term threat as well as long term threat.

You hear a monologue of a doctor briefing about Rubella disease.

30. The vaccine for Rubella is not meant for;

A. Mumps

B. Measles

C. Shingles

MANJORAN Connect

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

31. Anemia correction in patients with heart disease

a. Heat failure symptoms affected

b. Improves the quality of life and also helps in performing exercise

c. Found evidence which contribute in little amount regard to blood transfusion.

32. One of the non-compatible findings across the trials is:

a. It improves only exercise tolerance

b. Involved slightly old age patients

c. Findings with no effect

33. The recent proposal forward by an authoritative body indicates

a. Use of ESA in patients with Hgh of at least < 10g/dl

b. uncertainty of ESA in CKD and cancer population.

c. Elucidate the role of less aggressive ESA

MANJOORAN Connect

34. Intravenous iron carboxymaltose is not suggestable for

a. Patients with heart failure under special circumstance

b. Class III heart failure with NYHA

c. Patients with indistinct evidence of relative iron deficiency.

35. Which kind of patients are more vulnerable to ischemia

a. Patients with coronary stenosis

b. Patients with anemia

c. Patients with impair oxygen exchange

36. Randomized controlled trials support use of

a. Hemoglobin trigger trigger < sg/dL

b. Hemoglobin trigger > sg/dL

c. ESA's in heart disease patients.

Extract 2: Questions 37-42

In this part of the test you will hear a lecture based on the subject of anaphylaxis. For questions 37 to 42, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes

37. Anaphylaxis like reaction include

a. Allergic reactions

b. Reactions similar to anaphylaxis

c. Foreign substance is identically introduced.

38. Which type of anaphylaxis occurs more recurrently?

a. Mider forms of anaphylaxis

b. Life threatening anaphylaxis.

c. fatal anaphylaxis

39. Fatality rate of people with anaphylaxis is

a. I in 50 adults

b. 1 per 100-200 episodes

c. 1 per 3 million

40. The patients with hereditary pre-disposition to develop allergy are known as

a. Classic allergies

b. Atopic

c. Anaphylactic allergens

41. Which type of allergic reaction is sporadic

a. Severe allergy towards inhaled allergen

b. Severe allergy towards food

c. Skin contact with food

42. Epi-pen is a type of

a. Allergy tool kit

b. Treatment for shock

c. Adrenaline

MANJOORAN Connect

ANSWERS

Part A

1) Feldene

2) Six months

3) Building model plan

4) Get out

5) Lot of tissue

6) Church based organization

7) Second day

8) Subsided

9) Financial hardship

10) Chaplain

11) Settled

12) Next Thursday

13) Pain

14) Pale

15) Gymnastics

16) Practice exerciscs

17) Ventolin inhaler

18) Bruise

19) Swelling

20) Spiral

21) Right tibia

22) Sprain

23) Codeine

24) Crutches

MANJOORAN Connect

PART B

25) B

26) A

27) B

28) C

29) B

30) A

PART C

31) B

32) B

33) A

34) C

35) A

36) B

37) B

38) A

39) C

40) B

41) A

42) C

MANJOORAN Connect

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of

each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract

ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health

professional is talking to a patient. For questions 1 to 24, complete the notes with information you

hear. Now, look at the notes for extract one.

MANJOORAN Connect

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

In this part of the test, You hear a GP talking to a patient called Daniel Anderson.
For questions 1-12 complete the notes

with word or a short phrase.

You have 30 seconds to look at the notes.

Patient	:	Daniel Anderson
Presenting Symptoms	:	• 2 wks / (1) _____ than usual • Short of breath • Cold/sore throat
Recent History	:	• Last (2) _____ months • Ongoing condition • Patient managed condition • Bad sore throat and (3) _____ • Fatigue • Shortness of breath – becoming more (4) _____ • Breathing normal

(5) _____

• No history (6) _____ /chest problem

• Non-vegetarian
• Describes diet as (7) _____
• Low fruit and vegetable intake
• Drinks (8) _____ of coffee a day
• Ongoing mild indigestion
• Bowel Function – (9)_____

Family History : • No family history (10) _____
• Parents - healthy
• (11) _____ – no known health problems
• Patient suspects (12) _____

MANJOORAN Connect

Extract 2: Questions 13-24

In this part of the test, You hear a GP talking to a patient called Mrs Wright
For questions 13-24 complete the notes with word or a short phrase.

Patient : Mrs Wright
Background :
• Follow up consultation
• Patient has (13) _____
• Initial episode (14) months ago
• Prescribed (15) _____ for 5 days
• Alopurinol
• Recurrence of gout just (16) _____
• Gout originates at bottom of (17) _____
• on the (18) _____ foot
• Swelling radiates across toes towards (19) _____ of foot
• Hot and inflammed

• No swelling in (20)
• Responded well to medication
• Swelling settled within (21) _____

- Ongoing (22) _____ in toe
- Suspected (23) _____
- X-ray clear
- Patient has some concerns
- Patient agrees medication effective
- Clinical Decision: (24) _____ with current medications

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

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25. You hear a nurse in the emergency department discussing the care of a patient with a doctor.
What do the speakers agree to do?
A Put the patient's arm in a sling.
B Wait for the patient to calm down.
C Treat the patient's pain to relax the arm.

26. You hear a trainee doctor discussing a patient diagnosis with a tutor.
What is the diagnosis?
A Coeliac disease
B Constipation
C Bilateral lower abdominal pain

27. You hear a hospital nurse briefing a colleague about a patient recovering from elective surgery.
What does he want his colleague to do?
A review post-op information with the patient
B provide pain relief for the patient
C get the patient a medical certificate

28. You hear two hospital managers talking about an information session for people who want to do voluntary work.
What problem do the managers discuss?
A How difficult it is for participants to get to the event.
B Technical issues with the event management systems.
C Finding good quality food and drinks for the event.

29. You hear a pharmacist talking to a doctor about a patient's medication.
What is the pharmacist doing?
A reporting side effects
B checking the dosage
C recommending an alternative

30. You hear a doctor advising a patient about a change in medication.
What condition is the patient being treated for?
A hay fever.
B drowsiness.
C hives

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

MANJOORAN Connect

Extract 1: Questions 31-36

You hear an interview with Dr Cristine Ericson, who's talking about her research supporting non-fasting lipid blood tests for cholesterol.

You now have 90 seconds to read questions 31-36.

31. What is Dr Cristine Ericson's opinion about fasting before blood tests?
A It is not supported by scientific evidence.
B It is superior to non-fasting blood tests.
It is supported by convention not data
C It is supported by convention not data.
It is supported by convention not data

32. What does Dr Cristine Ericson say about the research on non-fasting blood tests?
A The majority comes from Copenhagen.
B More studies should focus on children.
C It includes data from multiple countries.

33. What is Dr Cristine Ericson's opinion on the difference between the results of fasting and non-fasting tests?
A The small differences are unimportant.
B There is no difference in the results.
C There is a small but significant difference.

34. Dr Cristine Ericson states that everyone in Copenhagen was happy with the introduction of
non-fasting blood tests.
What evidence does she provide to support this claim?
A Data showing an increase in test attendance.
B Increased public demand for non-fasting tests.
C Positive stories from other medical professionals.

35. According to Dr Cristine Ericson, why are non-fasting blood tests popular with the public?
A Because of the strong research evidence in the report.
B Because of positive publicity in the newspaper and on TV.
C Because they are more convenient for busy lifestyles.

36. Which of the following statements best summarises Dr Cristine Ericson's position on the use of fasting or non-fasting blood tests?
A In rare cases fasting before a blood test is necessary.
B Fasting before a blood test should no longer be recommended.
C New research is needed on the use of fasting and non-fasting tests.

Now look at extract two.

Extract 2: Questions 37-42
You hear a presentation in which a researcher called Dr Milan Patrecivich is talking about the relationship
between new technology and medicine in the future.

You now have 90 seconds to read questions 37-42.
37. Dr Patrecivich suggests that in the future medical technology will
A continue to replace human doctors.
B give doctors freedom to express core skills.
C become more accurate and reliable than doctors.

38. According to Dr Patrecivich, how will wearable sensors and smartphones help GPs?
A They will allow doctors mobility to visit patients in remote locations.
B They will allow doctors to check if patients are following their advice.
C They will allow doctors to monitor their patients with real-time data.

39. Dr Patrecivich believes that
A radiologists currently spend too much time checking through images.
B technology like IBM's Medical Sieve isn't yet capable of doing a radiologist's job.
C radiography algorithms should be designed by professional radiologists.

40. According to the talk, both sports medicine and oncology
A will use technology to make their processes faster and cheaper.
B will use technology to filter data and individualise therapy.
C will use technology like Xbox to treat patients from a distance.

41. What does Dr Patrecivich suggest about the effect of technology on cancer treatment?
A Artificial intelligence technology will eventually cure cancer.
B Oncologists will keep patients informed using social media.
C Technology will have a powerful and positive effect on oncology.

42. What is Dr Patrecivich's conclusion about the future of medicine and technology?
A Medical technology will give doctors more time and better data to care for patients.
B Physicians will have amazing opportunities to develop new medical technologies.
C Automated and robotic systems will gradually replace humans in medical practice.

ANSWERS

Part A

1) A bit more tired/ more tired

2) Couple of / two / 2

3) Big glands / glands felt quite big / swollen glands/ glands bigger

4) Noticeable

5) At rest

6) Of asthma

7) Terrible

8) 4 or 5 cups / 4 cups / 5 cups /4-5 cups

9) Normal / fine

10) Anemia / blood disorders / of anemia / of blood disorders

11) Brother

12) Related to glandular fever / glandular fever

13) Gout

14) 2 or 3/2-3

15) Steroids /prenisone/prednisone 30mg

16) After chirstmas

17) Second toe/ the second toe

18) Left

19) The outside/outside

20) Big toe/the big toe

21) A week/one week/7 days

22) Pain

23) Stress fracture/stress fracture

24) Carry on/continue

PART B

25) C

26) B

27) C

28) C

29) A

30) A

PART C

31) C

32) C

33) A

34) B

35) C

36) B

37) B

38) C

39) A

40) B

41) C

42) A

MANJOORAN Connect

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract

ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health

professional is talking to a patient. For questions 1 to 24, complete the notes with information you

hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

You will hear part of a consultation between a GP and a patient called Mrs Brownstone.

For questions **1-12**, complete the notes with a word or short phrase. For questions 1-12 complete the notes.

with word or a short phrase.

You have 30 seconds to look at the notes.

Patient	:	Mrs Brownstone
Background/Symptoms	:	Patient requesting urinalysis
		Recently **(1)** _____ burning sensation
		Symptoms present approximately **(2)** _____weeks
		Patient suggests symptoms related to **(3)** _____
		Describes pain as not severe
		Recurring condition

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Treated for similar symptoms **(4)** _____ in last **(5)** _____

Symptoms come and go

Patient forgets to drink water

Nocturia - **(6)** _____ nightly

Pain, urgency, **(7)** _____greater at night

Antibiotics effective in the past

(8) _____ urinary incontinence

No impact from coughing, sneezing

Successful colposuspension: aged **(9)** _____

Medication : Known allergies: **(10)** _____

Amoxil

(11) _____

Nitrofurantoine

Treatment Plan : Short course nitrofurantoine

Review appointment in **(12)** _____

Extract 2: Questions 13-24

You will hear part of a consultation between a doctor and a patient called Mrs Chambers. For questions **13-24**, complete the notes with a word or short phrase.

Patient	:	Mrs Chambers
Background/Symptoms	:	Presented after heavy bleeding approximately **(13)**_____ ago 36 weeks pregnant Previous episode at **(14)**_____lasted approx. two hours Estimated bleed 1/2 cup Current episode **(15)**_____ hours Ongoing **(16)**_____ and passing clots Estimated bleed 1 cup Tightness / cramps Severe pain in **(17)**_____ concurrent with bleeding Symptoms come and go - "like period pain" Pregnancy normal to date Previous scan found **(18)**_____ No further scans scheduled 12-week scan showed baby was **(19)**_____but fine Antenatal clinic visit normal Low **(20)**_____ Number of previous pregnancies: **(21)**_____

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Boys, **(22)**_____ healthy

Normal delivery

4 four years ago-abdominal myomectomy

Suffered from **(23)**_____body aches, fever

Emergency **(24)**_____ found large fibroids

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

25. You hear a doctor and a nurse reviewing a coma patient.

What is the Doctor checking for?

a.The symptoms the patient is exhibiting

b. The severity of the patient's coma

c. The range of mobility of the patient

26. You hear a consultant emergency doctor talking to a patient following an accident.

What is she doing?

a. explaining why she is removing the neck brace
b. clarifying the need for multiple scans and surgery
c. outlining the full extent of the patient's injuries

27. You hear a clinical researcher introducing a professional development workshop.

What is the aim of the workshop?

a. To give participants strategies to gain greater trust from the public.
b. To introduce participants to the basics of mathematical statistics.
c. To lead participants to a deeper critical understanding of research

28. You hear an emergency ward doctor directing her team while treating an accident victim.

28. What is the doctor most concerned about?

a. The severe injury to the patient's left leg.

b. The possibility of other less apparent injuries.

c. The limited circulation of blood around the injury.

MANJOORAN Connect

29. You hear a doctor speaking to a woman who has presented with heart palpitations.

What treatment does the patient require?

a. intravenous medication
b. a neck massage
c. training in breath control

30. You hear an emergency paramedic talking to an accident and emergency doctor about a patient who has fallen from his motorcycle.

What is the paramedic doing?

a. confirming what happened to the man in the accident
b. summarising the patient's injuries and treatment
c. suggesting appropriate pain medication to administer

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

Extract 1: Questions 31-36

you'll hear a health professional talking about aspects of their work.
You now have 30 seconds to read questions 31-36.

31. What is the stated purpose of the talk?

A. To evaluate if the audience are feeling burnout

B. To inform and stimulate discussion about burnout

C. To describe new research on treatment of burnout

32. What did the research from the Mayo Clinic show?

A. In 2011 more than half of US doctors claimed to have burnt out.

B. The large number of doctors with burnout is continuing to rise.

C. Typically one aspect of a doctors work makes them burn out.

33. What is the Anna Capstone's attitude to the problem of burnout?

A. Most doctors can handle burnout.

B. Doctors with burnout should retire.

C. Burnout endangers patients and doctors.

34. How does Anna Capstone think the problem of burnout should be addressed?

A. Hospitals should fund wellness programs for burnt out staff.

B. Multiple strategies are needed to keep doctors functioning and healthy.

C. By discouraging doctors from losing empathy and making mistakes.

MANJOORAN Connect

35. How does Anna Capstone evaluate the role of administrative technology and regulations?

A. As a cheap and reliable way of collecting payment.

B. As confusing and stressful for doctors.

C. As something few doctors have really complained about.

36. Anna Capston argues that protecting the well-being of highly trained medical staff

A. is an ethical issue.

B. isn't part of workplace safety.

C. will require too many resources.

Now look at extract two.

Extract 2: Questions 37-42
In this part of the test, you'll hear a health professional talking about aspects of their work.
You now have 90 seconds to read questions 37-42.

37. Dr MacGregor believes that

A. most people will find the healthiest diet quite surprising.

B. intake of meat, eggs, and dairy is an essential part of a balanced diet.

C. the public is quite well informed about the ideal foods for a healthy diet.

38. What is Dr MacGregor's attitude to dieting?

A. More people should go on diets.

B. Dieting is a lifestyle choice.

C. Diets are generally ineffective.

39. Which industry first "used science against itself"?

A. The meat industry.

B. The egg industry.

C. The tobacco industry.

MANJOORAN Connect

40. According to Dr MacGregor, in the mid 20th century

A. the public respected and trusted science to reveal the truth.

B. smokers clearly understood the risks associated with tobacco use.

C. the US Surgeon General first reported on the cancer risk from smoking.

41. According to Dr MacGregor the tobacco industry

A. gathered a mass of evidence on the dangers of smoking.

B. used its influence to slow down government responses.

C. has finally recognised that smoking causes lung cancer.

42. The article suggests that the tobacco and food industries share which tactic?

A. Funding their own research to confuse people.

B. Using the motto "doubt is our product".

C. Denying evidence that their products are bad.

ANSWERS

Part A

1) Going more

2) 3 to 4 weeks/3 weeks/4 weeks or 3-4 weeks

3) Age/old age

4) 5 times

5) 10 years

6) 3 times or more/3 times

7) Pressure

8) No

9) 50

10) Septrin

11) Trimethoprim

12) 5 days

13) 1 week/one week

14) 28 weeks

15) Three

16) Spotting

17) Lower abdomen

MANJOORAN Connect

18) The plancenta was low/plancenta low/the plancenta low

19) Small

20) Down's risk

21) One

22) Twins

23) Migraines

24) MRI

PART B

25) A

26) C

27) C

28) B

29) A

30) B

PART C

31) B

32) B

33) C

34) B

35) B

36) A

37) C

38) C

39) C

40) A

41) B

42) A

MANJOORAN Connect

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

Questions 1 to 12

You hear a doctor talking to Ms Myrna, an outpatient in the psychiatric unit. For questions 1 to 12, complete the notes with a word or short phrase.

You have 30 seconds to look at the notes.

MANJOORAN Connect

Patient	:	Chloe
6 months ago		Valium from GP
4 months ago		(1)_____ began
3 weeks ago		discharged from hospital
		felt like she was going to fall over
		(2)_____trouble walking
		heavy legs
		palpitations
Patient's description of symptoms		possible (3)_____
		became (4)_____
		heavy arms

		difficult to (5)_____
		things (6)_____
		in arms
Possible causes		busy lifestyle
		possible underlying (7)_____
Other symptoms		early stage (8)_____ in hands
Personal history	:	married for two years
		husband - steel fitter (own business)
		fights about keep the (9)_____
		have seen a gynaecologist (10)_____
		twice
		waiting on husband's test results
		returning in (11)_____
Recommendations	:	prescribed (12)_____for anxiety
		follow-up appointment in two weeks

MANJOORAN Connect

Extract 2: Questions 13-24

You hear a nurse talking to a patient called Mr McDonald.

For questions 13-24 complete the notes with word or a short phrase.

Patient	:	Mr Lajowski
Details about procedure		removal of lump from back
		lump located near (13)_____
		worried about (14)_____ after operation.

Medical history	:	has (15)_____
		duration - approximately 5 years
		trouble (16)_____
		falls over sometimes
		sometimes freezes in one spot (akinesia)
		(17)_____shake a lot
		(18)_____ arms
		(19)_____ in bed
		reaching for things
Problems with daily activities		turning around when (20)_____from a chair
		tying shoelaces
		doing and undoing (21)_____
		untidy (22)_____
Other issues		cramps at night - 12 months
		patient's (23)_____ is affected
		recently changed medication - cramps
		(24)_____

MANJOORAN Connect

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25 to 30, choose the answer (A, B or C) which fits best according to what you hear.

You hear a doctor talking to a patient who has been having trouble with his water.

25. The doctor wants to confirm if the patient

A. has had burning when passing water.

B. has a blockage in their kidneys.

C. has high blood pressure.

You hear a nurse briefing her colleague about a patient.

26. What does she want her colleague to do?

A. Check the patient is receiving the correct amount of fluids.

B. Order another chest radiograph to check for an effusion.

C. Confirm the patient is stable and check his blood culture.

You hear part of a consultation at a GP practice with a patient suffering from depression and sleep disturbances.

27. The GP recommends cognitive behavioural therapy because

A. there are better ways for the patient to respond to life's challenges.

B. it will help the patient better manage the way he feels when stressed.

C. the patient needs to control his anger and medication isn't working.

28. You hear a nurse seeking advice from a doctor.

Why does the nurse telephone the doctor?

A. She would like to know how to proceed with the patient.

B. The patient is in intense pain and has a fever.

C. She suspects that the patient has an infection.

You hear the principal of an aged care home talking to staff about osteoporosis.

29. She wants to highlight that

A. there are now treatments available for men.

B. residents are not getting enough calcium and vitamin D.

C. a lot of the residents may already have osteoporosis.

You hear two nurses discussing a patient, Mrs Robbins.

30. Julie says Mrs Robbins refused to mobilise because

A. she was happy having a sponge bath.

MANJOORAN Connect

B. she wanted to hear directly from the doctor.

C. she was upset and wanted to be left alone.

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

You hear a sleep physician and upper airway physiologist called James Tredellow giving a presentation about snoring and its link to health problems.

MANJOORAN Connect

31. Dr Tredellow believes snoring poses concerns because

A. it is possible for anyone to become a snorer.

B. it contributes to several other health issues.

C. everyone has an upper airway that collapses.

32. Dr Tredellow says the main cause of snoring is

A. drinking too much.

B. being overweight.

C. a narrow airway.

33. According to Dr Tredellow there may be societal impacts from snoring for a person who

A. is suffering from obesity.

B. has concentration issues.

C. is worried about their snoring.

34. What point does Dr Tredellow make about obstructive sleep apnoea?

A. snoring sometimes causes people to stop breathing.

B. snoring can be linked to obstructive sleep apnoea.

C. obstructive sleep apnoea and snoring are similar.

35. Dr Tredellow feels research on carotoid artery atherosclerosis

A. is still in its infancy despite highlighting some serious issues.

B. shows that snoring may be far more dangerous than once believed.

C. demonstrates a clear connection between snoring and heart disease.

36. What does Dr Tredellow say about the treatment for snoring?

A. Making specific lifestyle changes contributes to a reduction in snoring.

B. There are too many causes of snoring to be able to treat them all.

C. Treatment is best provided by a recognised sleep physician.

Now look at extract two.

Extract 2: Questions 37-42

In this part of the test, you'll hear a health professional talking about aspects of their work. You now have 90 seconds to read questions 37-42.

37. Why does Dr Munro say he volunteered to treat Ebola patients in Liberia?

A. because of the high death rate from Ebola in Africa.

B. because medical treatment in Africa can be aggressive.

C. because he believed he had a duty to try and help.

38. Dr Munro says the patient's he treated were often afraid because

A. there was a high death rate for patients who received treatment.

B. family members didn't believe that treatment was helping.

C. they didn't trust the doctors because they were foreigners.

39. Why does Dr Munro believe the case of the young girl is tragic?

A. It highlights how even young children can get Ebola.

B. It demonstrates how families were affected by the disease.

C. It is a situation that only affected a small group of patients.

MANJOORAN Connect

40. Why does Dr Munro believe the case of the young girl is tragic?

A. It highlights how even young children can get Ebola.

B. It demonstrates how families were affected by the disease.

C. It is a situation that only affected a small group of patients.

41. Treating Ebola patients in Liberia has made Dr Munro

A. improve how he manages the emotional needs of his patients.

B. realise that seeing patients suffering is part of a doctor's job.

C. understand that doctors can't do everything for their patients.

42. Dr Munro believes Ebola may spread to other parts of the world because

A. it takes longer for the disease to develop than people think.

B. plane travel has made the world a much smaller place.

C. an infected person in a crows could spread the disease.

MANJOORAN Connect

Answer key:

1. Dizzy spells
2. Unsteady
3. Panic attack
4. Breathlessness
5. Lift
6. Tingling
7. Stress
8. Arthritis
9. House clean
10. Stress
11. One month
12. Ativan

13.waist

14. recovery

15. parkinsonism disease

16 . walking

17. hands

18. stiff

19. turning over

20. getting UP

21. Buttons

22. handwriting

23. sleep

24.reduced.

PART B

25.B

26. C

27. B

28. A

29. A

30.B

PART-C

31. B

32.C

33.C

34.B

35.A

36.C

37.C

38.A

39.C

40.B

41.A

42.A

MANJOORAN Connect

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

Questions 1 to 12

You hear a nurse talking to a patient called Miss Wells. complete the notes with a word or short phrase. You have 30 seconds to look at the notes.

Patient	:	Miss Wells
Background details	:	22 years old
		lives with partner
		never been pregnant
		normal (1)_____ 3 months ago
Condition history	:	
Feburary	:	sharp stomach pain (2)_____
		started a few days before period
		ended when period finished
After Feburary	:	constant pain - (3)_____
		(4)_____ performed

MANJOORAN Connect

endometrosis (5)_____ and on left ovary

took the pill without break

prescribed progesterone – weight gain, bloating and (6)_____

May	:	readmitted to hospital (7)_____ treatment pain significantly reduced
August	:	pain returned – condition similar to February pain at different times of month significant pain during (8)_____
Medical history	:	regular periods - most recent (9)_____ never smoked drinks at weekends - 1 or 2 drinks no (10)_____ in the family regular bowel movements
Patient's concerns	:	wants (11)_____ (12)_____affecting her mood

MANJOORAN Connect

EXTRACT 2:

Questions 13 to 24

You hear a nurse talking to Mrs Georges about her husband, who has Alzheimer's disease and has been admitted to a nursing home.

For questions 13 to 24, complete the notes with a word or short phrase.

Patient	:	Mr Georges
		Consultations with Mrs Georges, patient's wife
Background details	:	Mrs Georges is feeling (13)_____
		Mr Georges used to be very (14)_____
		son lives close by.
		also has a (15)_____
		confused by common tasks
		has trouble (16)_____
		he was initially (17)_____ by his condition
		limited awareness now
		frequently repeated (18)_____
		(19)_____very little now
Patient history	:	needs help when (20)_____
		problem getting him to (21)_____
		no longer interested in his appearance
		has recently started to (22)_____ from house
		has trouble telling (23)_____ apart
		sometimes plays with (24)_____

MANJORAN Connect

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25 to 30, choose the answer (A, B or C) which fits best according to what you hear.

You hear two doctors discussing a patient.

25. Dr Jones speaks to Dr Khan because he wants to

A. confirm his own diagnosis.

B. hear Dr Khan's diagnosis.

C. send the patient for tests.

You hear a dietician talking to a patient.

26. What is she doing?

A. Showing the patient alternative diet plans that eliminate fat.

B. Explaining why the doctor wants the patient to eat a low-fat diet.

C. Empathising with the patient about the challenges of reducing fat.

You hear a professor of emergency medicine giving a presentation to a group of trainee doctors.

27. The presentation is about

A. why European wasps are a problem.

B. what European wasps like to eat.

C. how European wasps sting.

You hear a GP talking to a regular patient who has been having kidney problems.

28. What does the GP suggest?

A. That the patient's lifestyle will change forever.

B. That there is more than one way to treat the patient.

MANJOORAN Connect

C. That the patient needs dialysis for the rest of their life.

You hear a physiotherapist in a hospital talking with John, a new patient.

29. The physiotherapist is

A. providing sports recovery options.

B. explaining why warm showers are beneficial.

C. giving instructions to assist with recovery from an injury.

You hear a specialist physician and a nurse discussing a patient's treatment.

30. The nurse is unable to help the physician because

A. she isn't senior enough

B. she isn't familiar with the patient.

C. she doesn't understand the treatment options.

MANJOORAN Connect

Part C

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals

talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

You hear a nurse called Sally Perkins giving a presentation on tetanus to a group of hospital trainees.

31. What is the main focus of Sally's talk on tetanus?

A. It is impossible to avoid tetanus bacteria.

B. It is essential to be vaccinated against tetanus.

C. It is important to develop natural tetanus immunity.

32. Sally believes that deaths from tetanus still occur because

A. the bacteria is about to infect a person through the tiniest of wounds.

B. infected people don't seek medical treatment early enough.

C. not enough people are immunised against tetanus.

33. Why do some people believe the tetanus vaccine isn't necessary?

A. Tetanus bacteria can't enter a person's bloodstream.

B. Tetanus bacteria are everywhere, giving people natural immunity.

C. Tetanus bacteria aren't affected by medication or vaccination.

34. Sally suggests the woman didn't understand the dangers of tetanus because

A. she hasn't undergone a recent vaccination.

B. she sought medical treatment very late.

C. she ignored many early warning signs.

35. Sally highlights the case of the 4-year-old boy to

A. explain the symptoms of tetanus infection.

B. show how tetanus can affect small children.

C. illustrate how difficult it is to diagnose tetanus.

36. Why does Sally believe the man's case highlights how everyone is at risk of tetanus?

A. Because he arrived at hospital with no health complaints.

B. Because the man had no injuries, but he still had tetanus.

C. Because he couldn't remember when he was immunised.

Now look at extract two.

Extract 2: Questions 37-42
You hear an interview with Dr Delia Mene, who was Team USA's Chief Medical Officer (CMO) at the Sochi Winter Olympic Games.
You now have 30 seconds to read questions 37-42.

37. Dr Mene suggests that her role as CMO required her to

A. use a range of care options to assist athletic performance.
B. work longer hours than usual to manage all the injuries.
C. exercise sympathy because it was busy for everyone.

38. Dr Mene says the medical support of athletes

A. was tailored to the specific needs of the individual.
B. was the same as treating patients at a medical clinic.
C. depended on the type of health care provider available.

39. What is Dr Mene's view on providing care for life-threatening injuries?

A. A level of team work would have ensured a positive outcome.
B. They knew there wouldn't be any to attend at the Olympics.
C. If they had occurred, flexible treatment would have been required.

40. Dr Mene says her team prevented injuries by

A. reducing the amount of competition and travel for each athlete.
B. evaluating what athletes ate and what they did after competing.
C. having more advanced technology than other competing nations.

41. Dr Mene was impressed with he software they used because

A. it was particularly useful for musculoskeletal injuries.
B. it had tremendous communication potential.
C. it can help in critical care situations.

42. Dr Mene thinks any physician wanting to treat athletes at a major sporting event should

A. understand that you won't make a lot of money doing it.
B. apply what they learn at these events to their private practice.
C. take the time to forge connections that enhance your reputation.

ANJORAN Connect

ANSWERS

1. Cervical smear
2. Left side
3. Admitted to Hospital
4. Laparoscopy
5. Behind womb
6. Acne
7. Diathermy
8. Intercourse
9. 3 weeks ago
10. Serious illness
11. Children
12. Pain Mark
13. Worn out
14. Alert active
15. Daughter
16. Making tea
17. Frustrated
18. Questions
19. Speaks
20. Dressing
21. Shave
22. Wander off
23. Day and night
24. Oven controls
25. A
26. C
27. A
28. B
29. C
30. B
31. B
32. C
33. B
34. A
35. C
36. B
37. A
38. A
39. A
40. B
41. B
42. C

MANJOORAN Connect

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

Questions 1 to 12

Questions 1 to 12

MANJOORAN Connect

You hear a doctor talking to a new patient called Mrs Black. For questions 1 to 12, complete the notes with a word or short phrase.

Patient	:	Mrs Black
Patient's reasons for presenting	:	family recently moved to the area concerned daughter (1)_____
Details about daughter	:	Name: Mia Age: (2)_____
Mia's eating habits anymore	:	doesn't eat (3)_____free eats in (4)_____

		skips (5)_____ often
		change in eating habits began last year
Mia's school situation	:	good student but (6)_____
		has been having trouble (7)_____
		lately
		has a few close friends but not boyfriend
		mum thinks Mia is concerned about (8)_____
Mia's physical health	:	does ballet
		lies around in her room
		mum reports Mia has (9)_____
		recently
		weighs herself a lot
		looks at herself in the mirror
		doesn't sleep well (tired and moody)
		sometimes has headaches and has taken (10)_____
		may have been (11)_____
Doctor's advice for Mrs Black	:	schedule follow-up appointment
		with Mia and parents
		possible referral to a (12)_____

MANJORAN Connect

EXTRACT 2:

Questions 13 to 24

You hear a physiotherapist talking to a patient called Ryan Henderson.

For questions 13 to 24, complete the notes with a word or short phrase.

Patient	:	Ryan Henderson
Patient's description of symptoms	:	pain located in the left knee (13)_____ pain began 4 weeks ago (noticed first when running) described pain as (14)_____ worse going from sitting to standing position or when sitting with a (15)_____ worse (16)_____ in the day doesn't wake patient at night no other symptoms noted (pins, and needles, numbness)
Medical history	:	(17)_____left knee 15 years ago scare still visible (no associated discomfort) broke little toe on left foot aged 10 broke fibula in (18)_____aged 25 broke little finger on right hand last year last year bursitis in (19)_____ Previous treatment (20)_____ for bursitis stopped running for 6 months did stretching and (21)_____ two times a week

MANJORAN Connect

treatment helped - started running again last Xmas

Physical examination findings : niggly pain in front knee cap standing on left leg

half squat caused pain in (22)_____

pain in left knee worse after half squat (sharp pain)

left knee felt (23)_____on strength testing
Diagnosis

left knee-cap appears to be misaligne iliotibial band is tight

(24)_____on outside of left knee (underlying inflammation)

Part B

Questions 25- 30

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 1 to 6, choose the answer (A, B or C) which fits best according to what you hear.

Question 1

You hear a doctor talking to the daughter of a patient from an aged care home.

25. The doctor wants to

A. ensure that Mrs Cox's father has a good quality of life.

B. advise Mrs Cox that her father has Alzheimer's disease.

C. explain the improvements in medical treatment to Mrs Cox.

You hear a consultant giving a talk to a group of trainees.

26.What is the purpose of the talk?

A. to highlight the symptoms associated with a head injury.

B. to discuss the importance of waking patients after a head injury.

C. to convey the significance of the initial 24 hours after a head injury.

You head a vet talking to a client who brought this pet cat, Jimmy, to the vet clinic.

27. What does the vet say has caused Jimmy's problem urinating?

A. Stress from renovations.

B. A fatal urethral obstruction.

C. Struvite cyrstals in his urethra

You hear the nurse unit manager (NUM) discussing the progress if a patient at a multi-disciplinary team meeting.

28. What does she indicate about the patient's son?

A. That he hasn't been feeding the patient.

B. That the patient is largely dependent on him.

C. That he is having problems in his role as a carer.

MANJOORAN Connect

You hear a radiographer talking to nursing staff at an aged care centre.

29. The radiographer is

A. helping nurses prepare patients for CT scans.

B. stressing the importance of contrast media in CT scans.

C. highlighting problems that occur when conducting CT scans.

You hear a GP discussing psoriasis with a patient.

30. The GP recommends sunbed treatment because

A. UVB light is unique in a hospital setting.

B. there is currently a shortage of cream.

C. it will help with the effects of the rash.

In this part of the test, you'll hear two different extracts. In each extract, you'll hear health professionals talking about aspects of their work.

For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your

answers as you listen.

Now look at extract one.

You hear an emergency medicine physician called Dr Carly Dugan giving a presentation about improving health literacy and patient outcomes through communication.

questions

31. What does Dr Dugan suggest defines a patient's health literacy?

A. how frequently they use available health services.

B. how well they communicate with health professionals.

C. how clearly they understand health-related information.

32. As a result of her own research, and other studies, Dr Dugan believes

A. most patients don't listen carefully enough to health professionals.

B. patient health outcomes rely heavily on a person's health literacy.

C. governments aren't doing enough to assist with health literacy.

33. Dr Dugan says most health professionals have at one time

A. had complaints made about their poor communication.

B. incorrectly assumed their instructions were understood.

C. forgotten how to speak to patients in their own language.

34. Why does Dr Dugan share the examples about previous patients?

A. to show how she teaches health literacy.

B. to illustrate findings from recent research studies.

C. to demonstrate the importance of clear communication.

MANJOORAN Connect

35. The paediatrician example about the "teach-back" method demonstrates

A. how easy it is to overlook whether or not a patient has understood instructions.

B. what patients do and do not understand about health literacy.

C. the best way to confirm what a patient understands.

36. What does Dr Dugan make about improving patient safety and care?

A. It is essential to have reliable health care for all patients.

B. Focusing on acute care settings is the most important aspect.

C. Delivery of information beyond the traditional care settings is vital.

Now look at extract two.

Extract 2: Questions 37-42
you'll hear health professionals talking about aspects of their work.

You now have 30 seconds to read questions 37-42.

37. Dr Marshall believes that the low-fat era is coming to an end because

A. the US Dietary Guidelines have changed towards fat.

B. 35 years ago the US Dietary Guidelines were very different.

C. fat and cholesterol are no longer linked in the US Dietary Guidelines.

38. Dr Marshall suggests that LDL particles

A. are more dangerous when they're smaller.

B. have nothing to do with saturated fat.

C. pose no risk to a person's health.

ANJORAN Connect

39. What are Dr Marshall's views on saturated fats?

A. Refined carbohydrates are more dangerous than saturated fat.

B. Not all saturated fats are the same as others.

C. Vegetable oil should replace saturated fat.

40. Why doesn't Dr Marshall believe people should stop worrying about cholesterol?

A. Biological solutions haven't been able to solve the problem.

B. People continue to eat too many foods high in fat and sugar.

C. More research is required to understand how fat enters the blood.

41. Dr Marshall says there is no agreement over nutritional science because

A. there are too many opinions when it comes to a person's diet.

B. there are a lot of different ways to interpret the information.

C. there isn't enough money to fund high quality research.

MANJOORAN Connect

42. In Dr. Marshall's opinion, many organisations and experts need to

A. become more open-minded about embracing current studies.

B. encourage more people to eat a diet high in vegetables.

C. stop saying that eating saturated fat is bad.

ANSWERS

1. Has anorexia
2. 15 years old
3. With the family
4. Her room
5. Breakfast
6. Quiet
7. Concentrating
8. Her appearance
9. Lost a lot of weight
10. Laxatives
11. Throwing up
12. Psychiatrist

13. Below the kneecap
14. Niggling
15. Bent
16. Later
17. Cut
18. Right ankle
19. Left hip
20. Psyiotherapy
21. Yoga
22. Both knees
23. Weak
24. Minor swelling
25. A
26. C
27. C
28. B
29. A
30. C
31. C
32. B
33. B
34. C
35. A
36. C
37. A
38. A
39. B
40. C
41. B
42. A

MANJOORAN Connect

OCCUPATIONAL ENGLISH TEST. SAMPLE TEST ONE. LISTENING TEST.

This test has three parts. In each part you'll hear a number of different extracts. At the start of each extract, you'll hear this sound: ---***---.

You'll have time to read the questions before you hear each extract and you'll hear each extract ONCE ONLY. Complete your answers as you listen.

At the end of the test, you'll have two minutes to check your answers.

Part A. In this part of the test, you'll hear two different extracts. In each extract, a health professional is talking to a patient. For questions 1 to 24, complete the notes with information you hear. Now, look at the notes for extract one.

PAUSE: 5 SECONDS

Extract 1: Questions 1-12

In this part of the test, You will hear part of a consultation between a psychologist and a patient called Mr Barry. For questions **1-12**, complete the notes with a word or short phrase..
You have 30 seconds to look at the notes.

Patient	:	Mr Barry
Background	:	Continued anxiety and **(1**_____ Started medication **(2)** _____ ago
Medication	:	citalopram / chlordiazepoxide chlordiazepoxide **(3)** _____Mg **(4)** _____ daily non-compliant feels meds don't **(5)** _____ him sufficiently patient trying to **(6)** _____more citalopram 20mg

side effects: ongoing **(7)** _____
improving

stress related?

possible **(8)** _____

takes citalopram **(9)** _____

Symptoms : inability to leave house

palpitations / SOB / intense fear

feels **(10)** _____ when outside

patient normalizing

went to shops for the **(11)** _____

since beginning **(12)** _____

MANJOORAN Connect

Extract 2: Questions 13-24

In this part of the test, You will hear part of a consultation between a GP and a patient called Mr. Martin. For questions 13-24 complete the notes with word or a short phrase.

Patient : Mr Martin

Background/Symptoms : Stomach pain

Condition present **(13)**_____ for years but worse recently

Located across **(14)**_____ of abdomen radiating towards **(15)**_____ ___

Describes pain as not severe

Like an
(16)_____

Pain moves side to side

Patient
suspects **(17)**_____
problems

Feels **(18)**_____
___as well as pain

Ongoing problem

4 yrs ago diagnosed
suspected **(19)**_____
___

Bowel movements variable

Ongoing problems
with **(20)**_____but
recently
stool **(21)**_____
___

Drinks plenty of water

No diarrhea

No presence of **(22)**..... in
stool

No black motions

Medication : Previously prescribed Colofac

Monitors diet rather than
take **(23)**_____

Takes OTC **(24)**_____
capsules which provide relief

MANJORAN Connect

Part B

In this part of the test, you'll hear six different extracts. In each extract, you'll hear people talking in a different healthcare setting.

For questions 25-30, choose the answer (A, B or C) which fits best according to what you hear. You'll have time to read each question before you listen. Complete your answers as you listen.

25. You hear a doctor and a trainee discussing the application of a plaster cast.

What does the trainee need to tell the patient about the cast?

A. The appropriate length of time to let the cast dry properly

B. The temperature of the cast as it begins to harden.

C. The reduction in the fracture after the cast has been applied.

26. You hear a manager explaining new data management processes to clinical staff.

What should staff do with their feedback reports?

A. File them on paper and digitally

B. Copy them onto the new servers

C. Fill them out digitally to save time

27. You hear a presentation about the introduction of a new type of wound dressing.

What is the main advantage of the new wound dressing?

A. They are ideal for oozing wounds due to high absorbency.

B. They are made from naturally bacteria resistant seaweed.

C. They are somewhat less costly than traditional cloth bandages.

MANJORAN Connect

28. You hear two hospital managers discussing completion rates for an online course.

How will management inform staff who do not attend the meeting about the deadline?

A. Face to face

B. By email

C. Over the phone

29. You hear an educator describing methods for creating medical abbreviations to nursing trainees.

Why does the speaker think it is important for trainees to understand the different methods of abbreviating?

A. So they can create new abbreviations using the first letter rule.

B. So they can easily pronounce medical acronyms accurately.

C. So they can more efficiently learn commonly used abbreviations.

MANJOORAN Connect

30. You hear two colleagues discussing an online training course.

What advice does the female nurse give her colleague?

A. request IT help to reset so he can complete the course.

B. wait until the weekend and complete the course.

C. advise a manager that he has completed the course

Part C

In this part of the test, you hear an interview with a neurosurgeon called Dr Ian Marsh who specialises

In the treatment of concussion in sport. For questions 31-42 , choose the answer (A, B or C) which fits best according to what you hear. Complete your answers as you listen.

Now look at extract one.

31). How does Dr Lee describe the movement from traditional care systems to non-visit care?

A) Large pre-existing medical providers will probably dominate the market.

B) There will be a mixture of reactions from existing and new organisations.

C) Flexible new organisations will have an advantage over existing ones.

32). According to Dr Lee a non-visit care culture

A) views face to face consultations as a final resort in treatment.

B) will develop more rapidly in new medical start-ups.

C) requires a blend of responses from medical organisations.

33). Dr Lee believes that current technology is

A) closely matched to users medical needs.

B) changing expectations for medical services.

C) not capable of supporting non-visit care yet.

34). Dr Lee suggests that in person visits

A) ideally occur earlier in the care process.

B) have a negative impact on many patients.

C) should be easy for patients to manage.

35). Why does Dr Lee mention specimen collection services?

A) To highlight the expense of creating offices and labs.

B) To describe how these services could be provided more efficiently.

c) To suggest they could be delivered more nimbly with technology.

MANJOORAN Connect

36). What advice does Dr Lee give organisations wanting to move to non-visit care models?

A) Wait for more precise technology before transitioning to remote care.

B) Re-evaluate the safety and quality of non-visit care before making the change.

C) Find transition opportunities by close analysis of each instance of in-person care.

Extract 2: Questions 37-42

In this part of the test, you'll hear a health professional talking about aspects of their work.

For questions 37 to 42, complete the notes with a word or short phrase.

You now have thirty seconds to look at the notes

37). The research suggests that in the USA

A) supervised injection centres are fundamentally different.

B) supervised injection centres are being successfully integrated.

C) supervised injection centres have the same impact as elsewhere.

38). According to Dr Davidson the secret facility was

A) based on international evidence and good practice.

B) waiting for legal advocacy in the United States.

C) experiencing the death of too many patients.

39). According to Dr Davidson what is the outcome of having staff and drug users in the same room?

A) Patient check in and registration is more efficient.

B) Overdose prevention is the major benefit.

C) Staff can address many potential health issues.

40). What does Davidson suggest users of the centre most appreciate?

A) the clean and hygienic surfaces

B) the private and relaxed space

C) they can't get arrested there

MANJOORAN Connect

41). Some users of the facility suggested that the facility

A) needs a far more open space.

B) can't combine with other services since it is secret.

C) would be better as a treatment agency.

42). What concern did staff raise about the running of the facility?

A) That patients might disclose information about the centre

B) Necessary exclusion of psychologically ill patients

C) The small number of patients they could help

ANSWERS

PART-A

1. Panic attack
2. 3
3. 20
4. 3 times
5. Suppress
6. Exercise
7. Headaches
8. Ear infection
9. At night
10. Sick and dizzy
11. First time
12. Medication
13. On and off/off and on
14. Upper part
15. Middle
16. Irritation
17. Kidney
18. Bloated
19. Irritable bowel syndrome/IBS/irritable bowel
20. Constipation
21. Looses
22. Blood or slime
23. Laxatives
24. Peppermint oil

MANJOORAN Connect

PART-B

25. B
26. A
27. A
28. A
29. C
30. C

PART-C

31. B
32. A
33. B
34. C
35. B
36. C
37. C
38. A
39. C
40. B
41. B
42. B

MANJOORAN Connect

OET 2.0

READING

MANJOORAN Connect

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Connect

About the Reading sub-test

The Reading sub-test consists of three parts and a total of 42 question items. All three parts take a total of 60 minutes to complete. The topics are of generic healthcare interest and are therefore accessible to candidates across all professions.

The Reading sub-test structure

Part A – expeditious reading task (15 minutes)

Part A assesses your ability to locate specific information from four short texts in a quick and efficient manner. The four short texts relate to a single healthcare topic, and you must answer 20 questions in the allocated time period. The 20 questions consist of matching, sentence completion and short answer questions.

Part B and Part C – careful reading tasks (45 minutes)

Part B assesses your ability to identify the detail, gist or main point of six short texts sourced from the healthcare workplace (100-150 words each). The texts might consist of extracts from policy documents, hospital guidelines, manuals or internal communications, such as emails or memos. For each text, there is one three-option multiple-choice question.

Part C assesses your ability to identify detailed meaning and opinion in two texts on topics of interest to healthcare professionals (800 words each). For each text, you must answer eight four-optionmultiplechoicequestions.

How is reading ability assessed in OET?

Reading Part A (the expeditious reading task) tests your ability to skim and scan quickly across different texts on a given topic in order to locate specific information. For that purpose, Part A is strictly timed and you must complete all 20 question items within the allocated 15 minutes. To complete the task successfully, you will also need to understand the conventions of different medical text types and understand the presentation of numerical and textual information.

Reading Part B tests your ability to understand the detail, gist or main point of complex texts commonly found in the healthcare workplace. To complete the task successfully, you will need to identify specific ideas at sentence level.

Reading Part C tests your ability to understand the explicit or implied meaning as well as the attitude or opinion presented in a longer text. To complete the task successfully, you will need to identify the relationship between ideas at sentence and paragraph level. Part C also tests your ability to accurately understand lexical references and complex phrases within the text. Assessors who mark the Reading sub-test are qualified and highly trained. Candidate responses are assessed against an established marking guide. During the marking session, problematic or unforeseen answers are referred to a sub-group of senior assessors for guidance.

How are marks for the Reading sub-test distributed?

There is a total of 42 marks available in the Reading sub-test, Part A accounts for 20 marks, Part B accounts for 6 marks and Part C accounts for 16 marks.

Where do I write my answers for the Reading sub-test?

In Part A you should write your answers clearly in the spaces given in the question booklet.

In Part B and Part C, you must shade the circle next to the appropriate answer. Answers written elsewhere in your booklet will not be marked.
It is a good idea to use the sample tests to familiarise yourself with the different task formats you might find in the test.

Do I have time to check my answers for the Reading sub-test?
You will not be given extra time at the end of the sub-test to check your answers, and it is up to you to manage your time. The test is designed so that the time available is enough for you to read, choose your answers, and check your work.
Please remember that there is a strict time limit for Part A (the expeditious reading task), and Part A materials will be collected from you after 15 minutes. You will therefore not have any time to check your Part A answers later in the test.

Do my answers have to use the same words as given in the texts?
Yes, in Part A you must use exactly the same form of the word or short phrase as given in the four t

Can I use abbreviations in the Reading sub-test?
Abbreviations are not accepted in the Reading sub-test unless they appear in the texts.

Do I lose marks for spelling mistakes?
Yes, you must use correct spelling in the Reading sub-test to get the marks. Responses that are not spelled correctly will not receive any marks. American and British English spelling variations are accepted, e.g., foetus and fetus are both acceptable. Please note that the Reading sub-test is different from the Listening sub-test in the way misspellings are treated.

How many questions do I need to get correct for the Reading sub-test?
New reading tests are written for each test session incorporating new material and the grade boundaries are adjusted slightly for each test to allow for minor differences in the difficulty of items included in that particular version. The number of marks needed to secure grade B will therefore vary. However, test-takers awarded grade B (a scale score of 350) will typically have a score of at least 30 marks

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ASTHMA

TEXT A

Asthma is a chronic reactive airway disease characterized by reversible inflammation and constriction of bronchial smooth muscle, excessive secretion of mucus, and edema. Asthma causes recurring periods of wheezing, chest tightness, shortness of breath, and coughing. There are many factors that airways react to which can precipitate asthma, including allergens, physical and emotional stress, cold weather, exercise, chemicals, medications, and infections. There is no cure for asthma, but it can be controlled with effective treatment and management.

TEXT B

When people are diagnosed with asthma when they are older than age 20, it is known as adult-onset asthma. The thirties is the typical decade for symptoms to appear. Adult-onset asthma is most common in females during the childbearing years, when body and hormonal changes are occurring. Asthma may develop during or immediately after pregnancy.
Adult-onset asthma is different than childhood asthma because adults have a lower lung capacity and changes in muscles and stiffening of chest walls after middle age. Asthma is common among persons over age 65, and most deaths caused by asthma occur in this age group.

TEXT C

Vitamin E is a non enzymatic antioxidant that protects the body from free radicals and maintains the immune system. Vitamin E is not produced by the body and must be ingested. There are two forms of vitamin E: gamma-tocopherol and alpha-tocopherol.
Recent studies have shown that gamma-tocopherol has been linked to diminished lung function. Gamma-tocopherol is found in canola, soybean, and corn oils, which over the years have become the "healthier" replacements for butter and lard. It has been shown that higher concentrations of gamma-tocopherol in the blood plasma indicated a 10% to 17% reduction in lung function as measured by spirometry.
In contrast, alpha-tocopherol—found in olive oil, wheat germ, and almond and sunflower oils—has been found to have beneficial effects on lung function. Adult-onset asthma patients in the study were found to have significantly lower levels of alpha-tocopherol.

TEXT D

KIT-ON-A-LID-ASSAY (KOALA)

A new diagnostic tool has been developed that can diagnose asthma even in patients experiencing no symptoms at the time of examination and testing. The test requires only a single drop of blood.

This test takes advantage of a previously unknown correlation between asthmatic patients and neutrophils, the most abundant type of white blood cells in the blood. These white cells are the first cells to migrate toward inflammation. Neutrophils detect chemical signals in response to inflammation and migrate to the site to assist with the healing process.

KOALA can track the speed at which the neutrophils migrate (chemotaxis velocity) to differentiate non asthmatic samples from the significantly reduced speed of asthma patients. In the case of an asthmatic patient, the speed of neutrophils movement is slower as compared to a normal patient.

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STRATEGIES TO AVOID COMMON ASTHMA TRIGGERS

Trigger	Exposure Reduction Strategies
Animals (dander, urine, or saliva from animals and birds)	Keep pets with fur or feathers out of the home. If pets cannot be kept outdoors, keep them out of the bedroom and keep the door closed. Install a HEPA air cleaner in the bedroom. Keep pets off upholstered furniture and away from stuffed toys.
Dust mites (arachnids that live on human skin cells that have been shed and that colonize beds, upholstered furniture, and carpets)	Reduce indoor humidity and do not use humidifiers. Wash mattress covers and bedding in hot water each week. Wash stuffed animals frequently and dry completely.

STRATEGIES TO AVOID COMMON ASTHMA TRIGGERS	
Trigger	**Exposure Reduction Strategies**
	Steam clean bedding, mattresses, and furniture that cannot be washed. Use dust-proof pillow and mattress covers. Remove carpeting from the home. Vacuum weekly with a HEPA vacuum cleaner. Avoid lying on upholstered furniture.
Cigarette smoke	Do not allow smoking in the home, car, or anywhere nearby. Quit smoking. Ask a healthcare provider for help to quit and perhaps a referral to a smoking program. If family members smoke, ask them to quit.
Wood smoke, strong odors and sprays, chemical vapors	Avoid strong odors and sprays such as perfume, powder, hair spray, paints, incense, cleaning products, candles, and new carpeting. Avoid inhaling smoke from burning wood. Avoid air fresheners. When in a workplace with chemical vapors, limit or avoid exposure altogether by using respiratory protective gear (ALA, 2015c).
Outdoor air pollution	When the level of outdoor pollution is high, stay indoors as much as possible and avoid exertion when being outdoors. Check the Environmental Protection Agency's website or other sources for daily updates on air quality. Choose routes for walking or exercising that avoid major streets or highways. Instruct children with asthma to play in playgrounds that are not near major highways (NRDC, 2014).

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

ADHD

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. Incidence of asthma in patients
2. Immune system maintenance with the help of dietary management to fight against asthma
3. Adult-onset asthma is most common in females during the childbearing years
4. Reduce indoor humidity and do not use humidifiers
5. There is no cure for asthma, but it can be controlled with effective treatment and management.
6. Avoid strong odors and sprays such as perfume
7. In the case of an asthmatic patient, the speed of neutrophils movement is slower as compared to a normal patient.

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Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. What is the ultimate function of vitamin E?

9. What instruction to be given to children with asthma?

10. In patients experiencing no symptoms, when can asthma be diagnosed?

11. By which method body should obtain vitamin E?

12. Which machine can be used to remove arachnids from carpets, furniture, etc?

13 . Which component of blood shows immediate reaction to inflamation process?

14. What can track the chemotaxis velocity?

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Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

15. There are two forms of vitamin E: ---------- and ----------.

16. ---------- and ----------from animals and birds are common asthma triggers.

17. Reduce ---------- and humidifiers.

18. ---------- are healthier replacements for butter and lard.

19. ---------- detect chemical signals in response to inflammation and migrate to the site to assist with the healing process.

20. ---------- will diminish lung function and ---------- have beneficial effects on lung function.

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

Faciocardiomelic dysplasia

Lethal faciocardiomelic dysplasia is an extremely rare polymalformative syndrome. It was described only once, in 1975, in 3 affected males in a sibship of 13, from second-cousin parents. Patients were all of low birth weight, had microretrognathia, microstomia, and microglossia, hypoplasia of the radius and ulna with radial deviation of the hands, simian creases and hypoplasia of fingers I and V, hypoplasia of the fibula and tibia with talipes and wide space between toes I and II, and severe malformation of the left heart which may have been responsible for death of all 3 in the first week or so of life.

1. The study suggests that out of 13 subship, three males were affected due to

a) Second-Cousin parents.

b) Polymalformative disease

c) Low birth weight

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Menorrhagia

Menorrhagia is a common and major health problem for women. The early recognition of an underlying cause would potentially have a major impact in the diagnosis and treatment of menorrhagia. Recent studies report that the incidence of bleeding disorders as a cause of menorrhagia may be as high as 17-20% Inherited factor 2 deficiency (hypoprothrombinemia) is an extremely rare bleeding disorder, with not more than 50 cases of this disorder reported worldwide so far.Menorrhagia may be the first and the only clinical manifestation of an inherited bleeding disorder. The first patient described with von Willebrand's disease died of menorrhagia at the age of 13 years of age.Despite this, coagulopathies are not usually suspected as etiology of menorrhagia and surgical interventions are done, without getting the patient investigated for coagulopathies or any other systemic disorder known to cause abnormal uterine bleeding like hypothyroidism. Careful history taking and clinical suspicion for an underlying bleeding disorder in menorrhagic women will not only help in their early diagnosis but also for better management of ante partum/postpartum hemorrhage in such women. In the absence of a readily identifiable cause, all adolescents with menorrhagia should be examined for bleeding disorders.

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2. History taking and clinical suspicion in menorrhagic women helps in:

a) Early diagnosis and treatment.

b) Eradication of the bleeding disorders.

c) Reduce the occurrence of inherited bleeding disorders.

Tetralogy of Fallot

Tetralogy of Fallot is a congenital cardiac malformation that consists of an interventricular communication, also known as a ventricular septal defect, obstruction of the right ventricular outflow tract, override of the ventricular septum by the aortic root, and right ventricular hypertrophy.The aetiology is multifactorial, but reported associations include untreated maternal diabetes, phenylketonuria, and intake of retinoic acid. Associated chromosomal anomalies can include trisomies 21, 18, and 13, but recent experience points to the much more frequent association of microdeletions of chromosome 22. The risk of recurrence in families is 3%.

Useful diagnostic tests are the chest radiograph, electrocardiogram, and echocardiogram. The echocardiogram establishes the definitive diagnosis, and usually provides sufficient information for planning of treatment, which is surgical. Approximately half of patients are now diagnosed antenatally.Differential diagnosis includes primary pulmonary causes of cyanosis, along with other cyanotic heart lesions, such as critical pulmonary stenosis and transposed arterial trunks. Neonates who present with ductal-dependent flow to the lungs will receive prostaglandins to maintain ductal patency until surgical intervention is performed. Initial intervention may be palliative, such as surgical creation of a systemic-to-pulmonary arterial shunt, but the trend in centres of excellence is increasingly towards neonatal complete repair. Centres that undertake neonatal palliation will perform the complete repair at the age of 4 to 6 months. Follow-up in patients born 30 years ago shows a rate of survival greater than 85%. Chronic issues that now face such adults include pulmonary regurgitation, recurrence of pulmonary stenosis, and ventricular arrhythmias. As the strategies for surgical and medical management have progressed, the morbidity and mortality of those born with tetralogy of Fallot in the current era is expected to be significantly improved

3. What is the most effective test to diagnose of tetra logy of fallot is:

a) Chest radiograph

b) Echocardiogram

c) Electrocardiogram

Familial Thoracic Aortic Aneurysms and Dissections

The natural history of ascending aortic aneurysms in the absence of surgical intervention is to progressively enlarge over time and ultimately lead to an aortic dissection (Stanford type A) or rupture. Type A aortic dissections are life-threatening events causing sudden death in approximately 40% of affected individuals, and emergency repair of these dissections are associated with a high degree of morbidity and medical expenditure. In contrast, prophylactic repair of an ascending aortic aneurysm is associated with very low morbidity and mortality, leading to the current recommendation to repair an ascending aortic aneurysm before it dissects or ruptures.Although medical treatment can slow the enlargement of ascending aortic aneurysms, the mainstay of treatment to prevent an aortic dissection is surgical repair when the aortic diameter expands to 5.0 – 5.5 cm.Therefore, the optimal aortic diameter when the risk of aortic dissection exceeds that of surgical repair is still debated.

4. Effective treatment for aortic Aneurysms is

a) Surgical Repair

b) Prophylatic repair

c) Both

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Familial Thoracic Aortic Aneurysms and Dissections

Sudden sensorineural hearing loss (SSHL)), commonly known as sudden deafness , occurs as an unexplained, rapid loss of hearing—usually in one ear-either at once or over several days. It should be considered a medical emergency. Anyone who experiences SSHL should visit a doctor immediately. Sometimes, people with SSHL put off seeing a doctor because they think their hearing loss is due to allergies a sinus infection , earwax plugging the ear canal or other common conditions. However, delaying SHHL diagnosis and treatment may decrease the effectiveness of treatment.

Nine out of ten people with SSHL lose hearing in only one ear. SSHL is diagnosed by conducting a hearing test. If the test shows a loss of at least 30 decibels (decibels are a measure of sound) in three connected frequencies (frequency is a measure of pitch—high to low), the hearing loss is diagnosed as SSHL.

5. Anyone Experience SSHL immediately:

a) Visit Doctor

b) Do heavy test in 3 different frequencies

c) Frequency and pitch should be measured

Eosinophilia

Eosinophilia represents an increased number of eosinophils in the tissues and/or blood. Although enumeration of tissue eosinophil numbers would require examination of biopsied tissues, blood eosinophil numbers are more readily and routinely measured. Hence, eosinophilia is often recognized based on an elevation of eosinophils in the blood. Absolute eosinophil counts exceeding 450 to 550 cells/μL, depending on laboratory standards, are reported as elevated. Percentages generally above 5% of the differential are regarded as elevated in most institutions, although the absolute count should be calculated before a determination of eosinophilia is made. This is done by multiplying the total white cell count by the percentage of eosinophils.

Eosinophils are bone marrow–derived cells of the granulocyte lineage. They have an approximate half-life of 8 to 18 hours in the bloodstream, and mostly reside in tissueswhere they can persist for at least several weeks. Their functional roles are multifaceted and include antigen presentation; the release of lipid-derived, peptide, and cytokine mediators for acute and chronic inflammation; responses to helminth and parasite clearance through degranulation; and ongoing homeostatic immune responses. They can be part of the overall cellular milieu in malignant neoplasms and autoimmune conditions, and connective tissue disorders, and are also found in less well characterized entities as described elsewhere in this paper.

6. Eosinophils are:

a) Bone marrow derived cells

b) Autoimmune conditions

c) Connective tissue disorder

PART C

In this part of the test, there is a text about different aspects of healthcare.

For questions 7-22,

choose the answer (A, B, C or D) which you think fits best according to the text.

Hyperthyroidism

The thyroid gland is a butterfly-shaped endocrine gland that is normally located in the lower front of the neck. The thyroid's job is to make thyroid hormones, which are secreted into the blood and then carried to every tissue in the body. The thyroid hormone helps the body use energy, stay warm and keep the brain, heart, muscles, and other organs working as they should.

The term hyperthyroidism refers to any condition in which there are too many thyroid hormones produced in the body. In other words, the thyroid gland is overactive and working too hard. Another term that you might hear being used to describe the problem is

thyrotoxicosis, which refers to high thyroid hormone levels in the blood stream, irrespective of their source.

The thyroid hormone plays a significant role in the pace of many processes in the body; these processes are called your metabolism. If there is too much thyroid hormone being produced, every function of the body tends to speed up. It is not surprising then that some of the symptoms of hyperthyroidism are: nervousness, irritability, increased perspiration, heart racing, hand tremors, anxiety, difficulty sleeping, thinning of your skin, fine brittle hair and weakness in your muscles— especially in the upper arms and thighs.

Another symptom might be more frequent bowel movements, but diarrhea is uncommon. You may lose weight despite a good appetite and, for women, menstrual flow may lighten and menstrual periods may occur less often. Since hyperthyroidism increases your metabolism, many individuals initially have a lot of energy. However, as the hyperthyroidism continues, the body tends to break down, so feeling tired is very common.

Hyperthyroidism usually begins quite slowly but in some young patients these changes can be very abrupt. At first, the symptoms may be mistaken for simple nervousness due to stress. If you have been trying to lose weight by dieting, you may be pleased with your success until the hyperthyroidism, which has quickened the weight loss, causes other problems.

The most common cause (in more than 70% of people) is an overproduction of the thyroid hormone by the entire thyroid gland. This condition is also known as Graves' disease. Graves' disease is caused by antibodies in the blood that turn on the thyroid and cause it to grow and secrete too much thyroid hormone. This type of hyperthyroidism tends to run in families and

it occurs more often in young women. Little is known about why specific individuals get this disease.

Another type of hyperthyroidism is characterized by one or more nodules or lumps in the thyroid that may gradually grow and increase their activity; this causes the total output of thyroid hormones into the blood to become greater than normal. This condition is known as toxic nodular or multi nodular goiter. Also, people may temporarily have symptoms of hyperthyroidism if they have a condition called thyroiditis, a condition caused by a problem with the immune system or a viral infection that causes the gland to leak stored thyroid hormone. The same symptoms can also occur by taking too much thyroid hormone in tablet form. These last two forms of excess thyroid hormone are only called thyrotoxicosis, since the thyroid is not overactive.

If your physician suspects that you have hyperthyroidism, diagnosis is usually a simple matter. A physical examination usually detects an enlarged thyroid gland and a rapid pulse. The physician will also look for moist, smooth skin and a tremor of your fingertips. Your reflexes are likely to be fast, and your eyes may have some abnormalities if you have Graves' disease.

The diagnosis of hyperthyroidism will be confirmed by laboratory tests that measure the amount of thyroid hormones— thyroxine (T4), triiodothyronine (T3) and thyroid-stimulating hormone (TSH) in your blood. A high level of thyroid hormones in the blood plus a low level of TSH is common with an overactive thyroid gland. If blood tests show that your thyroid is overactive, your doctor may want to obtain a picture of your thyroid (a thyroid scan). The scan will find out if your entire thyroid gland is overactive or whether you have a toxic nodular goiter or thyroiditis (thyroid inflammation). A test that measures the ability of the gland to collect iodine (a thyroid uptake) may be done at the same time. .

No single treatment is best for all patients with hyperthyroidism. The appropriate choice of treatment will be influenced by your age, the type of hyperthyroidism that you have, the severity of your hyperthyroidism, and any other medical conditions that may be affecting your health, as well as your own preference. It may be a good idea to consult with an endocrinologist who is experienced in the treatment of hyperthyroid patients. If you are unconvinced or unclear about any thyroid treatment plan, a second opinion is a good idea.

Hyperthyroidism

Questions

7. The thyroid hormone helps with

A. energy consumption

B. utilization of energy

C. maintaining body temperature

D enhancing the functions of the kidney

8. In thyrotoxicosis

A. the thyroid gland is inactive

B. the thyroid gland is less active

C. the thyroid gland produces a greater amount of hormones then necessary

D. none of the above

9. An increase in the amount of thyroid hormones can

A. boost up other hormonal functions

B. improve metabolic functions

C. increase normal physiological functions

D. increase pulse rate

10. Which one of these is common in thyroid diseases?

A. loss of appetite

B. decreased metabolism

C. tiredness

D. none of the above

11. Hyperthyroidism can be the cause of .....

A. high BP

B. tiredness

C. weight loss

D. increase in weight, even while dieting

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12. According to the information given, "Grave's disease" occurs more commonly in

A. men

B. women

C. children

D. adult women

13. In hyperthyroidism, the level of thyroid hormones is

A. considerably higher

B. very low

C. much higher

D. normal

14. Eyes show abnormalities in

A. hyperthyroidism

B. grave's disease

C. thyroid inflammation

D. all

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Idiopathic Pulmonary Fibrosis (IPF)

Passage 1 Idiopathic pulmonary fibrosis (IPF) is a build-up of scar tissue in the lungs. This scar tissue damages the lungs and makes it hard for oxygen to get in. Not getting enough oxygen to the body can cause serious health problems and even death. "Idiopathic" is the term used when no cause for the scarring can be found; in these cases, doctors think the scarring starts by something that injures the lung. Scar tissue builds up as the lungs try to repair the injury and, in time, so much scarring forms that patients have problems breathing.

IPF usually worsens over time. However, while some patients get sick quickly, others may not feel sick for years. Unfortunately, there is no cure for IPF, but there are treatments that may be able to slow down the lung scarring. Understanding the condition will go a long way to help you cope with the effects it has on your body.

The two major symptoms of IPF are shortness of breath and a persistent cough. Other symptoms may include: • Fatigue and weakness • Chest pain or tightness in the chest • Loss of appetite • Rapid weight loss The causes of IPF are unknown. There are other conditions that cause lung scarring; the lung scarring that is the result of other conditions is often called "pulmonary fibrosis", but should be called by the name of the cause. These other causes

include the following: • Diseases, like rheumatoid arthritis and sarcoidosis • Medicines, such as those used for certain heart conditions • Breathing in mineral dusts, such as asbestos or silica • Allergies or overexposure to dusts, animals, or molds (There are many names for this condition, such as "bird breeder's lung," "farmer's lung," or "humidifier lung." These conditions are all called hypersensitivity pneumonitis).

Five million people worldwide have IPF, and it is estimated that up to 200,000 people in the United States have this condition. It usually occurs in adults between 40 and 90 years of age and it is seen more often in men than in women. Although rare, IPF can sometimes run in families.

Patients who have any symptoms of IPF

Smoking Cessation: Cigarette smoke not only damages the lining of the lungs, it can also make you more likely to get a lung infection. While some studies suggest that patients with IPF who smoke actually live longer, these studies are not accepted by everyone, and most experts agree that you should stop smoking.

Supplemental oxygen: As lung scarring gets worse, many patients need extra oxygen to help them go about their daily lives without getting too out of breath. You get this oxygen from a tank that you carry around with you and, in later stages of IPF, oxygen may be needed even while sleeping or resting. Oxygen is not addictive, so you do not have to worry about using it too much. To help maintain your oxygen levels, ask your doctor about a small, easy-to-use device called a pulse oximeter. This device helps you to know just how much oxygen-flow you require, especially during activity.

Exercise: Regular exercise can help patients with IPF. Staying in shape not only keeps your breathing muscles strong, it also gives you more energy; this is because healthy muscles need less oxygen to perform.

Nutrition

Many patients with IPF lose weight because of their disease. If you lose too much weight, your breathing muscles can become weak and you also may not be able to fight off infections very well. A well-balanced diet is important to keep up your strength, but be wary of supplements and other nutrition treatments that claim to improve IPF; it's best to consult a doctor first.

Questions

Idiopathic Pulmonary Fibrosis (IPF)

15. In IPF, patients

A. will have lung cancer
B. B will have difficulty in inhalation or exhalation
C. will find it difficult to move
D. require less oxygen

16. Scar tissue develops
A. when oxygen supplied is stopped
B. when the lungs do not function properly
C. when the lungs try to repair the damage done
D. when there is more oxygen supply

17. Major symptoms of IPF are
A. fatigue and weakness
B. chest pain and breathing
C. breathing problems and coughing
D. breathing problems and weakness

18. The cause of lung scaring is
A. still not known completely
B. known
C. allergies
D. D some of the common heart diseases

19. One of the simple IPF tests is
A. bronchoscopy
B. blood test
C. CT scan
D. breathing test to identify how well your lungs work

20. For lung scarring
A. no medication is available
B. medication is available
C. prevention is better
D. not given

21. Cessation means
A. to continue
B. to cease
C. to adopt
D. to gain

22. A patient with lung scarring
A. requires oxygen supply
B. should eat a healthy diet
C. should stop smoking
D. none of the above

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ANSWERS

1. B
2. C
3. B
4. D
5. A
6. D
7. D
8. Maintain immune system
9. Not near highways
10. At the time of examinations
11. Ingestion
12. HEPA vaccum cleaner
13. Neutrophils
14. KOALA
15. Gamma - tocopherol and alpha - tocopherol
16. Urine and saliva
17. Indoor humidity
18. Canola, soybean and corn oils
19. Neutrophils
20. Gamma - tocopherol and alpha - tocopherol

PART B

1. A
2. A
3. B
4. C
5. A

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6. A

PART C

7. A

8.C

9. B

10. C

11. C

12. B

13. C

14. B

EXTRACT 2

15. B

16. C

17. C

18. A

19. D

20. A

21. B

22. A / C

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NEONATAL JAUNDICE

TEXT A

Martin and Kim were both twenty-five when they had Michael, their first child. Kim remained very healthy during her pregnancy and went into labor at 9:00 a.m., just 3 days after her due date. Delivery went quite smoothly, and that evening, mother and child rested comfortably. Two days later, Kim and Michael were released from the hospital. That evening at feeding time, Kim noticed that the whites of Michael's eyes seemed just slightly yellow, a condition that worsened noticeably by the next morning. Kim called the pediatrician and made an appointment for that morning.

Upon examining Michael, the pediatrician informed Martin and Kim that the infant had neonatal jaundice, a condition quite common in newborns and one that need not cause them too much concern. The physician explained that neonatal jaundice was the result of the normal destruction of old or worn fetal red blood cells and the inability of the newborn's liver to effectively process bilirubin, a chemical produced when red blood cells are destroyed. The physician told the parents he would like to see Michael every other day in order to monitor blood bilirubin concentration until the bilirubin concentration dropped into the normal range. He recommended that Kim feed Michael frequently and instructed them to place Michael in sunlight whenever possible.

Neonatal jaundice in a disorder that affects nearly 50% of all newborns to at least a small degree. The yellow coloration of the skin and sclera of the eyes is due to the accumulation of bilirubin in adipose tissue and its adherence to collagen fibers. In neonatal jaundice, the excess bilirubin is not due to an abnormal level of red blood cell destruction. It is due to the inability of the young liver cells to conjugate bilirubin, or make it soluble in bile, so that it can be excreted and removed from the body by the digestive tract. This inability is corrected, usually within one week, as the liver cells synthesize the conjugation enzymes. If uncorrected, sufficiently high bilirubin concentrations can cause brain damage. Frequent feedings of a newborn with jaundice increase gastrointestinal tract motility and decrease the likelihood of reabsorbing significant amounts of bilirubin in the small intestine. Radiation from sunlight alters the chemical form of bilirubin, making is easier for the liver to excrete.

TEXT B

INCIDENCE/PREVALENCE

Jaundice is the most common condition requiring medical attention in newborn infants. About 50 percent of term and 80 percent of preterm infants develop jaundice in the first week of life.1Jaundice also is a common cause of readmission to the hospital after early discharge of newborn infants.2 Jaundice usually appears two to four days after birth and disappears one to two weeks later, usually without the need for treatment.

ETIOLOGY/RISK FACTORS

In most infants with jaundice, there is no underlying disease and the jaundice is termed physiologic. Physiologic jaundice occurs when there is accumulation of unconjugated bilirubin in the skin and mucous membranes. It typically presents on the second or third day of life and results from the increased production of bilirubin (caused by increased circulating red cell mass and a shortened red cell lifespan) and the decreased excretion of bilirubin (caused by low concentrations of the hepatocyte binding protein, low activity of glucuronyl transferase, and increased enterohepatic circulation) that normally occur in newborn infants. In some infants, unconjugated hyperbilirubinemia may be associated with breastfeeding (breast milk jaundice), and this typically occurs after the third day of life. Although the exact cause of breast milk jaundice is not clear, it is believed to be caused by an unidentified factor in breast milk. Nonphysiologic causes include blood group incompatibility (Rhesus or ABO problems), other causes of hemolysis, sepsis, bruising, and metabolic disorders. Gilbert's and Crigler-Najjar syndromes are rare causes of neonatal jaundice.

PROGNOSIS

In the newborn infant, unconjugated bilirubin can penetrate the blood–brain barrier and is potentially neurotoxic. Unconjugated hyperbilirubinemia can, therefore, result in neurodevelopmental sequelae including the development of kernicterus. Kernicterus is brain damage arising from the deposition of bilirubin in brain tissue. However, the exact level of bilirubin that is neurotoxic is unclear, and kernicterus at autopsy has been reported in infants in the absence of markedly elevated levels of bilirubin.3 Recent reports suggest a resurgence of kernicterus in countries in which this complication had virtually disappeared.4 This has been attributed primarily to early discharge of newborns from the hospital.

TEXT C

Neonatal jaundice is one of the most common conditions needing medical attention in newborn babies. About 60% of term and 80% of preterm babies develop jaundice in the first week of life, and about 10% of breast fed babies are still jaundiced at age 1 month.1 Neonatal jaundice is generally harmless, but high concentrations of unconjugated bilirubin may occasionally cause kernicterus (permanent brain damage). This is a rare condition (about seven new cases each year in the United Kingdom2) and sequelae include choreoathetoid cerebral palsy, deafness, and upgaze palsy. Jaundice can also be a sign of serious liver disease, such as biliary atresia, the prognosis for which is better if it is treated before age 6 weeks.3 Early recognition of jaundice is vital for treatment of any underlying condition and for the appropriate use of phototherapy, which can safely control bilirubin concentrations in most cases.

Information for parents and carers

Offer parents or carers information about neonatal jaundice that is tailored to their needs and expressed concerns, taking care to avoid causing unnecessary anxiety; discuss verbally and back up the discussions with written information. [Based on low quality qualitative studies and on the experience and opinion of the Guideline Development Group (GDG)]

The information might include the following:

- The fact that neonatal jaundice is common, and reassurance that it is usually transient and harmless
- Risk factors for significant hyperbilirubinaemia (a rise in the serum bilirubin concentration to a level requiring treatment)
- How to check the baby for jaundice
- What to do if they suspect jaundice
- The importance of recognising jaundice in the first 24 hours and of seeking urgent medical advice
- The importance of checking the baby's nappies for dark urine or pale, chalky stools
- Reassurance that breast feeding can usually continue.

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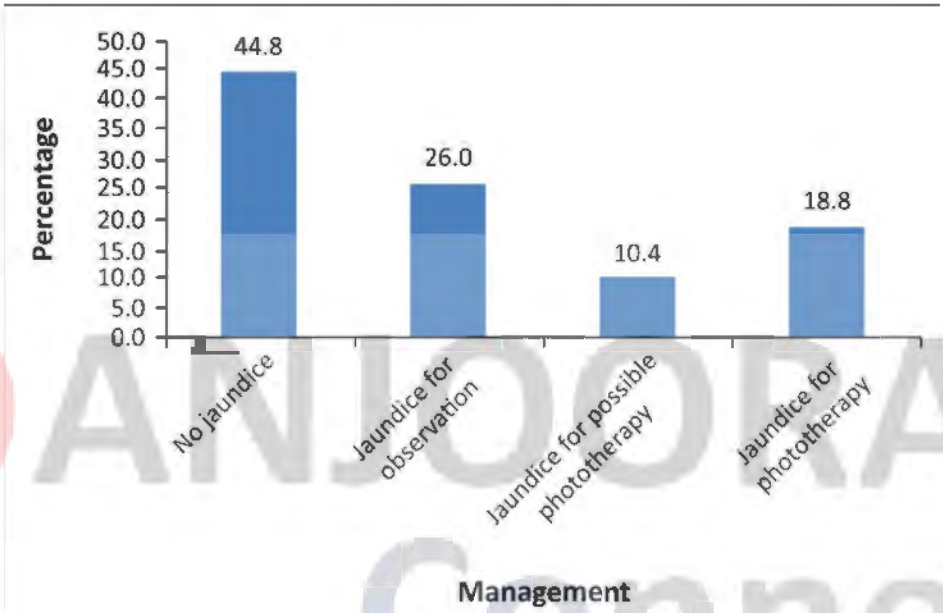
TEXT D

Prevalence of neonatal jaundice

The prevalence of neonatal jaundice, using the NICE guideline cut-off values, was 55.2% (n = 53). In Figure, the percentage of neonates per management category is depicted. No neonate needed an exchange transfusion. Only nine (17%) of the 53 infants diagnosed with jaundice appeared clinically jaundiced, of whom four were black infants.

FIGURE

Percentage of neonates per management category (n = 96).



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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

NEONATAL JAUNDICE

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. Jaundice occurs due to the destructions of fetus RBCs
2. Physiological and now-physiological causes of jaundice
3. Exchange transfusions is not necessary in all cases of neonatal jaundice.
4. Complications of neonatal jaundice
5. Neonatal jaundice is usually non-dangerous
6. Sclera of the eye turned to be yellow in neonatal jaundice babies
7. Dark urine and chalky stools are also the signs of neonatal jaundice

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Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. Neonatal Jaundice is more prevalent in which group of babies?

9. What is the main complications of neonatal jaundice?

10. What is the most easy and convenient treatment method of neonatal jaundice?

11. Mention two areas of the body where yellowish discoloration is evident in neonatal jaundice?

12. Which syndromes are the rare causes of neonatal jaundice?

13. What are the two types of neonatal jaundice?

14. What is the terminology of permanent medical damage?

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Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

15. Inability of the _____ to conjugate bilirubin results in excess accumulation of bilirubin in Adipose tissue

16. _____ occurs when there is collection of un conjugated bilirubin in the slim and mucus membrane

17. _____ of jaundice is vital for managing neonatal jaundice.

18. Metabolic disorders are one of the _____ of neonatal jaundice.

19. _____ Cerebral palsy is the consequence of neonatal jaundice in some cases

20. jaundice usually occurs ___ days after birth of term or preterm babies.

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

Mediastinoscopy, medical examination of the mediastinum (the region between the lungs and behind the sternum, or breastbone) using a lighted instrument known as a mediastinoscope. Because the region of the mediastinum contains the heart, trachea, esophagus, and thymus gland, as well as a set of lymph nodes, mediastinoscopy can be used to evaluate and diagnose a variety of thoracic diseases, including tuberculosis and sarcoidosis (a disease characterized by the formation of small grainy lumps within tissues). It fulfills an especially important role in the detection and diagnosis of cancers affecting the thoracic cavity, serving as one of the primary methods by which tissue samples are collected from the mediastinal lymph nodes for the staging of lung cancer; staging involves the investigation of cells to assess the degree to which cancer has spread. Mediastinoscopy is also frequently used in conjunction with noninvasive cancer-detection techniques, including computerized axial tomography (CAT) and positron emission tomography(PET).

1. Mediastinoscopy is _____

A. To detect thoracic diseases

B. To find out information or abnormal growth

C. To detect carcinoma

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Percussion

Percussion, in medicine, diagnostic procedure that entails striking the body directly or indirectly with short, sharp taps of a finger or, rarely, a hammer. The procedure was first described in 1761 by the Austrian physician Leopold Auenbrugger von Auenbrugg. Although generally ignored by his contemporaries, it is now routinely employed. The sounds produced by the procedure are helpful in determining the size and position of various internal organs, in revealing the presence of fluid or air in the chest, and in aiding in the diagnosis of certain lung disorders.

2.The sounds in percussion are_____

A. Determine various directions of internal organs

B. Rule out water and air in the thorax

C. Help to find out lung problems

Rubin's test

Rubin's test, diagnostic method for determining whether the fallopian tubes in the human female are occluded. (The fallopian tubes are slender hollow structures on each side of the uterus through which the eggs travel from the ovaries to the uterus.) The test is helpful in explaining certain instances of female infertility. It consists of introducing carbon dioxide into the uterus and through the fallopian tubes. The gas escapes into the abdominal cavity if the tubes are not occluded (positive test), causing referred shoulder pain. The abdominal gas may also be demonstrated by X-ray or fluoroscopy. The insufflation is usually carried out at a gas pressure of less than 120 mm of mercury. The manometer reading decreases to 100 or less if the tubes are clear; if between 120 and 130, there is probably partial stricture; if it rises to 200 and above, it is suggestive that the tubes are obstructed.

3.In Rubins test, the insufflation is carried out at a pressure of _____

A. Lessthan 120 mm of water

B. 115 mm of Hg

C. 120 mm of Hg

Tumor Segmentation

Researchers from China have used deep learning for segmenting brain tumors in MR images, where it provided more stable results as compared to manually segmenting the brain tumors by physicians, which is prone to motion and vision errors. A team led by Dr. Qi Zhang of Shanghai University found that deep learning can accurately differentiate between benign and malignant breast tumors on ultrasound shear-wave elastography (SWE), yielding more than 93% accuracy on the elastogram images of more than 200 patients

4. Dr. Qizhany's team found that

A. Deep learning can help elastography

B. Deep learning will help to differentiate benign and malignant breast tumors

C. 93 % accuracy found in elastogram

Histopathologic Cancer Diagnosis

With the advent of personalized medicine, diagnostic protocols need to focus equally on efficiency and accuracy, thus increasing the workload and complexity of histopathologic (microscopic examination of tissue in order to study the manifestations of disease) in cancer diagnosis. This has led researchers in the Netherlands to use deep learning to improve the efficiency of histopathologic slide analysis, where the workload for pathologists is reduced and the objectivity of diagnoses is increased. The researchers concluded that deep learning could improve the efficacy of prostate cancer diagnosis and breast cancer staging.

In another case, 'Philips and LabPON are planning to create the worlds largest pathology database of annotated tissue images for deep learning. One of the things the database will provide is data for research and discovery to develop new insights in disease assessment, including cancer.

In an effort to accelerate cancer research, Oak Ridge National Laboratory (ORNL) researchers are applying deep learning toward automating information collected from cancer pathology reports that are documented across a nationwide network of cancer registry programs.

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5. When did the objectivity of diagnosis increases

A. Histopathologic slide analysis's efficiency improved

B. The usage of deep learning by philippian researches so that, the histopathologic slide analysis

efficiency improved.

C. Netherland researcher's deep learning usage effectively improved histopathological

analysis's efficiency.

Tracking Tumor Development

Deep learning can be used to measure the size of tumors undergoing treatment and detect new metastases that might be overlooked. This is exactly what researchers from the Fraunhofer Institute for Medical Image Computing in Germany are doing. The more patient CT and MRI scans the deep learning algorithm reads, the more accurate it becomes, which is the core of deep learning technology.

Google Research is also hard at work developing deep learning tools that can "naturally complement pathologists' workflow." They used images to train their deep learning algorithm Inception (aka GoogLeNet) to identify breast cancer tumors that have spread to adjacent lymph nodes. The algorithm reached a localization score of 89%, exceeding the 73% accuracy rate for pathologists.

6. The researcher's used deep learning algorithm inception for

A. To identify the breast cancer tumors that spread to the lymphnodes

B. Identify the spread of tumors to adjacent lymphnodes

C. The breast cancer tumors have spread to adjacent lymphnodes

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PART C

In this part of the test, there is a text about different aspects of healthcare.

For questions 7-22,

choose the answer (A, B, C or D) which you think fits best according to the text.

Cancer and what is cervical cancer?

What is cancer? The body is made up of trillions of living cells. Normal body cells grow, divide, and die in an orderly fashion. During the early years of a person's life, normal cells divide faster to allow the person to grow. After the person becomes an adult, most cells divide only to replace worn-out or dying cells or to repair injuries.

Cancer begins when cells in a part of the body start to grow out of control. There are many kinds of cancer, but they all start because of out-of-control growth of abnormal cells. Cells

become cancer cells because of damage to DNA. DNA is in every cell and directs all its actions. In a normal cell, when DNA gets damaged the cell either repairs the damage or the cell dies. In cancer cells, the damaged DNA is not repaired, but the cell doesn't die like it should.

Instead, this cell goes on making new cells that the body does not need. These new cells will all have the same damaged DNA as the first cell does.

No matter where a cancer may spread, it is always named after the place where it started. For example, breast cancer that has spread to the liver is still called breast cancer, not liver cancer. Likewise, prostate cancer that has spread to the bone is metastatic prostate cancer, not bone cancer.

Different types of cancer can behave very differently. For example, lung cancer and breast cancer are very different diseases. They grow at different rates and respond to different treatments. That is why people with cancer need treatment that is aimed at their particular kind of cancer. Not all tumors are cancerous. Tumors that aren't cancer are called benign. Benign tumors can cause problems – they can grow very large and press on healthy organs and tissues. But they cannot grow into (invade) other tissues. Because they can't invade, they also can't spread to other parts of the body (metastasize). These tumors are almost never life threatening.

What is cervical cancer? The cervix is the lower part of the uterus (womb). It is sometimes called the uterine cervix. The body of the uterus (the upper part) is where a baby grows. The cervix connects the body of the uterus to the vagina (birth canal). The part of the cervix closest to the body of the uterus is called the endocervix. The part next to the vagina is the exocervix (or ectocervix). The 2 main types of cells covering the cervix are squamous cells (on the exocervix) and glandular cells (on the endocervix). The place where these 2 cell types meet is called the transformation zone. Most cervical cancers start in the transformation zone.

Most cervical cancers begin in the cells lining the cervix. These cells do not suddenly change into cancer. Instead, the normal cells of the cervix first gradually develop pre-cancerous changes that turn into cancer. Doctors use several terms to describe these pre-cancerous changes, including cervical intraepithelial neoplasia (CIN), squamous intraepithelial lesion (SIL), and dysplasia. These changes can be detected by the Pap test and treated to prevent the development of cancer.

Cervical cancers and cervical pre-cancers are classified by how they look under a microscope. There are 2 main types of cervical cancers: squamous cell carcinoma and adenocarcinoma. About 80% to 90% of cervical cancers are squamous cell carcinomas. These cancers are from the squamous cells that cover the surface of the exocervix. Under the microscope, this type of cancer is made up of cells that are like squamous cells. Squamous cell carcinomas most commonly begins where the exocervix joins the endocervix.

Most of the other cervical cancers are adenocarcinomas. Cervical adenocarcinomas seem to have become more common in the past 20 to 30 years. Cervical adenocarcinoma develops from the mucus-producing gland cells of the endocervix. Less commonly, cervical cancers have features of both squamous cell carcinomas and adenocarcinomas. These are called adenosquamous carcinomas or mixed carcinomas.

Although cervical cancers start from cells with pre-cancerous changes (pre-cancers), only some of the women with pre-cancers of the cervix will develop cancer. The change from cervical pre-cancer to cervical cancer usually takes several years, but it can happen in less than a year. For most women, pre-cancerous cells will go away without any treatment. Still, in some women pre-cancers turn into true (invasive) cancers. Treating all pre-cancers can prevent almost all true cancers.

Questions
An introduction on to Cancer

7. When a person becomes an adult, the cells divide only to .....
   A. replace old cells
   B. replace dying cells
   C. repair injuries
   D. all of the above

8. In cancer cells
   A. DNA is damaged (but it can be repaired)
   B. DNA is not totally damaged
   C. DNA is permanently damaged, but the cell doesn't die
   D. DNA is damaged but the cell doesn't die, instead, gives new cells, containing damaged DNA

9. Breast cancer that has spread to the liver is called
   A. liver cancer
   B. breast cancer
   C. prostate cancer
   D. bone cancer

10. Benign tumors
    A. can spread to the other parts of the body
    B. can not spread to the other parts of the body
    C. are cancerous
    D. are highly fatal

11. Most cervical cancers start
    A. in squamous cells
    B. in glandular cells
    C. in the transformation zone
    D. all of the above
    E.

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12. There are ..... main types of cervical cancers
    A. two
    B. three
    C. four
    D. five

13. About 80% to 90% of cervical cancers are .....
    A. adenocarcinoma
    B. adenosquamous carcinomas
    C. mixed carcinomas
    D. squamous cell carcinomas

14. The change from cervical pre-cancer to cervical cancer usually takes .....
    A. two years
    B. three years
    C. four years
    D. several years (can't say)

Hepatitis – Viral Liver Infection

Hepatitis (A, B, or C) can be caused by a virus (Viral Hepatitis), drugs, alcohol, medications, and blood transfusions. Scientists estimate that between 3.5 and 5.3 million people in the USA are living with Hepatitis. A blood test is required to diagnose Hepatitis infection.

Hepatitis A is a viral liver infection. In most cases the body easily defeats the virus (much like the flu, which is what you may feel like you have). Because of this it does not lead to long term liver challenges. Hepatitis A is the most common form of Hepatitis. It is spread through the feces of a contaminated person. This can easily be prevented by thoroughly washing hands after using the restroom, before eating, and after changing a diaper.

Eating raw oysters and undercooked clams can increase your chances of contracting the virus. If you are traveling in a country where Hepatitis is common make sure you wash your hands often and well, eat cooked oysters and clams, and use an antiviral essential oil such as Lemon to help protect yourself.

Hepatitis B is a viral liver infection. Again, most adult bodies are able to fight off the virus. In this case, it is referred to as Acute (something that does not last long) Hepatitis B. Hepatitis B is spread through contact with blood or bodily fluids of an infected person. This can include unprotected sexual intercourse, sharing drug needles, getting a tattoo with instruments that were not properly cleaned, or by sharing a personal item such as a razor or toothbrush with an infected person.

A mother who is infected can pass the virus on to her baby during delivery. Again, the symptoms are flu-like in nature so it often goes undiagnosed. A person who has Chronic (lasting three months or more) Hepatitis B may show no symptoms until liver damage has

occurred. Hepatitis B can lead to liver damage or cancer; your doctor may want to do a biopsy to determine the amount of damage your liver has experienced.

Hepatitis C is also a viral liver infection. A few people will contract Hepatitis C and get better. This is called Acute Hepatitis C. Most, however, will develop Chronic Hepatitis C and go on to deal with liver damage, cirrhosis of the liver, liver cancer, and possibly liver failure. Hepatitis C is the number one reason for liver transplants in the USA.

Hepatitis C is spread through contact with contaminated blood. This can occur by sharing a needle, receiving a blood transfusion or organ transplant (blood and organs have been screened for Hepatitis in the USA since 1992), getting a tattoo with equipment that has not been properly cleaned, and, in rare cases, a mother can pass the virus on to her baby during birth. Scientists are not sure, but think there may be a slim possibility that the virus may be passed through unprotected sexual intercourse.

Symptoms generally do not occur until the virus is causing damage. Again, the symptoms are flu-like; you may also experience jaundice (yellowish eyes and skin) after the flu-like symptoms go away. Most people discover they are infected by having routine tests done or by donating blood or organs and the standard tests show the infection. There is also a home test you can purchase and do if you suspect you are infected.

Home test:

If you are infected with a Hepatitis virus, or if you have been in the past, one of the most important things that you can do is strengthen your liver. The easiest way to do this is the Be Young Liver Cleanse: in the morning, take 1 drop of Be Young Lemon essential oil, 1 drop of Be Young Peppermint essential oil, and 1 teaspoon to 1 tablespoon of fresh lemon juice, followed by a glass of water.

Be Young essential oils are absolutely 100% pure, EOBBD tested and guaranteed to be free of synthetics and extenders. Do not try this with an essential oil that you are not certain has been properly cared for and tested as you do not want to increase challenges to your liver. When properly supported, the liver has a remarkable capacity for regeneration.

Hepatitis – Viral Liver Infection

15. Hepatitis is caused by

A. virus

B. alcohol consumption

C. medications

D. all of the above

16. This spreads through feces of contaminated person

A. hepatitis A

B. hepatitis B

C. both

D. can't say

17. Most adult bodies are able to fight off this virus

A. hepatitis A virus

B. hepatitis B virus

C. both

D. can't say

18. Hepatitis can lead to

A. cancer

B. more damage to the cells of liver

C. cancerous growth in liver

D. not given

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19. In the USA people go for liver transplantation because of

A. hepatitis A

B. hepatitis B

C. hepatitis C

D. all of the above

20. Hepatitis C spreads through

A. sharing needles

B. blood transfusion

C. organ transplantation

D. all of the above

21. A patient may experience jaundice when

A. the flu-like symptoms appear

B. the flu-like symptoms disappear

C. eyes become yellow

D. all of the above

22. Eating raw oysters and undercooked clams can increase your chances of contracting which virus?

A. hepatitis virus A

B. hepatitis virus B

C. hepatitis virus C

D. can't say

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ANSWERS

Part A

1 A

2 B

3 D

4 C

5 C

6 A

7 C

8 Preterm (Babies)

9 Kernicterus

10frequent feedings/radiation from sunlight

11 skin & sclera

12 Gilbert's & crigler-Najjar

13 Physiologic / Non physiologic

14 kernicterus

15 young liver cells

16 Physiologic jaundice

17 early recognition

18 Non physiologic causes

19 Choreoathetoid

20 2 to 4

Part B

1 A

2 C

3 B

4 B

5 C

6 C

PART C

7. D
8. D
9. B
10. B

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11.C
12.A
13.D
14.D
15.D
16.A
17.B
18.A
19.C
20.D
21.B
22.A

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Bell's palsy

TEXT A

A patient with Bell's palsy on the right side of his face, with the muscles on this side appearing to be paralyzed.

Bell's palsy involves a weakness or paralysis on one side of the face. Symptoms often appear first thing one morning. A person wakes up and finds that one side of their face does not move.

The person may find that they suddenly cannot control their facial muscles, usually on one side. The affected side of the face tends to droop. The weakness may also affect saliva and tear production, and the sense of taste.

Many people are afraid they are having a stroke, , but if the weakness or paralysis only affects the face, it is more likely to be Bell's palsy.

Approximately 1 in 5,000 people develop Bell's palsy each year. It is classed as a relatively rare condition.

In very rare cases, Bell's palsy can affect both sides of the face

TEXT B

The facial nerve controls most of the muscles in the face and parts of the ear. The facial nerve goes through a narrow gap of bone from the brain to the face.

If the facial nerve is inflamed, it will press against the cheekbone or may pinch in the narrow gap. This can result in damage to the protective covering of the nerve.

If the protective covering of the nerve becomes damaged, the signals that travel from the brain to the muscles in the face may not be transmitted properly, leading to weakened or paralyzed facial muscles. This is Bell's palsy.

The exact reason why this happens is unclear.

It may result when a virus, usually the herpes virus, inflames the nerve. This is the same virus that causes cold sores and genital herpes.

Other viruses that have been linked to Bell's palsy include:

Chickenpox

and shingles virus , coldsores and genital herpes virus, Epstein-Barr virus, or EBV, responsible for mononucleosis, cytomegalovirus, mumps virus

Influenza B, hand-foot-and-mouth disease (coxsackievirus)

Bell's palsy risk factors

Women who are in the last trimester of their pregnancy or who have just given birth may be at risk from Bell's palsy.

Some risk factors have been established.

Links have been found between migraine and facial and limb weakness. A study carried out in 2015 found that people with migraine may have a higher risk of bell's palsy.

The condition more commonly affects:

people aged 15 to 60 years, those with diabetes or upper respiratory diseases,

women during pregnancy, especially in the third trimester, women who gave birth less than 1 week ago.
Bell's palsy affects men and women equally.

TEXT C

Treatment
Most people will recover from Bell's palsy in 1-2 months, especially those who still have some degree of movement in their facial muscles.
Treatment with a hormone called prednisolone can speed up recovery. Astudy found that prednisolone, if administered within 72 hours of onset, can significantly reduce symptom severity and incidence after 12 months.
Prednisolone
This steroid reduces inflammation. This helps accelerate the recovery of the affected nerve. Prednisolone prevents the release of substances in the body that cause inflammation, such as prostaglandins and leukotrienes.
Patients take it by mouth, usually two tablets a day for 10 days.
Possible side effects include:
abdominal pain, bloating, acne, difficulty sleeping, dry skin, Headache , dizziness (spinning sensation), increased appetite, increased sweating
· Indigestion
· mood changes
· nausea
· Oral Thrush
· slow wound healing
· thinning skin
· tiredness
These side effects normally get better after a couple of days.
An allergic reaction to prednisolone, such as difficulty breathing, should immediately be reported to a healthcare professional.
Any allergic reaction to prednisolone should be reported to the doctor immediately.
Allergy symptoms may include:
· Hives
· breathing difficulties
· swelling of the face
· lips
· tongue
· throat
If the patient feels dizzy or drowsy they should refrain from driving or operating heavy machinery. As this symptom may not appear straight away, it is advisable to wait a day before driving or operating machinery.
Doctors usually reduce the dose gradually towards the end of the course of steroid medication. This helps prevent withdrawal symptoms, such as vomiting or tiredness.
Eye lubrication

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If the patient is not blinking properly the eye will be exposed and tears will evaporate. Some patients will experience a reduction in tear production. Both may increase the risk of damage or infection in the eye.
The doctor may prescribe artificial tears in the form of eye drops and also an ointment. The eye drops are usually taken during the waking hours, while the ointment is applied before going to sleep.
Patients who cannot close their eye properly during sleep will need to use surgical tape to keep it shut. Patients who experience worsening eye symptoms should seek medical help immediately. If you cannot get hold of your doctor, go the emergency department of your nearest hospital.
Antivirals
In some cases, an antiviral, such as acyclovir may be taken alongside prednisolone; however, evidence that they can help is weak.
Care at home
Facial exercises: As the facial nerve begins to recover, tightening and relaxing facial muscles can help strengthen them.
Dental care: If there is little or no feeling in the mouth it is easy for food to build up leading to decay or gum disease. Brushing and flossing can help prevent this.
Problems with eating: If there are difficulties with swallowing, the individual should chew food well and eat slowly. Choosing soft foods, such as yogurt can also help.
OTC pain relief: To ease any discomfort. Some pain relief medications may be purchased online, including ibuprofen and Advil.

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TEXT D

Symptoms
The facial nerves control blinking, opening and closing of the eyes, smiling, salivation, lacrimation (production of tears), and frowning. They also connect with the muscles of the stapes, a bone in the ear involved in hearing.
When the facial nerve malfunctions, as in Bell's palsy, the following symptoms can occur:

- sudden paralysis/weakness in one side of the face
- difficulty closing one of the eyelids
- irritation in the eye because it does not blink and becomes too dry
- changes in the amount of tears the eye produces
- dropping in parts of the face, such as one side of the mouth
- drooling from one side of the mouth
- difficulty with facial expressions
- sense of taste may become altered
- an affected ear may cause sensitivity to sound
- pain in front or behind the ear on the affected side
- headache

Bell's palsy

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. Tightening and facial muscle relaxation is done during recovery period
2. Excretory functions like salivation & tear production is affected in Bell's palsy
3. Sense of taste may become altered
4. Risk and damage to eye is associated with bell's palsy
5. Most of the people misunderstand bell's palsy as stroke
6. The real etiology for bell's palsy is idiopathic
7. Pathogenesis of bell's palsy.

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Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. Two over the count medications wwhich can be used at home to relieve pain?
9. At what time eye drops are generally administered?
10. Which causative agent is responsible for mononucleosis?
11. What is the main reason for irritation in the eye?
12. Which part of the nerve is affected in bell's palsy ?
13. Which age group of people are commonly affected?

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

14. Taking foods like ---------- can help in making swallowing easier.

15. ---------- can be used if patient cannot close their eye while sleeping.

16. We should be cautious regarding hypersensitivity reaction to ---------- .

17. In exceptional cases, bell's palsy affect ---------- of the face.

18. Bell's palsy can cause ---------- to sound

19. ---------- speed up rehabilitation of affected nerve.

20. People with ---------- are vulnerable to get bell's palsy.

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

Treatment of congenital fibrinogen deficiency

Afibrinogenemia is a rare bleeding disorder with an estimated prevalence of 1:1,000,000. It is an autosomal recessive disease resulting from mutations in any of the 3 genes that encode the 3 polypeptide chains of fibrinogen and are located on the long arm of chromosome 4. Spontaneous bleeding, bleeding after minor trauma and excessive bleeding during interventional procedures are the principal manifestations. We review the management of afibrinogenemia. Replacement therapy is the mainstay of treatment of bleeding episodes in these patients and plasma-derived fibrinogen concentrate is the agent of choice. Cryoprecipitate and fresh frozen plasma are alternative treatments that should be used only when fibrinogen concentrate is not available. Secondary prophylactic treatment may be considered after life-threatening bleeding whereas primary prophylactic treatment is not currently recommended. We also discuss alternative treatment options and the management of surgery, pregnancy and thrombosis in these patients. The development of new tests to identify higher risk patients and of safer replacement therapy will improve the management of afibrinogenemia in the future.

1. Fibrogenemia's treatment is

a) Replacement therapy

b) Prophylactic treatments

c) Alternative treatments.

Fibrochondrogenesis

Fibrochondrogenesis is a severe skeletal dysplasia characterized by a flat midface, short long bones, short ribs with broad metaphyses, and vertebral bodies that show distinctive hypoplastic posterior ends and rounded anterior ends, giving the vertebral bodies a pinched appearance on lateral radiographic views. The chest is small, causing perinatal respiratory problems which usually, but not always, result in lethality. Affected individuals who survive the neonatal period have high myopia, mild to moderate hearing loss, and severe skeletal dysplasia (summary by Tompson et al., 2012). For a discussion of genetic heterogeneity of fibrochondrogenesis

2. Fibrochondryogenesis is characterized by

a) Long ribs and broad metaphyses

b) Perinated respiratory problems

c) High to moderate hearing loss.

Fibromuscular Dysplasia

Renal artery stenosis, the most common cause of secondary hypertension, is predominantly caused by the atherosclerotic renovascular disease. Fibromuscular dysplasia (FMD) is a rare systemic vascular disease, affecting younger women and accounting for 10% to 20% of the cases of renal artery stenosis. FMD is an idiopathic, non-inflammatory, non-atherosclerotic disease commonly involving renal and carotid arteries; however, it can affect any arterial bed. FMD classically presents as renovascular hypertension but can also manifest as stroke in young adults. Early diagnosis and treatment are important for long-term prognosis. Etiology of FMD is unclear despite extensive research. Environmental and genetic factors have been associated with FMD. Biggazi et al. reported bilateral FMD in identical twins, raising the possibility of inheritance. In a retrospective analysis of 104 patients with renal FMD, Pannier-Moreau et al. reported an 11% prevalence of familial cases where at least one sibling showed angiographic evidence of renal FMD

3. Fibromuscular Dysplasia affect:

a) Younger Women

b) Older Women

c) Young adults

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Fibrochondrogenesis

Adult fibrosarcoma, defined by the World Health Organization as a 'malignant neoplasm composed of fibroblasts with variable collagen production and, in classical cases, a "herringbone" architecture', is a very rare soft tissue sarcoma. Once considered the most common adult sarcoma, the incidence of adult fibrosarcoma has declined dramatically over the past several decades. This is due to (i) evolution in the classification of soft tissue tumours (ii) recognition of clinically, morphologically and genetically distinctive subtypes of fibrosarcoma and (iii) increased understanding of the many other mesenchymal and non-mesenchymal tumours that may mimic fibrosarcoma. This review article will summarize the current state of our knowledge about strictly defined adult fibrosarcoma and discuss important entities in its differential diagnosis, including various fibrosarcoma variants, monophasic synovial sarcoma and other potential mesenchymal and non-mesenchymal mimics.

4. Various fibrosacroma varieties:

a) Will mimic non-mesenchymal

b) Resembles non-mesenchymal and other potential mesesnchyma

c) None of the above.

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Fibrosing alveolitis

Fibrosing alveolitis is a disease of unknown cause mainly involving the gas-exchanging portions of the lungs. It may occur in isolation and be called cryptogenic or idiopathic, in which case the clinical manifestations are mainly respiratory, or it may be associated with other disorders, such as rheumatoid arthritis. The histopathologic abnormalities of the pulmonary tissue are identical in either instance. Other names used for the disease have included usual interstitial pneumonia, desquamative interstitial pneumonia and the Hamman-Rich syndrome; these terms may describe different stages of the same pathologic process. Many authors in North America and those in the United Kingdom favour the term fibrosing alveolitis when describing chronic interstitial pneumonias. There may be accompanying nonspecific Immunologic abnormalities, which may denote that fibrosing alveolitis is part of the wide spectrum of diseases known as connective tissue disorders. Recently immune complexes have been found in the lung parenchyma; they probably result in the granulocyte destruction and reticuloendothelial proliferation seen in the acute phase of the disease.

5. Fibrosignalveoletis's cause is:

a) Unknown

b) Infection

c) Lung disease

Fibrous Dysplasia

Fibrous dysplasia (FD) is an uncommon mosaic disorder falling along a broad clinical spectrum. It arises from post-zygotic mutations in GNAS, resulting in constitutive activation of the cAMP pathway-associated G-protein, Gsα, and proliferation of undifferentiated skeletal progenitor cells. FD may occur in isolation, or in association with skin pigmentation and hyperfunctioning endocrinopathies, termed McCune-Albright syndrome (MAS). Disease may involve any part or combination of the skeleton, ranging from an isolated, asymptomatic monostotic lesion, to severe polyostotic disease resulting in fractures, deformity, functional impairment, and progressive scoliosis. FD may be diagnosed clinically in patients with polyostotic disease and/or extraskeletal features of MAS; however biopsy is typically required to diagnose monostotic disease. Management is focused on treating endocrinopathies, preventing fractures, optimizing function, and treating pain. All patients should be evaluated and treated for extraskeletal features of MAS at the time of diagnosis. In particular control of growth hormone excess is important to prevent craniofacial FD expansion, and control of FGF23-mediated hypophosphatemia is important to prevent fracture, deformity, and bone pain. A mainstay of FD treatment is surgical, and practitioners should be aware that techniques and procedures used in other skeletal disorders, such as bone grafting and prophylactic optic nerve decompression, are frequently ineffective in FD. There are currently no medical therapies capable of altering the disease course in FD. Bisphosphonates may be effective in treating FD-related bone pain, but are unlikely to impact bone quality or lesion expansion. There is a critical need to develop novel therapies capable of altering the disease activity of FD lesions. Ongoing efforts include developing drugs to target the mutant Gsα, and devising strategies for targeting mutant skeletal progenitor cells.

6. Fibrous Dysplasia is a _____ disorder:

a) Unknown mosaic

b) Functional impairment

c) Genetic

Viral Infection – Yellow Fever

Yellow fever is a viral infection spread by a particular species of mosquito. It's most common in the areas of Africa and South America, affecting both travellers to and residents of those areas. In mild cases, it causes fever, headaches, nausea and vomiting. However, it can become more serious, causing heart, liver and kidney problems along with bleeding (haemorrhaging). Up to 50 percent of people with the more severe form of yellow fever die of the disease.

There's no specific treatment for yellow fever, but getting a yellow fever vaccine before travelling to an area in which the virus is known to exist can protect you from the disease. During the first three to six days after contracting yellow fever — the incubation period — there won't be any signs or symptoms of the disease. After this, the virus enters an acute phase and, in some cases, a toxic phase follows which can be life-threatening.

Once the yellow fever virus enters the acute phase, you may experience signs and symptoms including: fever, headaches, muscle aches - particularly in your back and knees - nausea, vomiting or both, loss of appetite, dizziness, red eyes, face or tongue. These signs and symptoms usually improve and disappear within several days.

Although signs and symptoms may disappear for a day or two following the acute phase, some people with acute yellow fever then enter a toxic phase. During the toxic phase, acute signs and symptoms return and more severe and life-threatening ones also appear. These can include: yellowing of the skin and the whites of the eyes (jaundice), abdominal pain and vomiting - sometimes of blood – decreased urination, bleeding from your nose, mouth and eyes, heart dysfunction (arrhythmia), liver and kidney failure, and brain dysfunction, including delirium, seizures and coma.The toxic phase of yellow fever can be fatal.

Make an appointment to see your doctor four to six weeks before travelling to an area in which yellow fever is known to occur. If you don't have that much time to prepare, call your doctor anyway. Your doctor will help you determine whether you need vaccinations and can provide general guidance on protecting your health while abroad. Seek emergency medical care if you've recently travelled to a region where yellow fever is known to occur and you develop severe signs or symptoms of the disease. Even if you develop mild symptoms, call your doctor.Yellow fever is caused by a virus that is spread by the Aedes aegypti mosquito. These mosquitoes thrive in and near human habitations where they can breed in even the cleanest water. Most cases of yellow fever occur in sub-Saharan Africa and tropical South America.

Humans and monkeys are most commonly infected with the yellow fever virus; mosquitoes transmit the virus back and forth between monkeys, humans or both. When a mosquito bites a human or monkey infected with yellow fever, the virus enters the mosquito's bloodstream and circulates before settling in the salivary glands. When the infected mosquito bites another monkey or human, the virus then enters the host's bloodstream, where it may cause the illness to develop.

You may be at risk of the disease if you travel to an area where mosquitoes continue to carry the yellow fever virus. These areas include sub-Saharan Africa and tropical South America.

Even if there aren't current reports of infected humans in these areas, it doesn't necessarily mean you're risk-free. It's possible that local populations have been vaccinated and are protected from the disease, or that cases of yellow fever just haven't been detected and officially reported. If you're planning on travelling to these areas, you can protect yourself by getting a yellow fever vaccine at least 10 to 14 days before travelling. Anyone can be infected with the yellow fever virus, but older adults are at greater risk of becoming seriously ill.

Diagnosing yellow fever based on signs and symptoms can be difficult because, early in its course, the infection can be easily confused with malaria, typhoid, dengue fever and other viral hemorrhagic fevers. To diagnose your condition, your doctor will likely:

Ask questions about your medical and travel history

Collect a blood sample for testing

If you have yellow fever, your blood may reveal the virus itself. If not, blood tests known as enzyme-linked immuno sorbent assay (ELISA) and polymerase chain reaction (PCR) can also detect antigens and antibodies specific to the virus. Results from these tests may take several days.

No antiviral medications have proved helpful in treating yellow fever and, as a result, treatment consists primarily of supportive care in a hospital. This includes providing fluids and oxygen, maintaining adequate blood pressure, replacing blood loss, providing dialysis for kidney failure, and treating any other infections that develop. Some people receive transfusions of plasma to replace blood proteins that improve clotting. If you have yellow fever, you may also be kept away from mosquitoes, to avoid transmitting the disease to others.

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Questions

Viral Infection – Yellow Fever

7. Yellow fever is common in

A. Africa

B. South America

C. Both

D. Not given

8. Which of the following is not a sign of yellow fever?

A. Back pain

B. Vomiting

C. Nausea

D. Dry tongue

9. Signs/symptoms of toxic phase

A. Loss of appetite

B. Yellowness of eyes

C. Brain dysfunction

D. B and C

10. Seizures may occur during

A. Acute phase

B. Toxic phase

C. Sometimes in both the phases

D. Not given

11. Yellow fever, which is a viral disease, is spread by

A. Aedes agypti mosquito

B. Aedes aegypti mosquito

C. Female mosquito

D. Contamination

12. Mosquitoes transmit the virus from

A. Humans to monkeys

B. Monkeys to humans

C. Human to human

D. None

13. Being vaccinated ..... days before travelling to areas where the disease is common is recommended.

A. 10 days

B. 12 days

C. 14 days

D. 10-14 days

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14. Typhoid is

A. Similar to malaria

B. Just similar to yellow fever

C. One of common hemorrhagic fevers

D. Not given

Aortic Dissection or Dissecting Aneurysm

An aortic dissection is a serious condition in which a tear develops in the inner layer of the aorta, the large blood vessel branching off the heart. Blood surges through this tear into the

middle layer of the aorta, causing the inner and middle layers to separate (dissect). If the blood-filled channel ruptures through the outside aortic wall, aortic dissection can be fatal.

Aortic dissection, also called dissecting aneurysm, is relatively uncommon. Anyone can develop the condition, but it most frequently occurs in men between 60 and 70 years of age. Symptoms of aortic dissection may mimic those of other diseases, often leading to delays in diagnosis. However, when an aortic dissection is detected early and treated promptly, your chance of survival greatly improves.

Aortic dissection symptoms may be similar to those of other heart problems, such as a heart attack. Typical signs and symptoms include: sudden severe chest or upper back pain (often described as a tearing, ripping or shearing sensation, that radiates to the neck or down the back), loss of consciousness (fainting), shortness of breath, sweating, weaker pulse in one arm compared to the other etc.

If you have signs or symptoms such as severe chest pain, fainting, sudden onset of shortness of breath or symptoms of a stroke then seeking medical assistance is imperative. While experiencing such symptoms doesn't always mean that you have a serious problem, it's best to get checked out quickly because early detection and treatment may help to save your life.

An aortic dissection occurs in a weakened area of the aortic wall. Chronic high blood pressure may stress the aortic tissue, making it more susceptible to tearing. You can also be born with a condition associated with a weakened and enlarged aorta, such as Marfan syndrome or bicuspid aortic valve. Rarely, aortic dissections may be caused by traumatic injury to the chest area, such as during motor vehicle accidents.

Aortic dissections are divided into two groups, depending on which part of the aorta

is affected:

Type A: This is the more common and dangerous type of aortic dissection. It involves a tear in the part of the aorta just where it exits the heart or a tear extending from the upper to lower parts of the aorta, which may extend into the abdomen.

Type B: This type involves a tear in the lower aorta only, which may also extend into the abdomen.

Risk factors for aortic dissection include:

- Uncontrolled high blood pressure (hypertension), found in at least two-thirds of all cases
- Hardening of the arteries (atherosclerosis)
- Weakened and bulging artery (pre-existing aortic aneurysm)
- An aortic valve defect (bicuspid aortic valve)
- A narrowing of the aorta you're born with (aortic coarctation)

People with certain genetic diseases are more likely to have an aortic dissection than other people.

These genetic diseases include: Turner's syndrome, high blood pressure, heart problems, and a number of other health conditions may be a result of this disorder.

Marfan syndrome:

This is a condition in which connective tissue, which supports various structures in the body, is weak. People with this disorder often have a family history of aneurysms of the aorta and other blood vessels. These weak blood vessels are prone to tears (dissection) and rupture easily.

Ehlers-Danlos syndrome:

This group of connective tissue disorders is characterized by skin that bruises or tears easily, loose joints and fragile blood vessels.

Loeys-Dietz syndrome:

This is a connective tissue disorder marked by twisted arteries, especially in the neck. People who have Loeys-Dietz syndrome are thought to be at risk of developing aortic dissections and aneurysms.

An aortic dissection can lead to death, due to severe internal bleeding, including into the lining around the heart (pericardial sac), organ damage (such as kidney failure or life-threatening damage to the intestines), strokes (possibly including paralysis), and aortic valve damage, such as causing the aortic valve to leak (aortic regurgitation).

Detecting an aortic dissection can be tricky because the symptoms are similar to those of a variety of health problems. Doctors often suspect an aortic dissection if the following signs and symptoms are present: sudden tearing or ripping chest pain, widening of the aorta on a chest X-ray, blood pressure difference between the right and left arms.

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15. In aortic dissection a tear develops in

A. Outer layer of aorta

B. Inner layer of aorta

C. Middle aorta

D. A blood vessel branching off the heart

16. Dissecting aneurysm is common among

A. Men

B. Women

C. Both

D. Children

17. Symptoms of aortic dissection include

A. Chest pain and swelling

B. Weak pulse in both arms

C. Loss of consciousness

D. All of the above

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18. Aortic dissection can also be caused due to

A. High BP

B. Weak aortic wall

C. Inborn symptoms

D. Traumatic injury to chest during accidents

19. The most dangerous type of aortic dissection is

A. Type A

B. Type B

C. Aortic aneurism

D. Aortic coarctation

20. A condition in which connective tissue is weak is called

A. Turner's syndrome

B. Loeys-Dietz syndrome

C. Ehlers-Danlos syndrome

D. Marfan's syndrome

21. People with Loeys-Dietz syndrome are likely to develop

A. Aneurysms

B. Ruptured blood vessels

C. Twisted arteries in the neck

D. Aortic complications

22. Aortic dissection is

A. Extremely fatal at all times

B. Sometimes fatal

C. Not very severe

D. Sometimes severe

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ANSWERS

Part A

1.C

2.A

3.D

4.C

5.A

6.B

7.B

8.Ibuprofen and Advil

9.waking hours

10.EBV OR EPSTEIN BARR VIRUS

11.It doesn't blink

12.protective covering of the facial nerve

13.15-16 years

14.yougurt

15.surgical tape

16.prednisolone

17.both side

18.sensitivity

19.prednisolone

20.migraine

PART B

1.A

2.B

3.A

4.A

5.A

6.A

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PART C

7. C

8. D

9. B AND C

10. B

11. B

12. A /B

13. D

14. D

15. B

16. A

17. C

18. D

19. A

20. D

21.A

22.A

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Sedation: Iron deficiencies

TEXT A

Iron deficiency and iron deficiency anaemia are common. The serum ferritin level is the most useful
indicator of iron deficiency, but interpretation can be complex. Identifying the cause of iron
deficiency is crucial. Oral iron supplements are effective first-line treatment. Intravenous iron
infusions, if required, are safe, effective and practical.

Key Points

• Measurement of the serum ferritin level is the most useful diagnostic assay for detecting iron
deficiency, but interpretation may be difficult in patients with comorbidities.
• Identifying the cause of iron deficiency is crucial; referral to a gastroenterologist is often required.
• Faecal occult blood testing is not recommended in the evaluation of iron deficiency; a negative result does not impact on the diagnostic evaluation.
• Oral iron is an effective first-line treatment, and simple strategies can facilitate patient tolerance.
• For patients who cannot tolerate oral therapy or require more rapid correction of iron deficiency, intravenous iron infusions are safe, effective and practical, given the short infusion times of available formulations.
• Intramuscular iron is no longer recommended for patients of any age.

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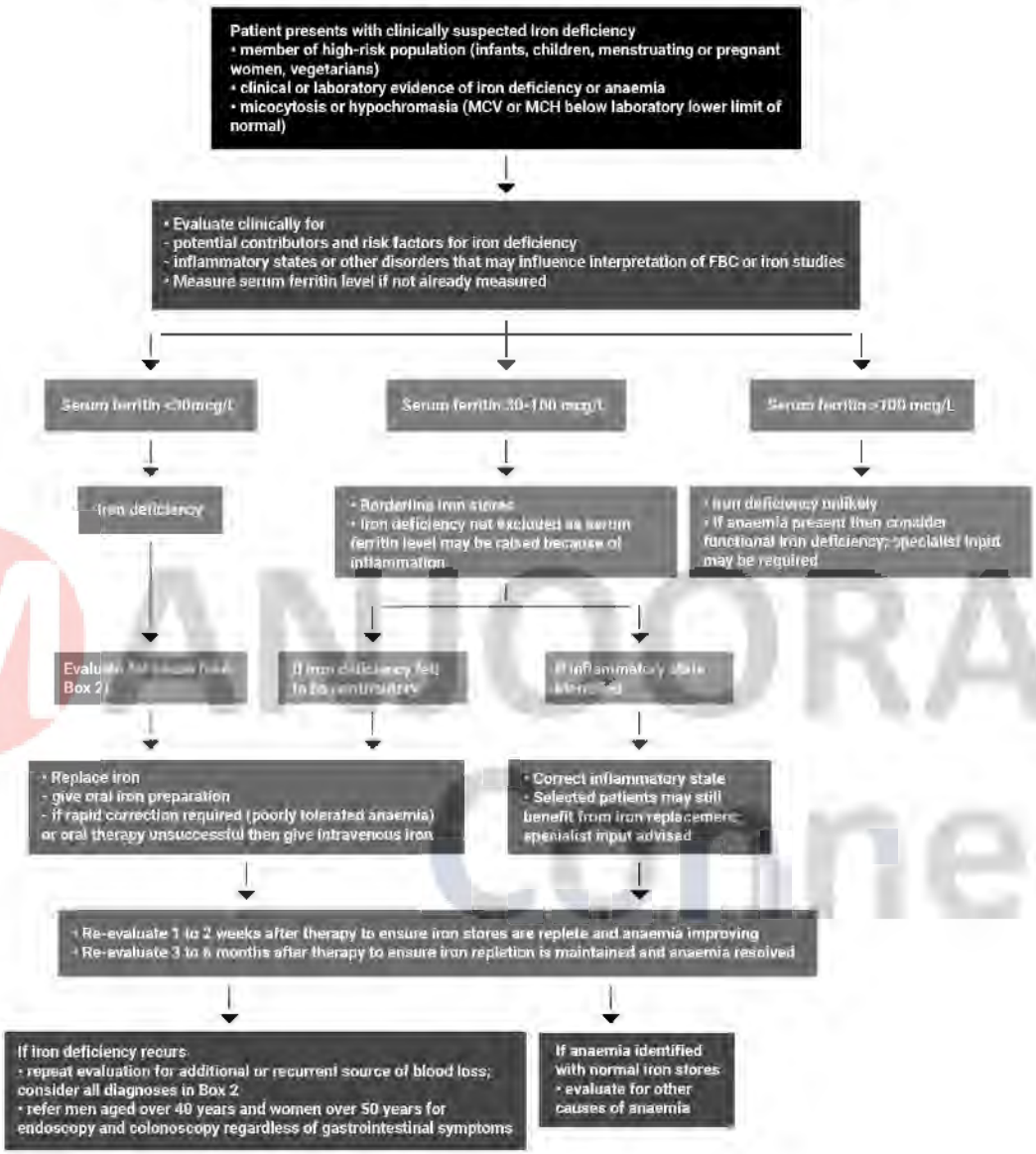
TEXT B

Treatment of infants and children

Although iron deficiency in children cannot be corrected solely by dietary change, dietary advice should be given to parents and carers. Cows' milk is low in iron compared with breast milk and infant formula, and enteropathy caused by hypersensitivity to cows' milk protein can lead to occult gastrointestinal blood loss. Excess cows' milk intake (in lieu of iron-rich solid foods) is the most common cause of iron deficiency in young children. Other risk factors for dietary iron deficiency include late introduction of or insufficient iron-rich foods, prolonged exclusive breastfeeding and early introduction of cows' milk.
Adult doses of iron can be toxic to children, and paediatric-specific protocols on iron supplementation should be followed. The usual paediatric oral iron dosage is 3 to 6mg/kg elemental iron daily. If oral iron is ineffective or not tolerated then consider other causes of
anaemia, referral to a specialist paediatrician and use of IV iron.

Text C

AN ALGORITHM FOR THE IDENTIFICATION AND MANAGEMENT OF ADULTS WITH IRON DEFICIENCY



Text D

INTRAVENOUS PREPARATIONS FOR IRON REPLACEMENT				
Form of Iron	**Presentation**	**Maximum dose per administration**	**Dosing frequency**	**Rate of administration**
Ferric carboxymaltose	500 mg/10 mL vial or 100 mg/2 mL vial	1000 mg (or 20 mg/kg)	Maximum dose once per week, or 200 mg three times per week	IV injection or infusion 100-200mg: 3 minutes 200-500 mg: 6 minutes 500-1000 mg: 15 minutes
Iron polymaltose	100 mg/2 mL ampoule	2500 mg	Not applicable as entire dose can be delivered in single administration	IV infusion: first 50 mL infused slowly (20 to 40 mL/h); if tolerated then rate can be increased to 120 mL/h*
Iron sucrose	100 mg/5 mL ampoule	100 mg	Maximum three times per week	IV infusion 100 mg over 15 minutes
*Iron polymaltose can also be administered by the intramuscular route. Different maximum doses and dosing frequencies apply.				

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Iron Deficiency:

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. considerations when treating children with iron deficiency?
2. essential steps for identifying iron deficiency?
3. evaluating iron deficiency by testing for blood in stool?
4. risk factors associated with dietary iron deficiency?
5. different types of iron solutions?
6. a treatment for iron deficiency that is no longer supported?
7. appropriate dosage when administering IV iron infusions?

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. What level of serum ferritin leads to a diagnosis of iron deficiency?
9. What is the most likely cause of iron deficiency in children?
10. Which form of iron can also be injected into the muscle?
11. What should a clinician do if iron stores are normal and anemia is still present?
12. How long after iron replacement therapy should a patient be re-tested?
13. Which form of iron is presented in a vial?
14. What is the first type of treatment iron deficient patients are typically given?

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

15. In comparison to breast milk and infant formula, cows' milk is (15) _____
16. Special procedures should be used because (16) _____ may be poisonous for children.
17. Men over 40 and women over 50 with a recurring iron deficiency should have an (17) _____
18. Iron sucrose can be given to a patient no more than (18) _____

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19. Although serum ferritin level is a good indication of deficiency, interpreting the results is sometimes difficult (19) _____

20. IV iron infusions are a safe alternative when patients are unable to (20) _____

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

Professional obligations

The Code of conduct contains guidance about the required standards of professional behavior, which apply to registered health practitioners whether they are interacting in person or online. The Code of conduct also articulates standards of professional conduct in relation to privacy and confidentiality of patient information, including when using social media. For example, posting unauthorized photographs of patients in any medium is a breach of the patient's privacy and confidentiality, including on a personal Facebook site or group, even if the privacy settings are set at the highest setting (such as for a closed, 'invisible' group).

1. The code of conduct applies to
A. doctors friending patients on Facebook.
B. privacy settings when using social media.
C. electronic and face to face communication.

General principles

Dysphagia management may be complex and is often multi-factorial in nature. The speech pathologist's understanding of human physiology is critical. The swallowing system works with the respiratory system. The respiratory system is in turn influenced by the cardiac system, and the cardiac system is affected by the renal system. Due to the physiological complexities of the human body, few clients present with dysphagia in isolation.

Complex vs. non-complex cases

Broadly the differentiation between complex and non-complex cases relates to an appreciation of client safety and reduction in risk of harm. All clinicians, including new graduates, should have sufficient skills to appropriately assess and manage noncomplex

cases. Where a complex client presents, the skills of an advanced clinician are required. Supervision and mentoring should be sought for newly graduated clinicians or those with insufficient experience to manage complex cases.

2. Why does dysphagia often require complex management?
A. Because it negatively influences the cardiac system.
B. Because it is difficult contrast complex and non-complex cases.
C. Because it seldom occurs without other symptoms.

Documentation

Every place where dental care is provided must have the following documents in either hard copy or electronic form (the latter includes guaranteed Internet access). Every working dental practitioner and all staff must have access to:
a). a manual setting out the infection control protocols and procedures used in that practice, which is based on the documents listed at sections 1.1(b), (c) and (d) of these guidelines and with reference to the concepts in current practice noted in the documents listed under References in these guidelines
b). The current Australian Dental Association Guidelines for Infection Control
(available at: http://www.ada.org.au)

3. The main point of the extract is
A. how to find documents about infection control in Australia.
B. that dental practices must have a guide for infection control.
C. that dental infection control protocols must be updated.

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Reasons for Drug-Related Problems: Manual for Geriatrics Specialists

Adverse drug effects can occur in any patient, but certain characteristics of the elderly make them more susceptible. For example, the elderly often take many drugs (polypharmacy) and have age-related changes in pharmacodynamics and pharmacokinetics; both increase the risk of adverse effects.
At any age, adverse drug effects may occur when drugs are prescribed and taken appropriately; e.g., new-onset allergic reactions are not predictable or preventable. However, adverse effects are thought to be preventable in almost 90% of cases in the elderly (compared with only 24% in younger patients). Certain drug classes are commonly involved: antipsychotics, antidepressants, and sedative-hypnotics.
In the elderly, a number of common reasons for adverse drug effects, ineffectiveness, or both are preventable. Many of these reasons involve inadequate communication with patients or between health care practitioners (particularly during health care transitions).

4. Negative effects from prescription drugs are often
A. avoidable in young people.
B. unpredictable in the elderly.

C. caused by miscommunication.

Terminology

Terminology in this guideline is a difficult issue since the choice of terminology used to distinguish groups of persons can be personal and contentious, especially when the groups represent differences in race, gender, sexual orientation, culture or other characteristics. Throughout the development of this guideline the panel endeavoured to maintain neutral and non-judgmental terminology wherever possible. Terms such as "minority", "visible minority", "non-visible minority" and "language minority" are used in some areas; when doing so the panel refers solely to their proportionate numbers within the larger population and infers no value on the term to imply less importance or less power. In some of the recommendations the term "under-represented groups" is used, again, to refer solely to the disproportionate representation of some citizens in those settings in comparison to the traditional majority.

5. The guideline tries to use terminology that
A. presents value-free information about different social groups.
B. distinguishes disadvantaged groups from the traditional majority.
C. clarifies the proportion of each race, gender and culture.

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Special needs

Special measures may be needed to ensure everyone in your client base is aware of your consumer feedback policy and is comfortable with raising their concerns. For example, should you provide brochures in a language other than English?

Some people are less likely to complain for cultural reasons. For example, some Aboriginal people may be culturally less inclined to complain, particularly to non-Aboriginal people. People with certain conditions such as hepatitis C or a mental illness, may have concerns about discrimination that will make them less likely to speak up if they are not satisfied or if something is wrong.

6. What is the purpose of this extract?
A. To illustrate situations where patients may find it difficult to give negative feedback.
B. To argue that hospital brochures should be provided in many languages.
C. To provide guidance to people who are victims of discrimination.

Part C

In this part of the test, there are two texts about different aspects of healthcare. For **questions 7-22**, choose the answer (**A**, **B**, **C** or **D**) which you think fits best according to the text.

Text 1: Difficult-to-treat depression

Depression remains a leading cause of distress and disability worldwide. In one country's survey of health and wellbeing in 2007, 7.2% of people surveyed had experienced a mood (affective) disorder in the previous 12 months. Those affected reported a mean of 11.7 disability days when they were "completely unable to carry out or had to cut down on their usual activities owing to their health" in the previous 4 weeks. There was also evidence of substantial under-treatment: amazingly only 35% of people with a mental health problem had a mental health consultation during the previous 12 months. Three-quarters of those seeking help saw a general practitioner (GP). In the 2015–16 follow-up survey, not much had changed. Again, there was evidence of substantial unmet need, and again GPs were the health professionals most likely to be providing care.

While GPs have many skills in the assessment and treatment of depression, they are often faced with people with depression who simply do not get better, despite the use of proven psychological or pharmacological therapies. GPs are well placed in one regard, as they often have a longitudinal knowledge of the patient, understand his or her circumstances, stressors and supports, and can marshal this knowledge into a coherent and comprehensive management plan. Of course, GPs should not soldier on alone if they feel the patient is not getting better.

In trying to understand what happens when GPs feel "stuck" while treating someone with depression, a qualitative study was undertaken that aimed to gauge the response of GPs to the term "difficult-to-treat depression". It was found that, while there was confusion around the exact meaning of the term, GPs could relate to it as broadly encompassing a range of individuals and presentations. More specific terms such as "treatment-resistant depression" are generally reserved for a subgroup of people with difficult-to-treat depression that has failed to respond to treatment, with particular management implications.

One scenario in which depression can be difficult to treat is in the context of physical illness. Depression is often expressed via physical symptoms, however it is also true is that people with chronic physical ailments are at high risk of depression. Functional pain syndromes where the origin and cause of the pain are unclear, are particularly tricky, as complaints of pain require the clinician to accept them as "legitimate", even if there is no obvious physical cause. The use of analgesics can create its own problems, including dependence. Patients with comorbid chronic pain and depression require careful and sensitive management and a long-term commitment from the GP to ensure consistency of care and support.

It is often difficult to tackle the topic of depression co-occurring with borderline personality disorder (BPD). People with BPD have, as part of the core disorder, a perturbation of affect associated with marked variability of mood. This can be very difficult for the patient to deal with and can feed self-injurious and other harmful behavior. Use of mentalisation-based techniques is gaining support, and psychological treatments such as dialectical behavior

therapy **form the cornerstone** of care. Use of medications tends to be secondary, and prescription needs to be judicious and carefully targeted at particular symptoms. GPs can play a very important role in helping people with BPD, but should not "go it alone", instead ensuring sufficient support for themselves as well as the patient.

Another particularly problematic and well-known form of depression is that which occurs in the context of bipolar disorder. Firm data on how best to manage bipolar depression is surprisingly lacking. It is clear that treatments such as unopposed antidepressants can make matters a lot worse, with the potential for induction of mania and mood cycle acceleration. However, certain medications (notably, some mood stabilisers and atypical antipsychotics) can alleviate much of the suffering associated with bipolar depression. Specialist psychiatric input is often required to achieve the best pharmacological approach. For people with bipolar disorder, psychological techniques and long-term planning can help prevent relapse. Family education and support is also an important consideration.

Text 1: Questions 7-14

7. In the first paragraph, what point does the writer make about the treatment of depression?
A. 75% of depression sufferers visit their GP for treatment.
B. GPs struggle to meet the needs of patients with depression.
C. Treatment for depression takes an average of 11.7 days a month.
D. Most people with depression symptoms never receive help.

8. In the second paragraph, the writer suggests that GPs
A. are in a good position to conduct long term studies on their patients.
B. lack training in the treatment and assessment of depression.
C. should seek help when treatment plans are ineffective.
D. sometimes struggle to create coherent management plans.

9. What do the results of the study described in the third paragraph suggest?
A. GPs prefer the term "treatment resistant depression" to "difficult-to-treat depression".

B. Patients with "difficult-to-treat depression" sometimes get "stuck" in treatment.
C. The term "difficult-to-treat depression" lacks a precise definition.
D. There is an identifiable sub-group of patients with "difficult-to-treat depression".

10. Paragraph 4 suggests that
A. prescribing analgesics is unadvisable when treating patients with depression.
B. the co-occurrence of depression with chronic conditions makes it harder to treat.
C. patients with depression may have undiagnosed chronic physical ailments.
D. Doctors should be more careful when accepting pain complaints as legitimate.

11. According to paragraph 5, people with BPD have
A. depression occurring as a result of the disorder
B. noticeable mood changes which are central to their disorder.

C. a tendency to have accidents and injure themselves.
D. problems tackling the topic of their depression.

12. In paragraph 5, what does the phrase ***'form the cornerstone'*** mean regarding BPD treatment?
A. Psychological therapies are generally the basis of treatment.
B. There is more evidence for using mentalisation than dialectical behavior therapy.
C. Dialectical behavior therapy is the optimum treatment for depression.
D. In some unusual cases prescribing medication is the preferred therapy.

13. In paragraph 6, what does the writer suggest about research into bipolar depression management?
A. There is enough data to establish the best way to manage bipolar depression.
B. Research hasn't provided the evidence for an ideal management plan yet.
C. A lack of patients with the condition makes it difficult to collect data on its management.
D. Too few studies have investigated the most effective ways to manage this condition.

14. In paragraph 6, what does the writer suggest about the use of medications when treating bipolar depression?
A. There is evidence for the positive and negative results of different medications.
B. Medications typically make matters worse rather than better.
C. Medication can help prevent long term relapse when combined with family education.
D. Specialist psychiatrists should prescribe medication for bipolar disorder rather than GPs.

Text 2: Are the best hospitals managed by doctors?
Doctors were once viewed as ill-prepared for leadership roles because their selection and training led them to become "heroic lone healers." However, the emphasis on patient centered care and efficiency in the delivery of clinical outcomes means that physicians are now being prepared for leadership. The Mayo Clinic is America's best hospital, according to the 2016 US News and World Report (USNWR) ranking. Cleveland Clinic comes in second. The CEOs of both — John Nose worthy and Delos "Toby" Cosgrove — are highly skilled physicians.

In fact, both institutions have been physician-led since their inception around a century ago. Might there be a general message here?

A study published in 2011 examined CEOs in the top-100 hospitals in USNWR in three key medical specialties: cancer, digestive disorders, and cardiovascular care. A simple question was asked: are hospitals ranked more highly when they are led by medically trained doctors or non-MD professional managers? The analysis showed that hospital quality scores are approximately 25% higher in physician-run hospitals than in manager-run hospitals. Of course, this does not prove that doctors make better leaders, though the results are surely consistent with that claim.

Other studies find a similar correlation. Research by Bloom, Sadun, and Van Reenen revealed how important good management practices are to hospital performance. However, they also found that it is the proportion of managers with a clinical degree that had the largest positive effect; in other words, the separation of clinical and managerial knowledge inside hospitals was associated with more negative management outcomes. Finally, support for the idea that physician-leaders are advantaged in healthcare is consistent with observations from many other sectors. Domain experts – "expert leaders" (like physicians in hospitals) — have been linked with better organizational performance in settings as diverse as universities, where scholar-leaders enhance the research output of their organizations, to basketball teams, where former All-Star players turned coaches are ***disproportionately linked*** to NBA success.

What are the attributes of physician-leaders that might account for this association with enhanced organizational performance? When asked this question, Dr. Toby Cosgrove, CEO of Cleveland Clinic, responded without hesitation, "credibility … peer-to-peer credibility." In other words, when an outstanding physician heads a major hospital, it signals that they have ***"walked the walk".*** The Mayo website notes that it is physician-led because, "This helps ensure a continued focus on our primary value, the needs of the patient come first." Having spent their careers looking through a patient-focused lens, physicians moving into executive positions might be expected to bring a patient-focused strategy.

In a recent study that matched random samples of U.S. and UK employees with employers, we found that having a boss who is an expert in the core business is associated with high levels of employee job satisfaction and low intentions of quitting. Similarly, physician-leaders may know how to raise the job satisfaction of other clinicians, thereby contributing to enhanced organizational performance. If a manager understands, through their own experience, what is needed to complete a job to the highest standard, then they may be more likely to create the right work environment, set appropriate goals and accurately evaluate others' contributions.

Finally, we might expect a highly talented physician to know what "good" looks like when hiring other physicians. Cosgrove suggests that physician-leaders are also more likely to tolerate innovative ideas like the first coronary artery bypass, performed by René Favaloro at

the Cleveland Clinic in the late '60s. Cosgrove believes that the Cleveland Clinic unlocks talent by giving safe space to people with extraordinary ideas and importantly, that leadership tolerates appropriate failure, which is a natural part of scientific endeavor and progress.

The Cleveland Clinic has also been training physicians to lead for many years. For example, a cohort-based annual course, "Leading in Health Care," began in the early 1990s and has invited nominated, high-potential physicians (and more recently nurses and administrators) to engage in 10 days of offsite training in leadership competencies which fall outside the domain of traditional medical training. Core to the curriculum is emotional intelligence (with 360-degree feedback and executive coaching), teambuilding, conflict resolution, and situational leadership. The course culminates in a team-based innovation project presented to hospital leadership. 61% of the proposed innovation projects have had a positive

institutional impact. Moreover, in ten years of follow-up after the initial course, 48% of the physician participants have been promoted to leadership positions at Cleveland Clinic.

Text 2: Questions 15-22

15. In paragraph 1, why does the writer mention the Mayo and Cleveland Clinics?
A. To highlight that they are the two highest ranked hospitals on the USNWR
B. To introduce research into hospital management based in these clinics
C. To provide examples to support the idea that doctors make good leaders
D. To reinforce the idea that doctors should become hospital CEOs

16. What is the writer's opinion about the findings of the study mentioned in paragraph 2?
A. They show quite clearly that doctors make better hospital managers.
B. They show a loose connection between doctor-leaders and better management.
C. They confirm that the top-100 hospitals on the USNWR ought to be physician-run.
D. They are inconclusive because the data is insufficient.

17. Why does the writer mention the research study in paragraph 3?
A. To contrast the findings with the study mentioned in paragraph 2
B. To provide the opposite point of view to his own position
C. To support his main argument with further evidence
D. To show that other researchers support him

18. In paragraph 3, the phrase ***'disproportionately linked'*** suggests
A. all-star coaches have a superior understanding of the game.
B. former star players become comparatively better coaches.
C. teams coached by former all-stars consistently outperform other teams.
D. to be a successful basketball coach you need to have played at a high level.

19. In the fourth paragraph, what does the phrase ***"walked the walk,"*** imply about physician leaders?
A. They have earned credibility through experience.
B. They have ascended the ranks of their workplace.

C. They appropriately incentivise employees.
D. They share the same concerns as other doctors.

20. In paragraph 6, the writer suggests that leaders promote employee satisfaction because
A they are often cooperative.
B they tend to give employees positive evaluations.
C they encourage their employees not to leave their jobs.
D they understand their employees' jobs deeply.

21. In the seventh paragraph, why is the first coronary artery bypass operation mentioned?

A. To demonstrate the achievements of the Cleveland clinic
B. To present René Favaloro as an exemplar of a 'good' doctor
C. To provide an example of an encouraging medical innovation
D. To show how failure naturally contributes to scientific progress

22. In paragraph 8, what was the outcome of the course "Leading in Health Care"?
A The Cleveland Clinic promoted almost half of the participants.
B 61% of innovation projects lead to participants being promoted.
C Some participants took up leadership roles outside the medical domain.
D A culmination of more team-based innovations.

Reading Part A: Answer Key

1 b
2 c
3 a
4 b
5 d
6 a
7 d
8 <30 mcg/L / less than 30 mcg/L / < 30 mcg / L / <30mcg/L
9 excess cow's milk / excess cow milk / excess cows' milk / excessive cow's milk / excessive cow milk / excessive cows' milk / excess cow's milk intake / excess cow milk intake / excess cows' milk intake / excessive cow's milk intake / excessive cow milk intake / excessive cows' milk intake
10 iron polymaltose
11 consider other cases / evaluate other causes / evaluate for other causes
12 1 to 2 weeks / one to two weeks / 1-2 weeks / 1 - 2 weeks

13 ferric carboxymaltose
14 oral iron / oral iron supplements
15 low in iron
16 adult doses of iron / adult iron doses
17 endoscopy and colonoscopy / colonoscopy and endoscopy
18 3 times per week / three times per week / 3 times a week / three times a week / 3 times weekly / three times weekly
19 in patients with comorbidities
20 tolerate oral iron / tolerate oral iron therapies / tolerate oral iron therapy

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part B reading Answer Key

1.c
2.c
3.b
4.c
5.c
6.a

Reading Part C: Answer Key

7	d
8	c
9	c
10	b
11	b
12	a
13	b
14	a
15	c
16	a
17	c
18	b
19	a
20	d
21	c
22	a

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Opioid dependence

TEXT A

Identifying opioid dependence The International Classification of Disease, Tenth Edition [ICD 10] is a coding system created by the World Health Organization (WHO) to catalogue and name diseases, conditions, signs and symptoms. The ICD-10 includes criteria to identify dependence. According to the ICD-10, opioid dependence is defined by the presence of three or more of the following features at any one time in the preceding year:

- a strong desire or sense of compulsion to take opioids
- difficulties in controlling opioid use
- a physiological withdrawal state
- tolerance of opioids
- progressive neglect of alternative interests or pleasures because of opioid use
- persisting with opioid use despite clear evidence of overtly harmful consequences. There are other definitions of opioid dependence or 'use disorder' (e.g. the Diagnostic and Statistical Manual of Mental Disorders, 5th edition, [DSM-5]), but the central features are the same. Loss of control over use, continuing use despite harm, craving, compulsive use, physical tolerance and dependence remain key in identifying problems.

TEXT B

Now that analgesics containing codeine are no longer available OTC (over the counter), patients may request a prescription for codeine. It is important for GPs to explain that there is a lack of evidence demonstrating the long-term analgesic efficacy of codeine in treating chronic non-cancer pain. Long-term use of opioids has not been associated with sustained improvement in function or quality of life, and there are increasing concerns about the risk of harm. GPs should explain that the risks associated with opioids include tolerance leading to dose escalation, overdose, falls, accidents and death. It should be emphasised that OTC codeine-containing analgesics were only intended for short-term use (one to three days) and that longer-term pain management requires a more detailed assessment of the patient's medical condition as well as clinical management. New trials have shown that for acute pain, nonopioid combinations can be as effective as combination analgesics containing opioids such as codeine and oxycodone. If pain isn't managed with nonopioid medications then consider referring the patient to a pain specialist or pain clinic. Patient resources for pain management are freely available online to all clinicians at websites such as:

- Pain Management Network in NSW - www.aci.health.nsw.gov.au/networks/pain-management
- Australian and New Zealand College of Anaesthetists Faculty of Pain Medicine - www.fpm.anzca.edu.au

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Text C

TABLE 2. COMPARISON OF MEDICATION APPROACHES FOR SHORT-TERM MANAGEMENT OF OPIOID WITHDRAWAL (OVER SEVEN TO 10 DAYS)[9,10]

Medication	Regulatory requirements	Requires off-label use	Strength of evidence	Advantages	Disadvantages	Notes on dosage
Buprenorphine–naloxone (sublingual)	Permit before treatment; patient registered as drug dependent in some jurisdictions	Indicated for opioid dependence (off-label for pain)	Multiple well-conducted RCTs demonstrate efficacy and safety[9]	• A large evidence base (e.g. Cochrane reviews) shows it is the most effective option for opioid taper[10]	• More restrictive than other options (regulatory requirements and supervised dosing) • Indicated only for opioid dependence, not for chronic pain in the absence of opioid dependence	• See sample withdrawal regimen (Box 8) or refer to state and national guidelines
Tramadol (oral)	Only those that apply to S4 medications	Yes (indicated for pain not opioid dependence)	Low – a small number of RCTs demonstrate efficacy and safety[9,12]	• A small number of studies examined a one-week tramadol taper, with outcomes comparable with those of other opioid tapers and superior to clonidine	• May produce serotonergic side effects, known drug interactions, use with caution in the elderly • Use is off label • Risk of seizures, even at usual doses • Variable metabolism through CYP P450, similar to codeine	• 100 to 300 mg sustained-release formulation twice a day for one week • Supervised dosing and daily dispensing may be indicated
Buprenorphine (transdermal patch)	Permit before treatment if patient is drug dependent	Yes (indicated for pain not opioid dependence)	None – no RCTs or published cases	• Good safety profile	• Dose likely to be insufficient for patients with clear evidence of opioid dependence	• 5, 10 or 20 mcg weekly patches are available • A single patch should be sufficient for taper from oral codeine
Symptomatic medications	Only those relating to S4/OTC medications	No (if use is consistent with product indication)	Moderate to high – well-conducted RCTs demonstrate efficacy; however, poorer outcomes than buprenorphine or tramadol[15]	• Fewer prescribing restrictions • Relatively safe in outpatient setting	• Shown to be less effective than buprenorphine and tramadol • Multiple medications can be confusing • Caution using sedative medications in outpatient setting • Clonidine can cause severe hypotension	• See Table 1

Abbreviations: OTC = over the counter; RCT = randomized controlled trial.

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TEXT D

Preparation for tapering As soon as a valid indication for tapering of opioid analgesics is established, it is important to have a conversation with the patient to explain the process and develop a treatment agreement. This agreement could include:

- time frame for the agreement
- objectives of the taper
- frequency of dose reduction
- requirement for obtaining the prescriptions from a designated clinician
- scheduled appointments for regular review
- anticipated effects of the taper
- consent for urine drug screening
- possible consequences of failure to comply. Before starting tapering, it needs to be clearly emphasised to the patient that reducing the dose of opioid analgesia will not necessarily equate to increased pain and that it will, in effect, lead to improved mood and functioning as well as a reduction in pain intensity. The prescriber should establish a therapeutic alliance with the patient and develop a shared and specific goal.

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Managing Opioid Dependence

Questions 1-7

For each of the questions, **1-7**, decide which text (**A, B, C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about...

1. what GPs should say to patients requesting codeine?

2. basic indications of an opioid problem?

3. different medications used for weaning patients off opioids?

4. decisions to make before beginning treatment of dependence?

5. defining features of a use disorder?

6. the development of a common goal for both prescriber and patient?

7. sources of further information on pain management?

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

8. What will reduced doses of opioids lead to a reduction of?

9. What is the most effective medication for tapering opioid dependence?

10. How long should over the counter codeine analgesics be used for?

11. When should doctors consider referring a patient to a pain expert or clinic?

12. What might a patient give permission to before starting treatment?

13. What might be increasingly neglected as a result of opioid use?

14. How many Buprenorphine patches are needed to taper from codeine tablets?

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both. Your answers should be correctly spelled.

The use of Buprenorphine-naxolone requires a **(15)** _____ before treatment.

The use of symptomatic medications for the treatment of opioid dependence has been found to have **(16)** _____ than tramadol.

Different definitions of opioid dependence share the same **(17)** _____

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Once it is decided that opioid taper is a suitable treatment the doctor and patient should create a **(18)** _____

Recent research indicates that **(19)** _____ can work as well as combination analgesics including codeine and oxycodone.

The ICD-10 defines a patient as dependent if they have **(20)** _____key symptoms simultaneously.

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

1. According to the guidelines nurse's must

a. advises the practice as soon as they get to the next home visit.

b. call the patient to confirm a time before they make a home visit.

c. informs fellow staff members when they return from a home visit.

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Home Visit Guidelines

The nurse will complete all consultation notes in the patient's home (unless not appropriate), prior to beginning the next consultation. With a focus on nurse safety, the nurse will call the practice at the end of each visit before progressing to the next home visit and will also communicate any unexpected circumstances that may delay arrival back at the practice (more than one hour).

Calling from the patient's home to make a review appointment with the GP is sufficient and can help minimise time making phone calls. On return to the practice the nurse will immediately advise staff members of their return. This time will be documented on the

patient visit list, scanned and filed by administration staff.

2. In progressive horizontal evacuation
a. patients are evacuated through fire proof barriers one floor at a time
b. patients who can't walk should not be moved until the fire is under control.
c. patients are moved to fire proof areas on the same level to safely wait for help.

Progressive horizontal evacuation

The principle of progressive horizontal evacuation is that of moving occupants from an area affected by fire through a fire-resisting barrier to an adjoining area on the same level, designed to protect the occupants from the immediate dangers of fire and smoke (a refuge). The occupants may remain there until the fire is dealt with or await further assisted onward evacuation by staff to a similar adjoining area or to the nearest stairway. Should it become necessary to evacuate an entire storey, this procedure should give sufficient time for non-ambulant and partially ambulant patients to be evacuated vertically to a place of safety.

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3. The main purpose of the extract is to
a. provide information of the legal requirements for disposing of animal waste.
b. describe rules for proper selling and export of animal products.
c. define the meaning of animal by-products for healthcare researchers.

Proper disposal of animal waste

Animal by-products from healthcare (for example research facilities) have specific legislative requirements for disposal and treatment. They are defined as "entire bodies or parts of animals or products of animal origin not intended for human consumption, including ova, embryos and semen." The Animal By-Products Regulations are designed to prevent animal by-products from presenting a risk to animal or public health through the transmission of disease. This aim is achieved by rules for the collection, transport, storage, handling, processing and use or disposal of animal byproducts, and the placing on the market, export and transit of animal by-products and certain products derived from them.

4. According to the extract, what is the outcome of reusing medical equipment meant to be used once?

a. The maker will take no legal responsibility for safety.

b. Endoscopy units will save on equipment costs.

c. There is a higher incidence of cross infection.

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Cleaning and disinfection of endoscopes should be undertaken by trained staff in a dedicated room. Thorough cleaning with detergent remains the most important and first step in the process. Automated washer/disinfectors have become an essential part of the endoscopy unit. Machines must be reliable, effective, easy to use and should prevent atmospheric pollution by the disinfectant if an irritating agent is used. Troughs of disinfectant should not be used unless containment or exhaust ventilated facilities are provided.

Whenever possible, "single use" or autoclavable accessories should be used. The risk of transfer of infection from inadequately decontaminated reusable items must be weighed against the cost. Reusing accessories labelled for single use will transfer legal liability for the

safe performance of the product from the manufacturer to the user or his/her employers and should be avoided unless Department of Health criteria are met.

5. According to the extract what is the purpose of the guidelines?
   a. To present statistics on the incidence of melanoma in Australia and New Zealand.
   b. To support the early detection of melanoma and select the best treatments.
   c. To explain the causes of melanoma in populations of Celtic origin.

Australia and New Zealand have the highest incidence of melanoma in the world. Comprehensive, up-to-date, evidence-based national guidelines for its management are therefore of great importance. Both countries have populations of predominantly Celtic origin, and in the course of day-to-day life their citizens are inevitably subjected to high levels of solar UV exposure. These two factors are considered predominantly responsible for the very high incidence of melanoma (and other skin cancers) in the two nations. In Australia, melanoma is the third most common cancer in men and the fourth most common in women, with over 13, 000 new cases and over 1, 750 deaths each year.

The purpose of evidence-based clinical guidelines for the management of any medical condition is to achieve early diagnosis whenever possible, make doctors and patients aware of the most effective treatment options, and minimise the financial burden on the health system by documenting investigations and therapies that are inappropriate.

6. What should employees declare?

a. Every item received from one donor.

b. Each item from one donor valued at over $50.

c. Every item from one donor if the combined value is more than $50.

Reporting of Gifts and Benefits

Employees must declare all non-token gifts which they are offered, regardless of whether or not those gifts are accepted. If multiple gifts, benefits or hospitality are received from the same donor by an employee and the cumulative value of these is more than $50 then each individual gift, benefit or hospitality event must be declared.

The Executive Director of Finance will be responsible for ensuring the gifts and benefits register is subject to annual review by the Audit Committee. The review should include analysis for repetitive trends or patterns which may cause concern and require corrective and preventive action. The Audit Committee will receive a report at least annually on the administration and quality control of the gifts, benefits and hospitality policy, processes and register.

MANJOORAN Connect

PART C

In this part of the test, there is a text about different aspects of healthcare. **For questions 7-22**, choose the answer (**A, B, C or D**) which you think fits best according to the text.

Text 1: The case for and against e-cigarettes

Electronic cigarettes first hit European and American markets in 2006 and 2007, and their popularity has been propelled by international trends favouring smoke-free environments. Sales reportedly have reached $650 million a year in Europe and were estimated to reach $3. 6 billion in the US in 2018.

Although research on e-cigarettes is not extensive, a picture is beginning to emerge. Surveys suggest that the vast majority of those who use e-cigarettes treat them as smoking-cessation aides and self-report that they have been key to quitting. Data also indicate that e-cigarettes help to reduce tobacco cigarette consumption. A 2011 survey, based on a cohort of first-time e-cigarette purchasers, found that 66. 8 percent reported reducing the number of cigarettes they smoked per day and after six months, 31 percent reported not smoking. These results compare favorably with nicotine replacement therapies (NRTs) like the patch and nicotine gum. Interestingly, a randomized controlled trial found that even e-cigarettes not containing nicotine were effective both in achieving a reduction of tobacco cigarette consumption and longer term abstinence, suggesting that "factors such as the rituals associated with cigarette handling and manipulation may also play an important role. " Some tobacco control advocates worry that they simply deliver an insufficient amount of nicotine to ultimately prove effective for cessation.

Nevertheless, the tobacco control community has embraced FDA approved treatments—NRTs, as well as the drugs bupropion and varenicline —that have relatively low success rates. In a commentary published in the Journal of the American Medical Association, smoking cessation experts Andrea Smith and Simon Chapman of the University of Sydney said that smoking cessation drugs fail most of those who try them. "Sadly, it remains the case that by far the most common outcome at 6 to 12 months after using such medication in real world settings is continuing smoking. Few, if any, other drugs with such records would ever be prescribed, " they wrote.

Amongst smokers not intending to quit, e-cigarettes—both with and without nicotine—substantially reduced consumption in a randomized controlled trial, not only resulting in decreased cigarette consumption but also in "enduring tobacco abstinence. " In a second study from 2013, the authors reported that after 24 months, 12. 5 percent of smokers remained abstinent while another 27. 5 percent reduced their tobacco cigarette consumption by 50 percent. Finally, a third study commissioned in Australia has come to the same conclusion, though a high dropout rate (42 percent) makes these findings questionable.

Users widely perceive e-cigarettes to be less toxic. While the FDA has found trace elements of carcinogens, levels are comparable to those found in nicotine replacement therapies. Results from a laboratory study released in 2013 found that that while e-cigarettes do contain contaminants, the levels range from 9 to 450 times lower than in tobacco cigarette smoke. **These** are comparable with the trace amounts of toxic or carcinogenic substances found in medicinal nicotine inhalers. A prominent anti-tobacco advocate, Stanton Glantz, has warned of the need to protect people from secondhand emissions. While one laboratory study indicates that passive "vaping, " as smoking an e-cigarette is commonly known, releases volatile organic compounds and ultrafine particles into the indoor environment, it noted that the actual health impact is unknown and should remain a chief concern. A 2014 study concluded that e-cigarettes are a source of second hand exposure to nicotine but not to toxins. Nevertheless, bystanders are exposed to 10 times less nicotine exposure from e-cigarettes compared to tobacco cigarettes.

There are a number of interesting points of agreement among proponents and skeptics of e-cigarettes. First, all agree that regulation to ensure the quality of e-cigarettes should be **uniform**. Laboratory analyses have found sometimes wide variation across brands, in the level of carcinogens, the presence of contaminants, and the quality of nicotine. Second, proponents and detractors of e-cigarettes tend to agree that — considered only at the individual level—e-cigarettes are a safer alternative to tobacco cigarette consumption. The main concern is how e-cigarettes might shape tobacco use patterns at the population level. Proponents stress the evidence base that we have reviewed. Skeptics remain worried that e-cigarettes will become "dual use" products. That is, smokers will use e-cigarettes, but will not reduce their smoking or quit.

Perhaps most troubling to public health officials is that e-cigarettes will "renormalize" smoking, subverting the cultural shift that has occurred over the past 50 years and transforming what has become a perverse habit into a pervasive social behaviour. In other words, the fear is that e-cigarettes will allow for re-entry of the tobacco cigarette into public view. This would unravel the gains created by smoke-free indoor (and, in some scientifically-unwarranted instances) outdoor environments. Careful epidemiological studies will be needed to determine whether the individual gains from e-cigarettes will be counteracted by population-level harms. For policy makers, the challenge is how to act in the face of uncertainty.

7. What does the writer suggest about the research into e-cigarettes?
   A. Not enough research is being carried out.
   B. Early conclusions are appearing from the evidence.
   C. Too much of the available data is self-reported.
   D. An extensive picture of e-cigarette use has emerged.

8. What explanation does the writer offer for the effect of non-nicotine e-cigarettes?
   A. They deliver an insufficient volume of nicotine to help smoking cessation.
   B. They compare well with patches, nicotine gum and other NRT's.
   C. First time e-cigarette buyers tend to use them
   D. Behavioural elements are significant in quitting smoking.

9. What is the attitude of Andrea Smith and Simon Chapman to the use of smoking cessation drugs?
A. They approve of and embrace these treatments.
B. They consider them largely unsuccessful as treatments.
C. They think they should be replaced with other treatments.
D. They believe they should never be prescribed as treatment.

10. What problem with one of the studies is mentioned in paragraph 4?
    The research questions the study asked.
    The number of participants who left the study.
    The similarity of the conclusion to other studies.
    The study used e-cigarettes without nicotine.

11. What is **"these"** in paragraph 5 referring to?
A. Laboratory study results
B. Nicotine inhalers
C. Contamination levels
D. Tobacco cigarettes

12. Research mentioned in paragraph 5 suggests that
A. E-cigarettes release dangerous toxins into the air.
B. E-cigarettes should be banned from indoor environments.
C. E-cigarettes are more toxic than nicotine replacement therapies
D. cigarettes present a far greater risk of secondhand exposure to toxins

13. The word **uniform** in paragraph 7 suggests that e-cigarettes should
A. Be clearly regulated against.
B. Only come in one brand.
C. Be of a standard quality.
D. Contain no contaminants.

14. What do both critics and supporters of e-cigarettes agree?
A. Available research evidence must be reviewed.
B. E-cigarette use may not result in quitting.
C. Smoking tobacco is more dangerous than vaping.
D. E-cigarettes are shaping the public's tobacco use.

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Extract 2:

Text 2: Vivisection

In 1875, Charles Dodgson, under his pseudonym Lewis Carroll, wrote a blistering attack on vivisection. He sent this to the governing body of Oxford University in an attempt to prevent the establishment of a physiology department. Today, despite the subsequent evolution of one of the most rigorous governmental regulatory systems in the world, little has changed. A report sponsored by the UK Royal Society, "The use of non-human primates in research", attempts to establish a sounder basis for the debate on animal research through an in-depth analysis of the scientific arguments for research on monkeys.

In the UK, no great apes have been used for research since 1986. Of the 3000 monkeys used in animal research every year, 75% are for toxicology studies by the pharmaceutical industry. Although expenditure on biomedical research has almost doubled over the past 10 years, the number of monkeys used for this purpose (about 300) has tended to fall. The report, which mainly discusses the use of monkeys in biomedical research, pays particular attention to the development of vaccines for AIDS, malaria, and tuberculosis, and to the nervous system and its disorders. The report assesses the impact of these issues on global health, together with potential approaches that might avoid the use of animals in research. Other research areas are also discussed, together with ethics, animal welfare, drug discovery, and toxicology.

The report concludes that in some cases there is a valid scientific argument for the use of monkeys in medical research. However, no blanket decisions can be made because of the speed of progress in biomedical science (particularly in molecular and cell biology) and because of the available non-invasive methods for study of the brain. Every case must be considered individually and supported by a fully informed assessment of the importance of the work and of alternatives to the use of animals.

Furthermore, the report asks for greater openness from medical and scientific journals about the amount of animal suffering that occurred in studies and for regular publication of the outcomes of animal research and toxicology studies. It calls for the development of a national strategic plan for animal research, including the dissemination of information about alternative research methods to the use of animals, and the creation of centres of excellence for better care of animals and for training of scientists. Finally, it suggests some approaches towards a better-informed public debate on the future of animal research.

Although the report was received favourably by the mass media, animal-rights groups thought that it did not go far enough in setting priorities for development of alternatives to the use of animals. In fact, it investigates many of these approaches, including cell and molecular biology, use of transgenic mice (an alternative to use of primates), computer modelling, in-silico technology, stem cells, microdosing, and pharmacometabonomic phenotyping. However, the report concludes that although many of these techniques have great promise, they are at a stage of development that is too early for assessment of their true potential.

The controversy of animal research continues unabated. Shortly after publication of the

report, two highly charged stories were published in the media. A study that used systematic reviews to compare treatment outcome from clinical trials of animals with those of human beings suggested that discordance in the results might have been due to bias, poor design, or inadequacies of animals for modelling of human disease. Although the study made some helpful suggestions for the future, its findings are not surprising. The imperfections of animals for study of human disease and of drug trials are documented widely.

The current furore about the UK Government's ban on human nuclear-transfer experiments involving animals should not surprise us either. This area of research had a bad start when this method of production of stem cells was labelled as therapeutic cloning, thus confusing it with reproductive cloning - a problem that, surely, licensing bodies and the scientific community should have anticipated. The possibilities that insufficient human eggs will be available, and that insertion of human nuclei into animal eggs might be necessary, have been discussed by the scientific community for several years, but have been aired rarely in public, leaving much room for confusion

Biomedical science is progressing so quickly that maintenance of an adequate level of public debate on ethical issues is difficult. Hopefully the sponsors of the recent report will now activate its recommendations, not least how better mechanisms can be developed to broaden and sustain interactions between science and the public. Although any form of debate will probably not satisfy the extremists of the antivivisection movement, the rest of society deserves to receive the information it needs to deal with these extremely difficult issues.

15. How does the writer characterise Lewis Carroll's attitude to vivisection?
A. He was in favour of clear regulations to control it.
B. He felt the Royal Society should not support it.
C. He was strongly opposed to it.
D. He supported its use in physiology.

16. The word **rigorous** in paragraph 1 implies that the writer thinks UK vivisection laws are
A. Strict and severe
B. Careful and thorough

C. Ambiguous and unhelpful
D. Accurate and effective

17. What is the major focus of the report mentioned in paragraph 2?
A. Animal experimentation in the pharmaceutical industry
B. Recent increases in spending on Biomedical research
C. Testing new treatments for serious disease on monkeys
D. Possible alternatives to testing new drugs on animals

18. What is the main conclusion of the report?
A. Scientific experimentation on monkeys is justified.
B. Rapid development in biomedicine makes it hard to draw conclusions.
C. Non-invasive techniques should be preferred in most cases.
D. Research that requires monkeys should be evaluated independently.

19. What conclusion is drawn about alternative techniques to vivisection?
A. Developing alternatives should be prioritised.
B. Transgenic mice are a viable alternative to monkeys.
C. Many alternative techniques are more promising than animal testing.
D. They aren't well enough understood yet to adopt for research.

20. What does the writer claim about the use of animals in medical research?
A. The limitations of using animals in research are well understood.
B. Results from too many animal trials are biased.
C. Human studies are known to be more reliable.
D. Strong media reaction has kept up the controversy.

21. The phrase **a problem** in paragraph 6 refers to the
A. Government licensing of animal experiments.
B. Confusion between the names of two different methods.
C. Chortage of human embryos available for experiments.
D. Prohibition against human nuclear transfer in the UK.

22. The author thinks it is hard to keep the public adequately informed about this research because
A. The report sponsors have not activated the recommendations.
B. Of the rapid evolution of biomedical technologies.
C. Scientists don't interact with the public enough.
D. Extreme views from opponents cloud the debate.

OET Reading Part A: Answer Key

1. B

2. A

3. C

4. D

5. A

6. D

7. B

8. pain intensity

9. Buprenorphine-naloxone / Buprenorphine - naloxone / buprenorphine-naloxone / buprenorphine - naloxone / Buprenorphine-naloxone (sublingual) / Buprenorphine - naloxone (sublingual) / buprenorphine-naloxone (sublingual) / buprenorphine - naloxone (sublingual)

10. one to three days / 1 to 3 days / 1-3 days / 1 - 3 days

11. if pain isn't managed with nonopioid medications / if pain isn't managed / if pain isn't managed with non-opioid medications

12. urine drug screening

13. alternative interests or pleasures / alternative interests and pleasures / interests or pleasures / interests and pleasures

14. a single patch / one patch / 1 patch

15. permit

16. poorer outcomes

17. central features / features

18. treatment agreement

19. nonopioid combinations / non-opioid combinations

20. three or more / at least three / 3 or more / at least 3

OET Reading Part b: Answer Key

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1.c

2.c

3.a

4. a

5.b

6.C

OET Reading Part c: Answer Key

7. b

8.d

9. b

10. d

11.c

12. d

13. c

14.c

15.c

16.b

17.c

18.d

19.d

20.a

21.b

22.b

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PART A

TIME: 15 minutes

- Look at the four texts, A-D, in the separate Text Booklet.
- For each question, 1-20, look through the texts, A-D, to find the relevant information.
- Write your answers on the spaces provided in this Question Paper.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Text A

Eczema / Dermatitis

The words `eczema` and `dermatitis` are often used synonymously to describe a polymorphic pattern of inflammation, which in the acute phase is characterized by erythema and vesiculation, and in the chronic phase by dryness, lichenification and fissuring. Contact dermatitis describes these patterns of reaction in response to external agents, which may be acting either as irritants, where the T cell-mediated immune response is not involved, or as allergens, where cell-mediated immunity is involved. Contact dermatitis may be classified into the following reaction types: Subjective irritancy ± idiosyncratic stinging and smarting reactions that occur within minutes of contact, usually on the face, in the absence of visible changes. Cosmetic or sunscreen constituents are common agents. Acute irritant contact dermatitis is often the result of a single overwhelming exposure or a few brief exposures to strong irritants or caustic agents. Chronic irritant contact dermatitis occurs following repetitive exposure to weaker irritants, which may be either `wet`, such as detergents, organic solvents, soaps, weak acids and alkalis, or `dry`, such as low humidity air, heat, powders and dust.

Text B

Patch testing

The mainstay of diagnosis in allergic contact dermatitis is the patch test. This test has a sensitivity and specificity of between 70% and 80%15. Patch testing involves the reproduction under the patch tests of allergic contact dermatitis in an individual sensitized to a particular antigen(s). The standard method involves the application of the antigen to the skin at standardized concentrations in an appropriate vehicle and under occlusion. The back is most commonly used principally for convenience because of the area available, although the limbs,

in particular the outer upper arms, are also used. Various application systems are available of which the most commonly used are Finn chambers. With this system, the investigator adds the individual allergens to test discs that are loaded on to adhesive tape. Available are available ± the TRUE and the Epiquic tests. There are few comparative studies between the different systems. Pre-prepared tests are significantly more reliable than operator-prepared tests. There is also some evidence that larger chambers may give more reproducible tests. However, this may only apply to some allergens. The open patch test, not so common, is used where potential irritants or sensitizers are being assessed. It is also useful in the investigation of contact urticaria and protein contact dermatitis. The open patch test is usually performed on the forearm but the upper outer arm or scapular areas may also be used. The site should be assessed at regular intervals for the first 30±60 min and a later reading should be carried out after 3±4 days. A repeated open application test, applying the suspect agent on to the forearm, is also useful in the assessment of cosmetics, where irritancy or combination effects may interfere with standard patch testing. This usually involves the application of the product twice daily for up to a week, stopping if a reaction develops.

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Text C

Photopatch testing

Where photoallergic dermatitis is suspected, photopatch testing may be carried out. Very briefly, the standard method of photopatch testing involves the application of the photo allergen series and any suspected materials in duplicate on either side of the upper back. One side is irradiated with ultraviolet (UV) after an interval (1 or 2 days) and readings are taken in parallel after a further 2 days. The exact intervals for irradiation and the dose of UVA given vary from centre to centre. The U.K. multicentre study into photopatch testing has now been completed and published. It is recommended that allergens be subjected to 5 J cm2 UVA and a reading to be taken after 2 days. The incidence of photoallergy in suspected cases was low at below 5%; however, further readings at 3 and 4 days increased the detection rate.

Text D

There are a number of aspects, which can have their effect on the accuracy of patch testing. Principal among these are the characteristics of the individual allergens and the method of patch testing. Some allergens are more likely to cause irritant reactions than others. These reactions may be difficult to interpret and are easily misclassified as positive reactions. Nickel,

cobalt, potassium dichromate and carba mix are the notable offenders in the standard series. As indicated above, preprepared patch tests are better standardized in terms of the amount of allergen applied and are therefore more reproducible, but are prohibitively expensive in the U.K. Patient characteristics are also important. It is essential that the skin on the back is free from dermatitis and that skin disease elsewhere is as well controlled as possible. This will help to avoid the `angry back syndrome` with numerous false positives. However, if a patient applies topical steroids to the back up to 2 days prior to the test being applied or is taking oral corticosteroids or immunosuppressant drugs, then there is a significant risk of false negative results.

Questions 1-7

For each question, 1-7, decide which text (A, B, C or D) the information comes from. You may use any letter more than once.

In which text can you find information about;

1. One of the most common tests.

Answer _____

2. The blockage or closing of a blood vessel or hollow organ.

Answer _____

3. Important factors that may influence the testing results.

Answer _____

4 .The condition that occurs in response to excessive itching or rubbing of the skin.

Answer _____

5. Two different types of tests which help in diagnosing the disease condition.

Answer _____

6 .The process or fact of irradiating or being irradiated.

Answer _____

7 .Splitting or cracking of the skin.

Answer _____

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Questions 8-14

Answer each of the questions, 8-14, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. Which test is known to be more helpful with respect to the determination of other irritants, which make it difficult to carry out other tests?

Answer _____

9. What are given orally or by injection and distribute throughout the body?

Answer _____

10. In which process do we apply the product two times?

Answer _____

11. What would be the outcome of the pre-prepared patch test, if there is dermatitis on the back of the skin?

Answer _____

12. What are the common precipitants of contact dermatitis?

Answer _____

13. What would be the outcome of the test, if the patient applies topical steroids two days prior to the test?

Answer _____

14. What was the proportion of photoallergic incidents in suspected cases?

Answer _____

Questions 15-20

Complete each of the sentences, 15-20, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

15. _____ as the cumulative irritant is known to create more worsening conditions.

16. The open patch is carried in order to examine the potentiality of the _____ or irritants.

17 .The chronic phase of a polymorphic pattern of inflammation is characterized by _____.

18. Carba mix is one of the _____ in the standard series.

19. The standard method of testing involves the application of the_____.

20. As per recommendations, allergens be subjected to _____.

PART B

In this part of the test, there are six short extracts relating to the work of health professionals. For questions 1-6, choose the answer (A, B or C) which you think fits best according to the text.

Questions 1-6

1 The notice talks about;

A How blood clots occur.

B Blood clot doesn`t form. (if the reaction doesn`t occur the way it should)

C Blocking of clotting reaction.

VKCFD

Inherited combined deficiency of the vitamin K dependent clotting factors is a very rare inherited bleeding disorder that is caused by a problem with clotting Factors II, VII, IX, and X. VKCFD can also be acquired later in life as a result of other disorders, or certain medications such as the blood-thinning drug Coumadin. Acquired VKCFD is more common than the inherited form. Some newborn babies have a temporary vitamin K deficiency, which can be treated with supplements at birth. In order to continue the chain reaction of the coagulation cascade, these four factors need to be activated in a chemical reaction that involves vitamin K. VKCFD is an autosomal recessive disorder, which means that both parents must carry the defective gene in order to pass it on to their child. It also means that the disorder affects both males and females.

2 What is correct?

A All types of Factor I deficiency affect both males and females.

B The fibrogen defect in impairment leads to disorder.

C Genes can be both recessive or dominant.

Factor I deficiency

It is an umbrella term for several related disorders in males and females, known as congenital Fibrinogen defects. Afibrinogenemia (a lack of Fibrinogen) and hypofibrinogenemia (low levels of Fibrinogen) are quantitative defects, meaning the amount of Fibrinogen in the blood

is abnormal. Dysfibrinogenemia is a qualitative defect in which Fibrinogen does not work the way it should. Hypodysfibrinogenemia is a combined defect that involves both low levels of Fibrinogen and impaired function. Afibrinogenemia is an autosomal recessive disorder, which means that both parents must carry the defective gene in order to pass it on to their child. Hypofibrinogenemia, dysfibrinogenemia, and hypodysfibrinogenemia can be either recessive (both parents carry the gene) or dominant (only one parent carries and transmits the gene).

3 What is referred to as â€oeweak muscleâ€•?

A Chronic fatigue

B Brachymesophalangy

C Hypotonia

2q37 deletion syndrome

It is a condition that can affect many parts of the body. This condition is characterized by hypotonia in infancy, mild to severe intellectual disability and developmental delay, behavioral problems, characteristic facial features, and other physical abnormalities.

Most babies with 2q37 deletion syndrome are born with potentially chronic fatigue, which usually improves with age. About 25 percent of people with this condition have autism, a developmental condition that affects communication and social interaction.

The characteristic facial features associated with 2q37 deletion syndrome include a prominent forehead, highly arched eyebrows, deep-set eyes, a flat nasal bridge, a thin upper lip, and minor ear abnormalities. Other features of this condition can include short stature, obesity, unusually short fingers and brachymesophalangy, sparse hair, heart defects, seizures, and an inflammatory skin disorder called eczema. A few people with 2q37 deletion syndrome have a rare form of kidney cancer called Wilms tumor.

4 The notice talks about;

A Every kind of procedure is used in the process of sterilization to keep sterile the objects or articles that are to be introduced into a wound or body cavity or that is

to penetrate the skin;

B General overview of sterilization techniques;

C The practices that the nurses will have to focus on;

Sterile technique

Surgical asepsis is used to maintain sterilize. Use of effective sterile technique means that no organisms are carried to the client. Microorganisms are destroyed before they can enter the body. Sterile technique is used when changing dressings, administering parenteral (other than the digestive tract) medications, and performing surgical and other procedures such as urinary catheterization. With surgical asepsis, first articles are sterilized, and then their contact with any unsterile articles is prevented. When a sterile article touches an unsterile article, it becomes contaminated. It is no longer sterile

5 It is known to prevent viral infection;

A PEG-IFN, RBV

B Boceprevir

C Sofosbuvir

Treatment for HCV

Before the commencement of HCV treatment, it is necessary to genotype the virus as different genotypes require different types and duration of treatment, and the protease inhibitors.

Current therapy for genotype 1 infection is a combination of PEG-IFN, RBV and a PI or nucleotide polymerase inhibitor, which results in high rates of sustained virological response (SVR; a negative HCV RNA test three or six months after the end of treatment). Boceprevir, simeprevir and telaprevir can also be used. Dual therapy with PEG-IFN and RBV or sofosbuvir with RBV is used for genotypes 2 and 3 infections. Patients with genotype 4 infection treated with treated with sofosbuvir, PEG-IFN and RBV have similar response rates when compared with genotype 1-infected individuals. Small studies of genotypes 5- and 6-infected patients have shown similar SVR rates to genotypes 2- and 3-infected ones.

6 The notice talks about;

A Rate of heterogeneity of genitourinary sarcomas.

B Survival rate and diagnosis.

C Survival rates of patients with genitourinary sarcomas.

Patients with genitourinary sarcomas

Patients with genitourinary sarcomas are relatively in a bad state, when compared with other soft tissue regions. Prognosis is relatively poor and can be explained by the high proportion seen in high degree tumors, a large proportion of patients with metastatic disease, large tumor and the area affected. In addition, the rarity and heterogeneity of genitourinary sarcomas can explain the great variability in clinical progress in different subgroups. Dissemination of urethral cancer follows the anatomic subdivision. The anterior urethra has a lymphatic drainage system for superficial and deep inguinal region. Posterior urethra drains the lymphatic ganglion of the external iliac artery, hypogastric, and internal obturator muscle. Late diagnosis is seen in one third of patients with inguinal lymphatic ganglion metastasis and in 20% of those with pelvic ganglion metastasis.

PART C

In this part of the test, there are two texts about different aspects of healthcare. For questions 7-22, choose the answer (A, B, C or D) which you think fits best according to the text.

Text 1: Cancer and Cervical Cancer

The body is made up of trillions of living cells. Normal body cells grow, divide, and die in an orderly fashion. During the early years of a person`s life, normal cells divide faster to allow the person to grow. After the person becomes an adult, most cells divide only to replace worn-out or dying cells or to repair injuries.

Cancer begins when cells in a part of the body start to grow out of control. There are many kinds of cancer, but they all start due to out-of-control growth of abnormal cells. Cells become cancer cells because of damage to DNA. DNA is in every cell and directs all its actions. In a normal cell, when DNA gets damaged the cell either repairs the damage or the cell dies. In cancer cells, the damaged DNA is not repaired, but the cell doesn`t die like it should. Instead, this cell goes on making new cells that the body does not need. These new cells will all have the same damaged DNA as the first cell does.

No matter where cancer may spread, it is always named after the place where it started. For example, breast cancer that has spread to the liver is still called breast cancer, not liver cancer. Likewise, prostate cancer that has spread to the bone is metastatic prostate cancer, not bone cancer. Different types of cancer can behave very differently. For example, lung cancer and

breast cancer are very different diseases. They grow at different rates and respond to different treatments. That is why people with cancer need the treatment that is aimed at their particular kind of cancer. Not all tumors are cancerous. Tumors that aren`t cancer are called benign. Benign tumors can cause problems ± they can grow very large and press on healthy organs and tissues. But they cannot grow into (invade) other tissues. Because they can`t invade, they also can`t spread to other parts of the body (metastasize). These tumors are almost never life threatening.

The cervix is the lower part of the uterus (womb). It is sometimes called the uterine cervix. The body of the uterus (the upper part) is where a baby grows. The cervix connects the body of the uterus to the vagina (birth canal). The part of the cervix closest to the body of the uterus is called the endocervix. The part next to the vagina is the exocervix (or ectocervix). The 2 main types of cells covering the cervix are squamous cells (on the exocervix) and glandular cells (on the endocervix). The place where these 2 cell types meet is called the transformation zone. Most cervical cancers start in the transformation zone and in the cells lining the cervix. These cells do not suddenly change into cancer. Instead, the normal cells of the cervix first gradually develop pre-cancerous changes that turn into cancer. Doctors use several terms to describe these pre-cancerous changes, including cervical intraepithelial neoplasia (CIN), squamous intraepithelial lesion (SIL), and dysplasia. These changes can be detected by the Pap test and treated to prevent the development of cancer.

Cervical cancers and cervical pre-cancers are classified by how they look under a microscope. There are 2 main types of cervical cancers: squamous cell carcinoma and adenocarcinoma. About 80% to 90% of cervical cancers are squamous cell carcinomas. These cancers are from the squamous cells that cover the surface of the exocervix. Under the microscope, this type of cancer is made up of cells that are like squamous cells. Squamous cell carcinomas most commonly begin where the exocervix joins the endocervix.

Most of the other cervical cancers are adenocarcinomas. Cervical adenocarcinomas seem to have become more common in the past 20 to 30 years. Cervical adenocarcinoma develops from the mucus-producing gland cells of the endocervix. Less commonly, cervical cancers have features of both squamous cell carcinomas and adenocarcinomas. These are called adenosquamous carcinomas or mixed carcinomas. Although cervical cancers start from cells with pre-cancerous changes (pre-cancers), only some of the women with pre-cancers of the cervix will develop cancer. The change from cervical pre-cancer to cervical cancer usually

takes several years, but it can happen in less than a year. For most women, precancerous cells will go away without any treatment. Still, in some women pre cancers turn into true (invasive) cancers. Treating all pre-cancers can prevent almost all true cancers.

Text 1: Questions 7-14

7 When a person becomes an adult, the cells divide only to;

A Replace old cells

B Replace dying cells

C Repair injuries

D All of the above

8 The DNA in cancer cells is;

A Damaged, but can be repaired.

B Not totally damaged.

C Permanently damaged, but the cell doesn`t die.

D Damaged but creates new cells with damaged DNA.

9 Cancer in liver having its origin in breast is a;

A Liver cancer

B Breast cancer

C Prostate cancer

D Bone cancer

10 Benign tumors;

A Can grow and invade the tissues.

B Can grow and press on healthy organs and tissues.

C Are cancerous.

D Are highly fatal.

11 Most cervical cancers start in;

A Squamous cells;

B Glandular cells;

C The transformation zone;

D All the above;

12 There are.....

A two

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B three

C four

D five

main types of cervical cancers

13 About 80 to 90% of cervical cancers are;

A Adenocarcinoma

B Adenosquamous carcinomas

C Mixed carcinomas

D Squamous cell carcinomas

14 Pre-cancerous cells in women can go away;

A With growth in more number of new cells.

B With medications from doctors.

C Without treatment

D Can`t say

Text 2: Hepatitis - Viral Liver Infection

Hepatitis (A, B, or C) can be caused by a virus (Viral Hepatitis), drugs, alcohol, medications, and blood transfusions. Scientists estimate that between 3.5 and 5.3 million people in the USA are living with Hepatitis. A blood test is required to diagnose Hepatitis infection. Hepatitis A is a viral liver infection. In most cases the body easily defeats the virus (much like the flu, which is what you may feel like you have). Because of this it does not lead to long term liver challenges. Hepatitis A is the most common form of Hepatitis. It is spread through the feces of a contaminated person. This can easily be prevented by thoroughly washing hands after using the restroom, before eating, and after changing a diaper.

Eating raw oysters and undercooked clams can increase your chances of contracting the virus. If you are traveling in a country where Hepatitis is common make sure you wash your hands often and well, eat cooked oysters and clams, and use an antiviral essential oil such as Lemon to help protect yourself. Hepatitis B is a viral liver infection. Again, most adult bodies are able to fight off the virus. In this case, it is referred to as Acute (something that does not last long) Hepatitis B. Hepatitis B is spread through contact with blood or bodily fluids of an infected person. This can include unprotected sexual intercourse, sharing drug needles, getting a

tattoo with instruments that were not properly cleaned, or by sharing a personal item such as a razor or toothbrush with an infected person.

A mother who is infected can pass the virus on to her baby during delivery. Again, the symptoms are flu-like in nature, so it often goes undiagnosed. A person who has Chronic (lasting three months or more) Hepatitis B may show no symptoms until liver damage has occurred. Hepatitis B can lead to liver damage or cancer;

your doctor may want to do a biopsy to determine the amount of damage your liver has experienced. Hepatitis C is also a viral liver infection. A few people will contract Hepatitis C and get better. This is called Acute Hepatitis C. Most, however, will develop Chronic Hepatitis C and go on to deal with liver damage, cirrhosis of the liver, liver cancer, and possibly liver failure. Hepatitis C is the number one reason for liver transplants in the USA.

Hepatitis C is spread through contact with contaminated blood. This can occur by sharing a needle, receiving a blood transfusion or organ transplant (blood and organs have been screened for Hepatitis in the USA since 1992), getting a tattoo with equipment that has not been properly cleaned, and, in rare cases, a mother can pass the virus on to her baby during birth. Scientists are not sure, but think there may be a slim possibility that the virus may be passed through unprotected sexual intercourse.

Symptoms generally do not occur until the virus is causing damage. Again, the symptoms are flu-like; you may also experience jaundice (yellowish eyes and skin) after the flu-like symptoms go away. Most people discover they are infected by having routine tests done or by donating blood or organs and the standard tests show the infection. There is also a home test you can purchase and do if you suspect you are infected.

If you are infected with a Hepatitis virus, or if you have been in the past, one of the most important things that you can do is strengthen your liver. The easiest way to do this is the Be Young Liver Cleanse: in the morning, take 1 drop of Be Young Lemon essential oil, 1 drop of Be Young Peppermint essential oil, and 1 teaspoon to 1 tablespoon of fresh lemon juice, followed by a glass of water. “Be Young essential oils” are absolutely 100% pure, EOBBD tested and guaranteed to be free of synthetics and extenders. Do not try this with an essential oil that you are not certain has been properly cared for and tested as you do not want to increase challenges to your liver. When properly supported, the liver has a remarkable capacity for regeneration.

Text 2: Questions 15-22

15 Hepatitis is caused by;

A Virus

B Alcohol consumption

C Medications

D All of the above

16 Which of the following spreads through feces of a contaminated person;

A Hepatitis A

B Hepatitis B

C Both

D Can`t say

17 Most adult bodies are able to fight off this virus;

A Hepatitis A virus

B Hepatitis B virus

C Both

D Can`t say

18 Hepatitis can lead to;

A Cancer

B Severe damage to the liver cells.

C Cancerous growth in the liver.

D Not given

19 In the USA people go for liver transplantation due to;

A Hepatitis A

B Hepatitis B

C Hepatitis C

D All of the above

20 Hepatitis C spreads through

A Sharing needles

B Blood transfusion

C Organ transplantation

D All of the above

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21 A patient may experience jaundice when;

A The flu-like symptoms appear.

B The flu-like symptoms disappear.

C Eyes become yellow.

D All of the above.

22 â€oeBe Young essential oilsâ€• are;

A Free from extenders

B EOBBD

C Full of synthetics

D Not given

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ANSWERS

PART A

Questions 1-7

For each question, 1-7, decide which text (A, B, C or D) the information comes from. You may use any letter more than once.

In which text can you find information about;

1: C

2: B

3: D

4: A

5: B

6: C

7: A

Questions 8-14

Answer each of the questions, 8-14, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8: repeated open application test

9: Corticosteroids

10: repeated open application test

11: false positive result

12: cosmetic and sunscreen constituents

13: false negative result

14: below 5%

Questions 15-20

Complete each of the sentences, 15-20, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

15: Chronic irritant contact dermatitis

16: sensitizers

17: dryness, lichenification and fissuring

18: notable offenders

19: photo allergen series

20: 5 J cm2 UVA

PART B

Questions 1-6

1: B Blood clot doesn`t form. (if the reaction doesn`t occur the way it should)

2: A All types of Factor I deficiency affect both males and females.

3: C Hypotonia

4: A Every kind of procedure is used in the process of sterilization to keep sterile the objects or articles that are to be introduced into a wound or body cavity or that is to penetrate the skin;

5: B Boceprevir

6: C Survival rates of patients with genitourinary sarcomas.

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PART C

Text 1: Questions 7-14

7: D All of the above

8: D Damaged but creates new cells with damaged DNA.

9: B Breast cancer

10: B Can grow and press on healthy organs and tissues.

11: C The transformation zone;

12: A two

13: D Squamous cell carcinomas

14: B With medications from doctors.

Text 2: Questions 15-22

15: D All of the above

16: A Hepatitis A

17: B Hepatitis B virus

18: A Cancer

19: C Hepatitis C

20: A Sharing Needles

21: B The flu-like symptoms disappear.

22: A Free from extenders

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PULMONARY EMBOLISM

TEXT A

A pulmonary embolism is a blockage in the pulmonary artery, which supplies the blood to the lungs. It is one of the most common cardiovascular diseases in the United States.

Pulmonary embolism affects around 1 in 1,000 people in the U.S. every year. The blockage, usually a blood clot, prevents oxygen from reaching the tissues of the lungs. This means it can be life-threatening. The word "embolism" comes from the Greek émbolos, meaning "stopper" or "plug." In a pulmonary embolism, the embolus, forms in one part of the body, it circulates throughout the blood supply, and then it blocks the blood flowing through a vessel in another part of the body, namely the lungs. An embolus is different from a thrombus, which forms and stays in one place.

Fast facts on pulmonary embolism

Here are some key points about pulmonary embolism. More detail and supporting information is in the main article.

The risk of pulmonary embolism increases with age

Symptoms include chest pain, dizziness, and rapid breathing

The risk of pulmonary embolism is high for individuals who have had a blood clot in the leg or arm

In rare cases, a pulmonary embolism can be caused by amniotic fluid

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TEXT B

Symptoms of pulmonary embolism include:

chest pain, a sharp, stabbing pain that might become worse when breathing in

increased or irregular heartbeat

dizziness

difficulty catching breath, which may develop either suddenly or over time

rapid breathing

cough, normally dry but possibly with blood, or blood and mucus

Severe symptoms call for immediate emergency medical assistance.

More severe cases may result in shock, loss of consciousness, cardiac arrest, and death.

A pulmonary embolism occurs when an embolus, usually a blood clot, blocks the blood flowing through an artery that feeds the lungs.

A blood clot may start in an arm or leg, known as deep venous thrombosis (DVT).

After that, it breaks free and travels through the circulatory system towards the lungs. There, it is too large to pass through the small vessels, so it forms a blockage.

This blockage stops blood from flowing into a part of the lung. This causes the affected section of the lung to die through lack of oxygen.

Rarely, a pulmonary embolism can result from an embolus that is formed from fat droplets, amniotic fluid, or some other particle that enters the bloodstream.

TEXT C

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Treatments for embolism aim to:

stop the clot from growing

prevent new clots from forming

destroy or remove any existing clot

A first step in treating most embolisms is to treat shock and provide oxygen therapy.

Anticoagulant medications, such as heparin, enoxaparin, or warfarin are usually given to help thin the blood and prevent further clotting.

Clot-busting drugs called thrombolytics may also be administered. However, but these carry a high risk of excessive bleeding. Thrombolytics include Activase, Retavase, and Eminase.

If the patient has low blood pressure, dopamine may be given to increase pressure.

The patient will normally have to take medications regularly for an indefinite amount of time, usually at least 3 months.

Prevention

A number of measures can reduce the risk of a pulmonary embolism.

A high-risk patient may use anticoagulant drugs such as heparin or warfarin.

Compression of the legs is possible, using anti-embolism compression stockings or pneumatic compression. An inflatable sleeve, glove, or boot holds the affected area and increases pressure when required.

Compression methods prevent blood clots by forcing blood into deep veins and reducing the amount of pooled blood.

Other ways to decrease the risk include physical activity, regular exercise, a healthy diet, and giving up or avoiding smoking tobacco.

TEXT D

To reach a diagnosis, the doctor will look at the patient's history and consider whether an embolism is likely. They will carry out a physical examination. Diagnosis can be challenging because other conditions have similar symptoms.

Tests for diagnosing pulmonary embolism include:

- electrocardiogram (EKG), to record the electrical activity of the heart
- arterial blood gas study, to measure oxygen, carbon dioxide, and other gases in the blood
- chest X-rays, to generate a picture of the heart, lungs, and other internal organs
- pulmonary V/Q scan, two tests that analyze the ventilation and structural properties of the lungs
- computerized tomography (CT) scan, which can reveal abnormalities in the chest, brain, and other organs
- ultrasound of the legs, to measure the speed of blood flow velocity and any changes
- d-Dimer test, a blood test that can diagnose thrombosis
- pulmonary angiogram, to reveal blood clots in the lungs
- magnetic resonance imaging (MRI), to obtain detailed pictures of internal structures

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A person has a higher risk of pulmonary embolism if they have, or have had a blood clot in the leg or arm (DVT), or if they have had a pulmonary embolism in the past.

Long periods of bed rest or inactivity increase the risk of DVT and, therefore, increase the risk of pulmonary embolism. This could be a long flight or car ride.

When we do not move much, our blood pools in the lower parts of our body. If blood is moving around less than normal, a blood clot is more likely to form.

Damaged blood vessels also increase the risk. This can occur because of injury or surgery. If a blood vessel is damaged, the inside of the blood vessel may become narrower, increasing the chances of a blood clot forming.

Other factor risk include certain cancers, inflammatory bowel disease, obesity, pacemakers, catheters in the veins, pregnancy, estrogen supplements, a family history of blood clots, and smoking.

With effective and timely treatment, most people who experience a pulmonary embolism can make a full recovery.

The condition carries a high risk of fatality. However, early treatment can dramatically reduce this risk.

The period of highest risk is in this hours after the embolism first occurs. The outlook is also worse if the embolism was caused by an underlying condition, such as a type of cancer.

However, most people with pulmonary embolism can make a full recovery.

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Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.

- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Pulmonary Embolism

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1.Managing shock and oxygen supplementation is very important in treating patient with embolism

2.Prognosis for embolism is poor if patient has cancer.

3. A thrombus does not circulates throughout the blood supply

4. Less movement can also lead to embolism.

5. Eminase is a thrombolytic.

6. Deep venous thrombosis is manifested mainly in an arm or leg.

7. Most severe cases may result in shocks and death

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. One of the infrequent causes for pulmonary embolism?

9. What cause high risk for excessive bleeding?

10. What is administered to elevate blood pressure?

11. What helps in lowering the amount of pooled blood?

12. What is responsible for the death of lung tissue?

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13. What prevent oxygen supply to the tissues of the lungs?

14. What type of drugs might be used by high-risk patients?

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

15. Demographic factor like _____ is directly proportional to likelihood of pulmonary embolism.

16. _____ requires urgent.

17. Ruling out of disease is often _____ due to similar symptoms of other condition.

18. _____ blocks the blood flowing through a vessel in another part of the body.

19. Early _____ can gradually reduce the risk.

20. Forestalling new _____ can cure embolism.

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Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

toxoplasmosis

Many pathogens are restricted to the site of colonization or have a distribution restricted to specific tissues. For others, the ability to disseminate from the initial point of infection and invade different niches is an integral part of their biology. For example, various protozoan and helminth parasites need to migrate through distinct host tissues to complete their life cycles. Thus, the success of Plasmodium is dependent on the ability of different developmental

stages to migrate from the skin to the liver, and finally to the blood for transmission. Similarly, Schistosoma mansoni undergoes a protracted migration that starts in the skin and proceeds through multiple tissues, including the lungs, before adults pair in the mesenteric venules, allowing eggs to exit through the intestine. For these parasites, the clinical features and tissues affected are a consequence of the natural progression of the infection. For other parasites, inappropriate migration or dissemination forms the basis for disease and this is illustrated by the ability of Entamoeba histolytica to cross from the intestine and cause the development of liver abscesses. Similarly, there are several helminthes and nematodes that, when in inappropriate hosts, fail to develop fully and continuously migrate through tissues such as the brain where they can cause extensive tissue damage.

1. When does the plasmodium successfully cause infection:

a) When it affects the liver.

b) When it reaches the blood circulation.

c) When it invades the skin.

Kreb's Cycle

Cancer cells exhibit alterations in many cellular processes, including oxygen sensing and energy metabolism. Glycolysis in non-oxygen condition is the main energy production process in cancer rather than mitochondrial respiration as in benign cells. Genetic and epigenetic alterations of Krebs cycle enzymes favour the shift of cancer cells from oxidative phosphorylation to anaerobic glycolysis. Mutations in genes encoding aconitase, isocitrate dehydrogenase, succinate dehydrogenase, fumarate hydratase, and citrate synthase are noted in many cancers. Abnormalities of Krebs cycle enzymes cause ectopic production of Krebs cycle intermediates (oncometabolites) such as 2-hydroxyglutarate, and citrate. These oncometabolites stabilize hypoxia inducible factor 1 (HIF1), nuclear factor like 2 (Nrf2), inhibit p53 and prolyl hydroxylase 3 (PDH3) activities as well as regulate DNA/histone methylation, which in turn activate cell growth signalling. They also stimulate increased glutaminolysis, glycolysis and production of reactive oxygen species (ROS). Additionally, genetic alterations in Krebs cycle enzymes are involved with increased fatty acid β-oxidations and epithelial mesenchymal transition(EMT) induction. These altered phenomena in cancer could in turn promote carcinogenesis by stimulating cell proliferation and survival. Overall, epigenetic and genetic changes of Krebs cycle enzymes lead to the production of oncometabolite intermediates, which are important driving forces of cancer pathogenesis and progression. Understanding and applying the knowledge of these mechanisms opens new therapeutic options for patients with cancer.

2. In which condition does the glycolysis becomes active?

a) Inside mitochondria

b) Non-Oxygen condition.

c) In energy production process.

Leptospira

Leptospirosis, the most widespread zoonosis in the world, is an emerging public health problem, particularly in large urban centers of developing countries. Several pathogenic species of the genus Leptospira can cause a wide range of clinical manifestations, from a mild, flu-like illness to a severe disease form characterized by multiorgan system complications leading to death. However, the mechanisms of pathogenesis of Leptospira are largely unknown. This article will address the animal models of acute and chronic leptospire infections, and the recent developments in the genetic manipulation of the bacteria, which facilitate the identification of virulence factors involved in pathogenesis and the assessment of their potential values in the control and prevention of leptospirosis.

3. The mechanism of pathogenesis of leptospira is:

a) Most immune response.

b) Characteristics of multi-organ system.

c) Unknown entiliogy..

Niacin-Induced flushing:

This comprehensive review of the mechanism of action of niacin-induced flushing critically evaluates research regarding flushing mitigating formulations and agents. Niacin induces flushing through dermal Langerhans cells where the activation of G protein-coupled receptor 109A (GPR109A) increases arachidonic acid and prostaglandins, such as prostaglandin D 2 (PGD 2) and prostaglandin E 2 (PGE 2), subsequently activating prostaglandin D 2 receptor (DP 1), prostaglandin E 2 receptor (EP 2) and prostaglandin Ereceptor 4 (EP 4) in capillaries and causing cutaneous vasodilatation. Controlling niacin absorption rates, inhibiting prostaglandin production, or blocking DP 1 , EP 2 and EP 4 receptors can inhibit flushing. Niacin extended-release (NER) formulations have reduced flushing incidence, duration and severity relative to crystalline immediate-release niacin with similar lipid efficacy. Non-steroidal anti-inflammatory drugs (NSAIDs), notably aspirin given 30 min before NER at bedtime, further reduce flushing. An antagonist to the DP 1 receptor (laropiprant) combined

with an ER niacin formulation can reduce flushing; however, significant residual flushing occurs with clinically-relevant dosages. Niacin is an attractive option for treating dyslipidemic patients, and tolerance to niacin-induced flushing develops rapidly. Healthcare professionals should particularly address flushing during niacin dose titration.

4. The drug which is administered at bedtime to reduce further flushing is:

a) NSAIDS

b) Aspirin

c) Niacin

Magnetic resonance spectroscopy

Magnetic resonance spectroscopy (MRS), also called nuclear magnetic resonance spectroscopy, diagnostic imaging technique based on the detection of metabolites in tissues. Magnetic resonance spectroscopy (MRS) is related to magnetic resonance imaging (MRI) in that it uses the same machinery; however, instead of measuring blood flow, MRS measures the concentration of specific chemicals, such as neurotransmitters. MRS holds great promise in the diagnosis of diseases of the brain and of other parts of the body, including cancers of the cervix, pancreas, and prostate. By measuring the molecular and metabolic changes that occur in the brain, this technique has provided valuable information on brain development and aging, Alzheimer disease, schizophrenia, autism, and stroke. Because it is noninvasive, MRS is ideal for studying the natural course of a disease or its response to treatment. See also nuclear magnetic resonance and magnetic resonance.

5. Magnetic resonance spectroscopy (MRS) test is :

a) Invasive

b) Non-invasive

c) Chemical induced

Mammography

Mammography, medical procedure employing X-ray technology to detect lesions in the breast that may be indicative of breast cancer. Although not all lesions in breast tissue are detectable by X-ray examination, many lesions often can be detected by mammography before they are palpable in the breast by physical examination. Thus, the primary purpose for mammography is the early detection of cancer. Early detection increases the chances of

successful treatment, since the disease is most susceptible to anticancer drugs when detected in its initial stages of development. Mammography is most useful in older women whose breast tissue is less dense than that of younger women. Some groups recommend an initial mammogram between ages 35 and 40 to serve as a baseline for subsequent screening. The American Cancer Society recommends a yearly mammogram for women starting at age 45 followed by biennial screening from age 55. Women who are suspected to be at increased risk of breast cancer may begin regular mammographic screening at an earlier age (e.g., 40 years). The risk of breast cancer is significantly increased in women who have a sister with breast cancer or whose mother was diagnosed with breast cancer before age 40.

6. The incidence of breast cancer is more in

a) Genetic

b) Race

c) Airborne

PART C

Breast Cancer and the Elderly

Breast cancer is one of the highest-profile diseases in women in developed countries. Although the risk for women younger than 30 years is minimal, this risk increases with age. One-third of all breast cancer patients in Sweden, for example, are 70 years or older at diagnosis. Despite these statistics, few breast cancer trials take these older women into account. Considering that nowadays a 70-year-old woman can expect to live for at least another 12–16 years, this is a serious gap in clinical knowledge, not least because in older women breast cancer is more likely to be present with other diseases, and doctors need to know whether cancer treatment will affect or increase the risk for these diseases.

In 1992, guidelines were issued to the Uppsala/Örebro region in Sweden (with a population of 1.9 million) that all women with breast cancer should be able to receive equal treatment. At the same time, a breast cancer register was set up to record details about patients in the region, to ensure that the guidelines were being followed. Sonja Eaker and colleagues set out to assess data from the register to see whether women of all ages were receiving equal cancer treatment.

They compared the 5-year relative survival for 9,059 women with breast cancer aged 50–84 years. They divided them into two age groups: 50–69 years, and 70–84 years. They also categorized the women according to the stage of breast cancer. They looked at differences between the proliferative ability of breast cancer cells, estrogen receptor status, the number of lymph nodes examined, and lymph node involvement. The researchers also compared types of treatment—i.e., surgical, oncological (radiotherapy, chemotherapy, or hormonal)—and the type of clinic the patients were treated in.

They found that women aged 70–84 years had up to a 13% lower chance of surviving breast cancer than those aged 50–69 years. Records for older women tended to have less information on their disease, and these women were more likely to have unknown

proliferation and estrogen receptor status. Older women were less likely to have their cancer detected by mammography screening and to have the stage of disease identified, and they had larger tumours. They also had fewer lymph nodes examined, and had radiotherapy and chemotherapy less often than younger patients.

Current guidelines are vague about the use of chemotherapy in older women, since studies have included only a few older women so far, but this did not explain why these women received radiotherapy less often. Older women were also less likely to be offered breast-conserving surgery, but they were more likely to be given hormone treatment such as tamoxifen even if the tumours did not show signs of hormone sensitivity. The researchers suggest that this could be because since chemotherapy tends to be not recommended for older women, perhaps clinicians believed that tamoxifen could be an alternative.

The researchers admit that one drawback of their study is that there was little information on the other diseases that older women had, which might explain why they were offered treatment less often than younger patients. However, the fact remains that in Sweden, women older than 70 years are offered mammography screening much less often than younger women— despite accounting for one-third of all breast cancer cases in the country— and those older than 74 years are not screened at all. Eaker and co-workers' findings indicate that older women are urgently in need of better treatment for breast cancer and guidelines that are more appropriate to their age group. Developed countries, faced with an increasingly aging population, cannot afford to neglect the elderly.

7. The main idea presented in paragraph one is that.....

a. only older women need to be concerned about breast cancer.

b. breast cancer trials seldom consider older women.

c. breast cancer is more common than other diseases in older

woman.

d. older woman do not take part in breast cancer trials.

8. Regarding cancer treatment, it can be concluded that....

a. doctors know cancer treatment will increase the risk of disease

in elderly patients.

b. cancer treatments may be a risk for all elderly people

c. it is unknown whether or not cancer treatments will affect the

treatment of other diseases in elderly people.

d. older woman are less likely to have other diseases

9. 1992 Guidelines issued to the Uppsala/Orebro region in Sweden

stated that...

a. Sweden has a population of 1.9 million.

b. women with breast cancer need to register their condition to ensure they receive equal treatment.

c. identical breast cancer treatment should be available to women of all ages.

d. all women with breast cancer should have access to equivalent breast cancer treatment.

10. Which of the following was not part of Sonja Eaker and her colleagues research?

a. Comparing ability of breast cancer cells to increase in number.

b. Grouping woman according to their survival rate.

c. Identifying differences in treatment methods.

d. Splitting the groups based on age.

11. Findings by the researchers indicate that.....

a. older women are less likely to have chemotherapy recommended.

b. older women prefer hormone treatment to breast-conversing surgery.

c. older women have fewer lymph nodes.

d. older women respond better to chemotherapy than to hormone treatment.

12. The word vague is paragraph 5 means.....

a. uncertain

b. unclear

c. unknown

d. doubtful

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13. One limitation of the study is that.....

a. older women are treated less often than younger women.

b. older women have a lower incidence of breast cancer.

c. younger women are treated more often than older women.

d. there is a lack of information on other diseases which older women have.

14. Which of the following statements best represents the view expressed by the writer at the end of the article?

a. Due to ageing population in developed countries, the needs of the elderly must not be ignored.

b. Older women need more appropriate treatment to suit their age.

c. Developed countries have neglected the elderly for too long.

d. It is too expensive treat the elderly.

MANJOORAN Connect

Swine Flu Found in Birds

Last week the H1N1 virus was found in turkeys on farms in Chile. The UN now says poultry farms elsewhere in the world could also become infected. Scientists are worried that the virus could theoretically mix with more dangerous strains. It has previously spread from humans to pigs. However, swine flu remains no more severe than seasonal flu.

Chilean authorities first reported the incident last week. Two poultry farms are affected near the seaport of Valparaiso. Juan Lubroth, interim chief veterinary officer of the UN Food and Agriculture Organization (FAO), said: "Once the sick birds have recovered, safe production and processing can continue. They do not pose a threat to the food chain."

Chilean authorities have established a temporary quarantine and have decided to allow the infected birds to recover rather than culling them. It is thought the incident represents a "spill-over" from infected farm workers to turkeys. Canada, Argentina and Australia have previously reported spread of the H1N1 swine flu virus from farm workers to pigs.

The emergence of a more dangerous strain of flu remains a theoretical risk. Different strains of virus can mix together in a process called genetic reassortment or recombination. So far there have been no cases of H5N1 bird flu in flocks in Chile. However, Dr Lubroth said: "In Southeast Asia there is a lot of the (H5N1) virus circulating in poultry. "The introduction of H1N1 in these populations would be of greater concern."

Colin Butter from the UK's Institute of Animal Health agrees. "We hope it is a rare event and we must monitor closely what happens next," he told BBC News. "However, it is not just about the H5N1 strain. Any further spread of the H1N1 virus between birds, or from birds to humans would not be good. "It might make the virus harder to control, because it would be more likely to change."

William Karesh, vice president of the Wildlife Conservation Society, who studies the spread of animal diseases, says he is not surprised by what has happened. "The location is surprising, but it could be that Chile has a better surveillance system. "However, the only constant is that the situation keeps changing."

The United States has counted 522 fatalities through Thursday, and nearly 1,800 people had died worldwide through August 13, U.S. and global health officials said. In terms of mortality rate, which considers flu deaths in terms of a nation's population, Brazil ranks seventh, and the United States is 13th, the Brazilian Ministry of Health said in a news release Wednesday.

Argentina, which has reported 386 deaths attributed to H1N1 as of August 13, ranks first per capita, the Brazilian health officials said, and Mexico, where the flu outbreak was discovered in April, ranks 14th per capita. Brazil, Argentina, Chile, Mexico and the United States have the most total cases globally, according to the World Health Organization.

The Brazilian Ministry of Health said there have been 6,100 cases of flu in the nation, with 5,206 cases (85.3 percent) confirmed as H1N1, also known as swine flu. The state of Sao Paulo had 223 deaths through Wednesday, the largest number in the country. In addition, 480 pregnant women have been confirmed with H1N1, of whom 58 died. Swine flu has been shown to hit young people and pregnant women particularly hard.

Many schools in Sao Paulo have delayed the start of the second semester for a couple of weeks, and students will have to attend classes on weekends to catch up. Schools also have suspended extracurricular activities such as soccer, volleyball and chess to try to curtail spread of the disease.

Flu traditionally has its peak during the winter months, and South America, where it is winter, has had a large number of cases recently. The World Health Organization said this week that the United States and other heavily populated Northern Hemisphere countries need to brace for a second wave of H1N1 as their winter approaches.

Officials at the Centres for Disease Control and Prevention and other U.S. health agencies have been preparing and said this week that up to half of the nation's population may contract the disease and 90,000 could die from it. Seasonal flu typically kills about 64,000 Americans each year.

A vaccine against H1N1 is being tested but is not expected to be available until at least mid-October and will probably require two shots at least one week apart, health officials have said. Since it typically takes a couple of weeks for a person's immunity to build up after the vaccine, most Americans would not be protected until sometime in November. The World Health Organization in June declared a Level 6 worldwide pandemic, the organization's highest classification.

QUESTIONS

Swine Flu Found in Birds

15. Scientists are worried that the virus could potentially spread...

a.) ...from pigs to humans

b.) ...to chicken and turkey farms elsewhere

c.) ...to other types of animals

d.) ...to the seaport of Valparaiso

16. What does Dr. Lubroth recommend should be done with the sick birds?

a.) They should be processed immediately.

b.) They should be killed.

c.) They should be allowed to recover.

d.) They should be given Tamiflu.

17. What is the meaning of the "spill-over" effect mentioned in the passage?

a.) The virus has spread from Chile to Argentina.

b.) The virus has spread from factory workers to birds.

c.) Turkey blood has been spilled during the production process.

d.) Turkeys have become infected by eating spilled contaminated pig food.

18. Which possibility is Dr. Lubroth most concerned about?

a.) H5N1 virus spreading to Chile

b.) H591 virus spreading to Australia

c.) H191 virus spreading to Asia

d.) H191 virus spreading to Canada

19. Which statement best describes the opinion of the representative from the Institute of Animal Health?

a.) He doesn!t want the virus to spread further because it could lead to genetic reassortment.

b.) He thinks H5N1 is no longer important but he is worried about H1N1.

c.) He hopes that BBC News will pay more attention to closely monitoring the virus.

d.) Birds and humans should be under more control otherwise the virus may change.

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20.Which statement best describes the opinion of the Vice President of the Wildlife Conservation Society?

a.) He is not surprised that not enough people are studying the spread of animal diseases.

b.) He is not surprised that swine flu has been reported in birds in Chile.

c.) He is surprised that the situation is constantly changing.

d.) He is surprised that swine flu has been reported in birds in Chile, but suspects other countries may be unaware of the spread to birds.

21. According to the Brazilian Ministry of Health...

a.) ...The United States has counted 522 fatalities.

b.) ...more people have died in Brazil than in the USA.

c.) ...more people have died in the USA than in Brazil.

d.) ...Brazil is the 13th worst country for swine flu deaths.

22. Which of the following statements is FALSE?

a.) 52 pregnant women have died of Swine Flu in Brazil.

b.) Argentina has reported 386 H591 related deaths.

c.) Swine flu was first discovered in Mexico in April.

d.) The USA is one of the most severely affected countries annually.

ANSWERS

1.Text C

2.Text D

3.Text A

4.Text D

5.Text C

6.Text B

7.Text B

8.amniotic fluid

9.thrombolytics

10.Dopamine

11.compression method

12.lack of oxygen

13.blood clot

14.anti-coagulant

15.age

16.severe symptoms

17.challeging

18.embolus

19.treatment

20.clot formation

PART B

1.B

2.B

3.C

4.B

5.A

6.B

PART C

1. B
2. C
3. D
4. B
5. A
6. B
7. D
8. A

15. B

16. C

17. B

18. A

19. B

20. C

21. D

22. B

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Cholecystitis

TEXT A

Cholecystitis is an inflammation of the gallbladder. It normally happens because a gallstone gets stuck at the opening of the gallbladder. It can lead to fever, pain, nausea, and severe complications.
Untreated, it can result in perforation of the gallbladder, tissue death and gangrene fibrosis and shrinking of the gallbladder, or secondary bacterial infections.
Gallstone are involved in 95 percent of cholecystitis cases. These may be formed from cholestrol a pigment known as bilirubin, or a mix of the two. It can also be triggered by biliary sludge when bile collects in the biliary ducts.
Other causes include trauma, critical illness, immunodeficiency, or certain medications.
Some chronic medical conditions, like kidney failure, coronary heart disease, or certain types of cancer also increase the risk of cholecystitis.
In the United States, there were 215,995 hospital admissions for cholecystitis in 2012, and the average hospital stay was 3.9 days.
Acute cholecystitis starts suddenly. Chronic cholecystitis develops slowly over time.

TEXT B

The main cause of cholecystitis is gallstones or biliary sludge getting trapped at the gallbladder's opening. This is sometimes called a pseudolith, or "fake stone."
Other causes include:

- injury to the abdomen from burns, sepis or trauma, or because of surgery
- shock
- immune deficiency
- prolonged fasting
- vasculitis

An infection in the bile can lead to inflammation of the gallbladder.
A tumor may stop the bile from draining out of the gallbladder properly, resulting in an accumulation of bile. This can lead to cholecystitis.
Symptoms

Gallstones in the gall bladder can lead to cholecystitis.
Signs and symptoms of cholecystitis include right upper quadrant pain, fever, and a high white blood cell count. Pain generally occurs around the gallbladder, in the right upper quadrant of the abdomen.
In cases of acute cholecystitis, the pain starts suddenly, it does not go away, and it is intense. Left untreated, it will usually get worse, and breathing in deeply will make it feel more intense. The pain may radiate from the abdomen to the right shoulder or back.
Other symptoms may include:

- abdominal bloating
- tenderness on the upper-right hand side of the abdomen

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- little or no appetite
- nausea
- vomiting
- sweating

A slight fever and chills may be present with acute cholecystitis.
After a meal, especially one that is high in fat, symptoms will worsen. A blood test may reveal a high white blood cell count.

TEXT C

Treatment
A healthy diet can help prevent gallstones, a common cause of cholecystitis.
A patient with cholecystitis will be hospitalized, and they will probably not be allowed to consume any solid or liquid foods for some time. They will be given liquids intravenously while fasting. Pain medications and antibiotics may also be given.
Surgery is recommended for acute cholecystitis because there is a high rate of reccurence from inflammation related to gallstones. However, if there is a low risk of complications, surgery can be done as an outpatient procedure.
If there are complications, such as gangrene or perforation of the gallbladder, the patient will need immediate surgery to remove the gall bladder.if the patient has an infection, a tube may be inserted through the skin into the gallbladder to drain the infection.
Removal of the gallbladder, or cholecystectomy, can be performed by open abdominal excision or laparoscopically.
Laparoscopic cholecystectomy involves several small incisions in the skin. A camera is inserted into one incision to help the surgeon see inside the abdomen, and tools for removing the gallbladder and inserted through the other incisions.
The benefit of laparoscopy is that the incisions are small, so patients usually have less pain after the procedure and less scarring.
After surgically removing the gallbladder, the bile will flow directly into the small intestine from the liver. This does not normally affect the patient's overall health and digestive system. Some patients may have more frequent episodes of diarrhea.
The gallbladder is a small, pear-shaped organ connected to the liver, on the right side of the abdomen. It stores bile and releases it into the small intestine to help in the digestion of fat.
The gallbladder holds bile, a fluid that is released after we eat, especially after a meal that is high in fat, and this bile aids digestion. The bile travels out of the gallbladder through the cystic duct, a small tube that leads to the common bile duct, and from there into the small intestine.

TEXT D

Complications

Cholecystitis can cause abdominal pain.

Untreated acute cholecystitis can lead to:

· A fistula, a kind of tube or channel, can develop if a large stone erodes the wall of the gallbladder. This can link the gallbladder and the duodenum, and the stone may pass through.

· Gallbladder distention: If the gallbladder is inflamed because of bile accumulation, it may stretch and swell, causing pain. There is then a much greater risk of a perforation, or tear, in the gallbladder, as well as infection and tissue death.

· Tissue death: Gallbladder tissue can die, and gangrene develops, leading to perforation, or the bursting of the bladder. Without treatment, 10 percent of patients with acute cholecystitis will experience localized perforation, and 1 percent will develop free perforation and peritonitis.

If a gallstone becomes impacted in the cystic duct, it can compress and block the common bile duct, and this can lead to cholestasis. This is rare.

Gallstones can sometimes pass from the gallbladder into the biliary tract, leading to an obstruction of the pancreatic duct. This may cause pancreatitis.

In 3 percent to 19 percent of cases, of cases, acute cholecystitis can lead to a pericholecystic abscess. Symptoms include nausea, vomiting, and abdominal pain.

Prevention

Some measures can reduce the risk of developing gallstones, and this can decrease the chance of developing cholecystitis:

· avoiding saturated fats

· keeping to a regular breakfast, lunch and dinner times and not skipping meals

· exercising 5 days per week for at least 30 minutes each time

· losing weight, because obesity increases the risk of gallstones

· avoiding rapid weight loss as this increases the risk of developing gallstones

A healthy weight loss is generally around 1 to 2 pounds, or 0.5 to 1 kilograms, of body weight per week.

The nearer a person is to their ideal body weight, the lower the risk will be of developing gallstones. Gallstones are more prevalent in people with obesity, compared with those who have an appropriate body weight for their age, height, and body frame.

ANJORAN Connect

Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Cholecystitis

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. If the bile directly flow from liver to small intestine, it doesn't affect patient's overall health

2. If acute cholecystitis is left untreated, it will get worsened and breathing in deeply will make it more intense

3. Gall stones commonly affect obese people

4. Formation of gall stones can be triggered by biliary sludge when bile collects in the biliary ducts

5. In some occasions, obstruction of pancreatic duct occurs if gall stones past from the gall badder

   into biliary tract

6. Pseudolith is also known as fake stone

7. Function of gall bladder

MANJOORAN Connect

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. Avoiding which kind of food can prevent cholecystitis?

9. List one of the causative factor which might lead to accumulation of bile?

10. What is the reccurrent cause of cholecystitis?

11. What is the management for patient who has gangrene or perforation of the gall bladder?

12. What is the function of bile ?

13 What can be done to manage the infection associated with gall bladder?

14. What can reduce the chance of gall stones progress

MANJOORAN Connect

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

15. ---------- is an unaccustomed complication of cholecystitis

16. During hospitalization, patient's with cholecystitis are administered with ---------- while

fasting .

17. ----------, high in fat can worsen the symptom .

18. ---------- occurs in the upper right quadrant of the abdomen.

19. ---------- can be triggered by sludge

20. If patient is not in a danger to get complication, ---------- can be done as outpatient procedure .

PART C

In this part of the test, there, are six short extracts relating to the work of health care.

For questions 1-6

Treatment of congenital fibrinogen deficiency

Pap smear, also called Papanicolaou smear, laboratory method of obtaining secretions from the cervix for the examination of cast-off epithelial cells to detect the presence of cancer. The Pap smear, named for Greek-born American physician George Papanicolaou, is notably reliable in detecting the early stages of cancer in the uterine cervix. Two specimens are usually taken for laboratory staining and examination, one consisting of vaginal secretions and the other of scrapings of the surface of the cervix at the site where cancerous growth frequently originates. The Pap smear may reveal malignant cells not only from the cervix but also from the endometrium (the mucous coat of the uterus) and the ovaries. The traditional Pap smear, in which cells are literally smeared directly onto a glass slide, is now less common than the Pap test, in which the cells are first placed in a liquid medium before processing. The latter method has the advantage of allowing the laboratory technician to centrifuge the cells and to filter blood, mucus, and debris that can make slide interpretation difficult. The American College of Obstetricians and Gynecologists recommends a biennial Pap test for all women once they have reached age 21. The frequency of Pap tests may be reduced if a woman has had multiple consecutive tests prove negative. For example, women age 30 and older who have had negative results may require a Pap test only once every three years.

1. Pap smear is most reliable in detecting

a) Early stages of cancer

b) Uterine cancer

c) Cervical Malignancy.

Phenolsulfonphthalein test

Phenolsulfonphthalein test, also called Psp Test, clinical procedure for the estimation of overall blood flow through the kidney; the the kidney; the test is used only infrequently now. A specific dose of the PSP dye is injected intravenously, and its recovery in the urine is measured at successive 15-, 30-, 60-, and 120-minute intervals. The kidney secretes 80 percent of the PSP dye, the liver the remaining 20 percent. The recovery value at 15 minutes after injection (normally about 25–35 percent) is the most significant diagnostically, since even a damaged kidney may be able to remove the PSP dye from circulation given a longer time to do so. PSP excretion is decreased in most chronic kidney diseases and may be increased in some liver disorders. See also kidney function test.

2. In PSP test recovery of the urine is measured at what intervals.

a) 20-minute

b) 60-minute

c) 50-minute

Pregnancy test

Pregnancy test, procedure aimed at determining whether a woman is pregnant. Pregnancy tests are based on a detectable increase in human chorionic gonadotropin (HCG) in the blood serum and urine during early pregnancy. HCG is the principal hormone produced by the chorionic layers of the placenta, the temporary organ that provides nourishment for the developing fetus. Levels of HCG increase significantly following implantation of the fertilized egg in the uterine wall, which occurs sometime between 6 and 12 days after fertilization. In home pregnancy tests, which are qualitative (determining whether HCG is present), a small amount of urine is applied to a chemical strip. The result is usually indicated by some visible change in the strip (whether this is a change in colour or the appearance of a symbol depends upon the way in which the test is manufactured). A positive home pregnancy test should be confirmed with a laboratory test and pelvic examination by a doctor. Pregnancy tests performed in a laboratory on a sample of blood or urine are quantitative and therefore are more accurate than a home pregnancy test. Laboratory tests using a sample of blood also have a high degree of sensitivity and can be used to detect increased levels of HCG early in the implantation process.

3. Temporary organ that provides nourishment for the developing fetus

a) HCG

b) Placenta

c) Chorlonic layers of the placenta

Myelography

Myelography, medical procedure for examining the spinal cord by means of X rays. It is especially useful in diagnosing spinal abscesses and tumours and dislocated intervertebral disks. In this procedure a positive contrast agent, usually in the form of a water-soluble radiopaque substance or iodinated oil, is injected into the spinal canal. This contrast agent, which makes body tissue more visible when irradiated with X rays, is maneuvered throughout the spinal canal from the lumbrosacral (tail bone) region up to the base of the brain by tilting the examination table to which the patient is strapped. A fluoroscope is used to observe the various parts of the canal as the contrast agent passes through them. The contrast medium is generally removed after a myelogram has been completed, though the use of certain water-soluble radiopaque substances makes this step unnecessary since they are readily eliminated

from the body by natural means. Iodinated oil may cause irritations, resulting in temporary discomfort.

4. The use of fluroscope in myelography

a) Observation of various parts of the spinal canal

b) Observation of body tissues

c) Identification of spinal tissues

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4. The use of fluroscope in myelography

a) Observation of various parts of the spinal canal

b) Observation of body tissues

c) Identification of spinal tissues

Tomography

Tomography,, radiologic technique for obtaining clear X-ray images of deep internal structures by focusing on a specific plane within the body. Structures that are obscured by overlying organs and soft tissues that are insufficiently delineated on conventional X rays can thus be adequately visualized. The simplest method is linear tomography, in which the X-ray tube is moved in a straight line in one direction while the film moves in the opposite direction. As these shifts occur, the X-ray tube continues to emit radiation so that most structures in the part of the body under examination are blurred by motion. Only those objects lying in a plane coinciding with the pivot point of a line between the tube and the film are in focus. A somewhat more complicated technique known as multidirectional tomography produces an even sharper image by moving the film and X-ray tube in a circular or elliptical pattern. As long as both tube and film move in synchrony, a clear image of objects in the focal plane can

be produced. These tomographic approaches have been used to study the kidneys and other abdominal structures that are surrounded by tissues of nearly the same density and so cannot be differentiated by conventional X-ray techniques. They have also been employed to examine the small bones and other structures of the ear, which are surrounded by relatively dense temporal bone.

5. Tomography studies are used except

a) Kidney function analysis

b) Examination of small bones

c) Cardiac function analysis

Tomography

Ultrasound, also called ultrasonography, in medicine, the use of high-frequency sound (ultrasonic) waves to produce images of structures within the human body. Ultrasonic waves are sound waves that are above the range of sound audible to humans. The ultrasonic waves are produced by the electrical stimulation of a piezoelectric crystal and can be aimed at a specific area of the body. As the waves travel through bodily tissues, they are reflected back at any point where there is a change in tissue density, as, for instance, in the border between two different organs of the body. The reflected echoes are received by an electronic apparatus that determines the intensity level of the echoes and the position of the tissue giving rise to the echoes. The images thus formed can be displayed in static form, or, through the use of rapid multiple sound scans, they can in effect provide a moving picture of the inside of the body.

6. Function of electronic apparatus

a) Reception of reflected echoes and determination of intensity

b) Electrical stimulation

c) Use of multiple scan

PART C

The Mental Health Risks of Adolescent Cannabis Use

Since the early 1970s, when cannabis first began to be widely used, the proportion of young people who have used cannabis has steeply increased and the age of first use has declined. Most cannabis users now start in the mid-to-late teens, an important period of psychosocial transition when misadventures can have large adverse effects on a young person's life chances. Dependence is an underappreciated risk of cannabis use. There has been an increase in the numbers of adults requesting help to stop using cannabis in many developed countries, including Australia and the Netherlands. Regular cannabis users develop tolerance to many of the effects of delta-9- tetrahydrocannabinol, and those seeking help to stop often report withdrawal symptoms. Withdrawal symptoms have been reported by 80% of male and 60% of female adolescents seeking treatment for cannabis dependence.

In epidemiological studies in the early 1980s and 1990s, it was found that 4% of the United States population had met diagnostic criteria for cannabis abuse or dependence at some time in their lives and this risk is much higher for daily users and persons who start using at an early age. Only a minority of cannabis-dependent people in surveys report seeking treatment, but among those who do, fewer than half succeed in remaining abstinent for as long as a year. Those who use cannabis more often than weekly in adolescence are more likely to develop dependence, use other illicit drugs, and develop psychotic symptoms and psychosis.

Surveys of adolescents in the United States over the past 30 years have consistently shown that almost all adolescents who had tried cocaine and heroin had first used alcohol, tobacco, and cannabis, in that order; that regular cannabis users are the most likely to use heroin and cocaine; and that the earlier the age of first cannabis use, the more likely a young person is to use other illicit drugs. One explanation for this pattern is that cannabis users obtain the drug from the same black market as other illicit drugs, thereby providing more opportunities to use these drug.

In most developed countries, the debate about cannabis policy is often simplified to a choice between two options: to legalize cannabis because its use is harmless, or to continue to prohibit its use because it is harmful. As a consequence, evidence that cannabis use causes harm to adolescents is embraced by supporters of cannabis prohibition and is dismissed as "flawed" by proponents of cannabis legalisation.

A major challenge in providing credible health education to young people about the risks of cannabis use is in presenting the information in a persuasive way that accurately reflects the remaining uncertainties about these risks. The question of how best to provide this information to young people requires research on their views about these issues and the type of information they find most persuasive. It is clear from US experience that it is worth trying to change adolescent views about the health risks of cannabis; a sustained decline in cannabis use during the 1980s was preceded by increases in the perceived risks of cannabis use among young people.

Cannabis users can become dependent on cannabis. The risk (around 10%) is lower than that for alcohol, nicotine, and opiates, but the earlier the age a young person begins to use cannabis, the higher the risk. Regular users of cannabis are more likely to use heroin, cocaine, or other drugs, but the reasons for this remain unclear. Some of the relationship is attributable to the fact that young people who become regular cannabis users are more likely to use other illicit drugs for other reasons, and that they are in social environments that provide more opportunities to use these drugs.

It is also possible that regular cannabis use produces changes in brain function that make the use of other drugs more attractive. The most likely explanation of theassociation between cannabis and the use of other illicit drugs probably involves a combination of these factors. As a rule of thumb, adolescents who use cannabis more than weekly probably increase their risk of experiencing psychotic symptoms and developing psychosis if they are vulnerable—if they have a family member with a psychosis or other mental disorder, or have already had unusual psychological experiences after using cannabis. This vulnerability may prove to be genetically mediated.

Questions

7. In paragraph 1, which of the following statements does not match the information on cannabis use?

a. The use of cannabis by teenagers has been increasing over the past 40 years.

b. Cannabis use has adverse effects on young people.

c. Withdrawal symptoms are more common in males.

d. People try cannabis for the first time at a younger age than previously.

8. Epidemiological studies in the 1980s & 1990s have found that....

a. 4% of the US population currently suffer from cannabis abuse or dependence.

b. starting cannabis use at a young age increases the risk of dependence or abuse.

c. only a minority of surveys researched treatment options for cannabis dependent people.

d. people who start cannabis use at a young age have high risk of becoming daily users.

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9. The main point of paragraph 3 is that...

a. alcohol, tobacco and cannabis can lead to the use of heroin and cocaine.

b. most adolescents who have used cocaine or heroin first try alcohol, followed by tobacco and then cannabis.

c. there is a clear link between habitual cannabis use and the use of heroin and cannabis.

d. the black market is the main source of illicit drugs.

10. Which of the following would be the most appropriate heading for paragraph 4?

a. Opinion on an effective cannabis policy is divided.

b. Cannabis use is harmful to adolescents and should be prohibited.

c. Cannabis use is a serious problem in a majority of developed countries.

d. Cannabis use should be legalised.

11. The word closest in meaning credible in paragraph 5 is...

a. believable

b. possible

c. high quality

d. inexpensive

12. Cannabis use in the US declined during the 1980s because...

a. parents were able to explain the health risks of cannabis use.

b. there was good health education regarding the health risks associated with cannabis use available at that time.

c. cannabis had increased in price

d. young people had became more worried about its effect on their health

13. The word relationship in paragraph 6 refers to the connection between...

a. legal drugs such as alcohol and nicotine and illegal drugs such as cannabis, cocaine and heroin.

b. cannabis use and dependency.

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c. the use of hard drugs such as heroin and cocaine and cannabis use.

d. regular users and their partners.

14. Which of the following statements best matches the information in the last paragraph?

a. Regular cannabis use produces changes in brain function.

b. Regular adolescent cannabis users with a genetic predisposition to mental disorders have an increased risk of encountering psychosis.

c. Regular adolescent users of cannabis are vulnerable to psychosis.

d. Occasional use of cannabis can make other drugs more appealing.

PART B

Seasonal Influenza Vaccination and The H1N1 Virus

As the novel pandemic influenza A (H1N1) virus spread around the world in late spring 2009 with a well-matched pandemic vaccine not immediately available, the question of partial protection afforded by seasonal influenza vaccine arose. Coverage of the seasonal influenza vaccine had reached 30%– 40% in the general population in 2008–09 in the US and Canada, following recent expansion of vaccine recommendations.

Unexpected Findings in a Sentinel Surveillance System

The spring 2009 pandemic wave was the perfect opportunity to address the association between seasonal trivalent inactivated influenza vaccine (TIV) and risk of pandemic illness. In an issue of PLoS Medicine, Danuta Skowronski and colleagues report the unexpected results of a series of Canadian epidemiological studies suggesting a counterproductive effect of the vaccine. The findings are based on Canada's unique near-real-time sentinel system for monitoring influenza vaccine effectiveness. Patients with influenza-like illness who presented to a network of participating physicians were tested for influenza virus by RT-PCR, and information on demographics, clinical outcomes, and vaccine status was collected. In this sentinel system, vaccine effectiveness may be measured by comparing vaccination status among influenza-positive "case" patients with influenza- negative "control" patients. This approach has produced accurate measures of vaccine effectiveness for TIV in the past, with estimates of protection in healthy adults higher when the vaccine is well-matched with circulating influenza strains and lower for mismatched seasons. The sentinel system was expanded to continue during April to July 2009, as the H1N1 virus defied influenza seasonality and rapidly became dominant over seasonal influenza viruses in Canada.

Additional Analyses and Proposed Biological Mechanisms

The Canadian sentinel study showed that receipt of TIV in the previous season (autumn 2008) appeared to increase the risk of H1N1 illness by 1.03- to 2.74-fold, even after adjustment for

the comorbidities of age and geography. The investigators were prudent and conducted multiple sensitivity analyses to attempt to explain their perplexing findings.

Importantly, TIV remained protective against seasonal influenza viruses circulating in April through May 2009, with an effectiveness estimated at 56%, suggesting that the system had not suddenly become flawed. TIV appeared as a risk factor in people under 50, but not in seniors—although senior estimates were imprecise due to lower rates of pandemic illness in that age group. Interestingly, if vaccine were truly a risk factor in younger adults, seniors may have fared better because their immune response to vaccination is less rigorous.

Potential Biases and Findings from Other Countries

The Canadian authors provided a full description of their study population and carefully compared vaccine coverage and prevalence of comorbidities in controls with national or province-level age-specific estimates—the best one can do short of a randomized study. In parallel, profound bias in observational studies of vaccine effectiveness does exist, as was amply documented in several cohort studies overestimating the mortality benefits of seasonal influenza vaccination in seniors. Given the uncertainty associated with observational studies, we believe it would be premature to conclude that TIV increased the risk of 2009 pandemic illness, especially in light of six other contemporaneous observational studies in civilian populations that have produced highly conflicting results. We note the large spread of vaccine effectiveness estimates in those studies; indeed, four of the studies set in the US and Australia did not show any association whereas two Mexican studies suggested a protective effect of 35%–73%.

Policy Implications and a Way Forward

The **alleged** association between seasonal vaccination and 2009 H1N1 illness remains an open question, given the conflicting evidence from available research. Canadian health authorities debated whether to postpone seasonal vaccination in the autumn of 2009 until after a second pandemic wave had occurred, but decided to follow normal vaccine recommendations instead because of concern about a resurgence of seasonal influenza viruses during the 2009–10 season. This illustrates the difficulty of making policy decisions in the midst of a public health crisis, when officials must rely on limited and possibly biased evidence from observational data, even in the best possible scenario of a well-established sentinel monitoring system already in place.

What happens next? Given the timeliness of the Canadian sentinel system, data on the association between seasonal TIV and risk of H1N1 illness during the autumn 2009 pandemic wave will become available very soon, and will be crucial in confirming or refuting the earlier Canadian results. In addition, evidence may be gained from disease patterns during the autumn 2009 pandemic wave in other countries and from immunological studies characterizing the baseline immunological status of vaccinated and unvaccinated populations. Overall, this perplexing experience in Canada teaches us how to best react to disparate and conflicting studies and can aid in preparing for the next public health crisis.

Questions

15. The question of partial protection against H1N1 arose...

a. before spring 2009

b. during Spring 2009

c. after spring 2009

d. during 2008-09

16. According to Danuta Skowronski....

a. the inactivated influenza vaccine may not be having the desired effects.

b. Canada's near-real-time sentinel system is unique.

c. the epidemiological studies were counterproductive

d. the inactivated influenza vaccine has proven to be ineffective.

17. The vaccine achieved higher rates of protection in healthy adults when....

a. it was supported by physicians.

b. the sentinel system was expanded.

c. used in the right season.

d. it was matched with other current influenza strains.

18. Which one of the following is closest in meaning to the word prudent?

a. anxious

b. cautious

c. busy

d. confused

19. The Canadian sentinel study demonstrated that.....

a. age and geography had no effect on the vaccine's effectiveness.

b. vaccinations on senior citizens is less effective than on younger people.

c. the vaccination was no longer effective.

d. the risk of H1N1 seemed to be higher among people who received the TIV vaccination.

ANJOORAN Connect

20. Which of the following sentences best summarises the writers' opinion regarding the uncertainty associated with observational studies?

a. More studies are needed to determine whether TIV increased the risk of the 2009 pandemic illness.

b. It is too early to tell whether the risk of catching the 2009 pandemic illness increased due to TIV.

c. The Australian and Mexican studies prove that there is no association between TIV and increased risk of catching the 2009 pandemic illness.

d. Civilian populations are less at risk of catching the 2009 pandemic illness.

21. Which one of the following is closest in meaning to the word alleged?

a. reported

b. likely

c. suspected

d. possible

22. Canadian health authorities did not postpone the Autumn 2009 seasonal vaccination because...

a. of a fear seasonal influenza viruses would reappear in the 2009-10 season.

b. there was too much conflicting evidence regarding the effectiveness of the vaccine.

c. the sentinel monitoring system was well established.

d. observational data may have been biased.

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ANSWERS

Part-A

1. C
2. B
3. D
4. A
5. D
6. B
7. C
8. Food containing saturated fat
9. Tumor
10. Gallstones
11. Surgery
12. Helps in digestion
13. Draining
14. Ideal body wesight
15. Cholestasis
16. liquids
17. meals
18. pain
19. gallstones
20. surgery

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Part b

1 . A
2. B
3. B
4. A
5. C
6. A

7. b

8.b

9. c

10. a

11. a

12. d

13. c

14.b

15. b

16. a

17. d

18. b

19. d

20. b

21. c

22.a

MANJOORAN Connect

Dry skin

TEXT A

Dry skin, which refers to roughened, flaky, or scaly skin that is less flexible than normal and dry to feel, is relatively common problem in all age groups, but is more common in elderly individuals. The water content of the stratum corneum is of paramount importance in maintaining the normal appearance and texture of human skin. The relative hydration of the stratum corneum is a composite of 3 factors viz. the rate of water transport from dermis to stratum corneum, the rate of surface loss of water and the rate of water binding ability of stratum corneum. Loss of integrity of the barrier function is a central factor in the development of dry skin conditions and eczema. The various factors involved in producing dry skin, various causes of dry skin and the role of emollients in the management of these conditions are discussed.

Dry skin is not a unique, well defined condition, but represents a medley of total; unrelated changes in the structure of the stratum corneum which is associated with decreased water content of the stratum corneum. Many causes car induce dry skin through a number of biologic pathways. Dry skin is a relatively common problem in all age groups but is more common it elderly individuals. Synonyms are xerosis, xeroderma aesteatosis, winter itch, dermatitis hiemalis, etc The self descriptive term, dry skin is morE preferable than the others.

Dry skin refers to roughened, flaky o scaly skin that is less flexible than normal and dry to the touch. Follicular accentuation is offer prominent on extensor surfaces and the condition is aggravated by cold, dry climates or Seasons.

MANIPURAN Connect

TEXT B

The surface of the human skin normally feels smooth and supple as opposed to the harsh and brittle scaly surface of dry skin and there is no visible scale on the surface. Many factors are responsible for maintaining the normal appearance of the skin. The relative hydration of the stratum corneum is a composite of 3 factors viz. the rate of water transport from dermis to stratum corneum, the rate of surface loss of water and the water binding ability of stratum corneum. Stratum corneum is soft and pliable when its water content reaches 10mg/100mg dry weight, corresponding to 60 percent relative humidity. As relative humidity increases, transepidermal water loss falls.Surface ventilation also accelerates water evaporation. Cold, dry and windy climates enhance water evaporation. Skin dryness has also been shown to worsen with increasing dew point.

Temperature and climate are important determining factors of the state of the normal skin which have their effects on the abnormally dry skin and in the hydration. There is a gradient between the water content of the stratum corneum and the atmosphere. Low relative humidity and high wind velocity tend to remove water from the stratum corneum and cause chafing or mild scaling even in most normal individuals. Dryness and scaling of the skin tend to improve spontaneously during the summer months due to heat and humidity and become more severe during the winter months.

TEXT C

Skin acts as a two-way barrier to prevent the inward or outward passage of water and electrolytes.This barrier function is largely localized to the stratum compactum of the horny layer,and depends on both cornified material of the keratinocytes and the intercellular lipids that have been released from the lamellar bodies (Odland bodies, membrane coating granules) in the underlying stratum granulosum.The intercellular lipids undergo enzymatic processing to produce a lipid mixture of ceramides, fatty acids and cholesterol. These molecules are arranged in a highly organized multilamellar fashion fusing with each other and the corneocytes to form the skin's lipid barrier. In addition the water soluble hygroscopic substances within the keratinocytes allow them to retain moisture and cause them to swell and press into one another, preventing cracks and fissures. The currently accepted view is that the proteinaceous keratinocytes are embedded within a continuous lipid-rich matrix and act as a barrier. The fibrous proteins of keratin and a histidin-rich protein known as keratohyalin or filaggrin are synthesized within the keratinocytes.An envelope, formed by cross-linking of the precursors-volucrin and keratohyalin around each keratinocyte, forms an exoskeleton and acts as a rigid scaffold for the internal keratin filaments. There is evidence that lipids are chemically bound to the surface of the corneocytes. Important intercellular stratum corneum lipids implicated in the barrier function are ceramides, free fatty acids and free sterols. Transepidermal water loss is necessary to stimulate the lipid synthesis. The major lipid by weight found in the stratum corneum is ceramide (sphingolipid). Ceramides possess the majority of the long-chain fatty acids and linoleic acid in the skin. Other lipids present in the stratum corneum are triglycerides, squaline, sterol wax/esters, n-alkanes and cholesterol sulphate. Cholesterol sulphate which comprises only 2-3% of the total lipids is important in corneocyte desquamation. It appears that corneocyte desquamotion is mediated through the desulfation of cholesterol sulphate.

TEXT D

The causes of dry skin are centered around three overlapping areas: lack of water in the stratum corneum, overactive epidermal turnover, and barrier damage. If the stratum corneum is desiccated, the result is dry skin. Chornic low level irritation either from chemicals, UV exposure, or in most dermatitides, can increase turnover of cells as part of the inflammatory process. When this occurs, there is inadequate time for the keratinocytes to differentiate properly and the appropriate lipids to be produced. The result is a defective barrier that cannot hold water and the result is dry skin. The third major cause of dry skin is barrier damage. When there is depletion of the intercellular lipids, such as with detergent exposure during dish washing, the barrier is damaged and the rate at which water traverses the skin from within is markedly increased. If the water cannot be stopped at the stratum corneum level, then desiccation of the outer surface of the skin occurs and produces dry skin.

There are multiple causes of dry skin. It may be acquired, constitutional and genetic in origin. It may be localized dry skin in each of the conditions of genetic origin is probably related to a single biochemical defect in the corneocytes. In that group keratins, filaggrin, keratolinin, involucrin, cholesterol sulphatase, membrane coating granules and so many other molecules and organelles play critical roles. Ichthyosis, the dry skin of anhydrotic type of congential ectodermal defect, and dry skin of atopic dermatitis-as a result of abnormal barrier function due to ceramide distribution (also observed in psoriasis) a genetic defect in the metabolism of essential fatty acids (lack of 6-6 desaturase) are examples of irreversible types of dryness of skin of genetic origin. Ichthyosis constitutes a heterogenous group of genetic and acquired disorders characterized by a generalized persistent non-inflammatory scaling of the skin surface. They are generally disorders of keratinization. Acquired dry skin may arise from normal (or sometimes even from greasy) skin, which is rendered temporarily and locally dry by external factors including solar (UV) radiation; exposures to extremes of climate: cold, heat, wind, dryness; exposure to chemicals: detergents; and various therapeutic measures(e.g., retinoids).

Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

DRY SKIN

Questions 1-7

For each question, **1-7**, decide which text (**A**, B, C or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. Proteinaceous keratinocytes are embedded within a continous lipid-rich matrix & act as barrier

2. Dry skin is less flexible than normal skin

3. Genetic and acquired disorders are usually disorders of keratinization

4. Dry skin is more common in elderly people

5. Exposure to detergents can lead to barrier damage

6. Provision of fresh air to the surface precipitation water evaporation

7. Loss of water through transepidermal is very crucial to refresh lipid synthesis

MANJOORAN Connect

Questions 8-14

Answer each of the questions, **8-14**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. The cross linking of the precursors forms an instument of execution for?

9. What are two common features of dry skin?

10. What is sphingolipid?

11. What is the ultimate effect on skin by environmental factors?

12. What happens to the stratum corneum of the skin?

13. What is the main component to retain appearence and texture of human skin?

14. What helps in preventing cracks and fissures?

Questions 15-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

15. A healthy human skin usually _____ & _____

16. Ichthyosis is a type of _____ requires urgent.

17. The rate of _____ capability of stratum corneum is a contributing factor for it's hydration

18. _____ contains majority of long chain fatty acids and linoleic acid in the skin

19. Follicular accentuation is mainly notable _____

20. Etiological factors for dry skin can be acquired _____ and hereditary

MANJORAN Connect

Functional magnetic resonance imaging

Functional magnetic resonance imaging (fMRI), neuroimaging technique used in biomedical research and in diagnosis that detects changes in blood flow in the brain. This technique compares brain activity under resting and activated conditions. It combines the high-spatial- resolution noninvasive imaging of brain anatomy offered by standard magnetic resonance imaging (MRI) with a strategy to distinguish between the magnetic resonance states of hemoglobin in the presence or absence of oxygen. This enables the detection of increases in blood oxygen levels when brain activity brings fresh blood to a particular area of the brain. Functional magnetic resonance imaging allows for the generation of detailed maps of brain areas that underlie human mental activities in health and disease. This technique has been applied to the study of various functions of the brain, ranging from primary sensory responses to cognitive activities.

1. Functional magnetic resonance imaging FMRI is the study of
   a. Function of brain
   b. Chemical reaction in brain
   c. None of the above

MANJOORAN Connect

Amniocentesis

Amniocentesis, the surgical insertion of a hollow needle through the abdominal wall and into the uterus of a pregnant female and the aspiration of fluid from the amniotic sac for analysis. Examination of the amniotic fluid itself as well as the fetal cells found in the fluid can reveal such things as fetal sex (the significant factor in inherited diseases that are sex-linked), chromosomal abnormality, and other types of potential problems. The procedure, generally carried out in the 15th to 17th week of gestation, is relatively painless and can be carried out under local anesthesia. Once used only in medical management of erythroblastosis fetalis (a blood disorder of the fetus and newborn that is caused by antibodies in the mother's blood), amniocentesis was first performed in the 1930s. More than 50 metabolic diseases can now be diagnosed by use of the procedure, but it is used most often for the identification of chromosomal anomalies and neural tube defects. It is also often recommended for women 35 years or older and for women who have experienced three or more spontaneous abortions.

2. Functional magnetic resonance imaging FMRI is the study of
   a. Antibodies of mother's blood
   b. Blood disorder of newborn
   c. Antibodies of fetus

Thyroid function test

Thyroid function test, any laboratory procedure that assesses the production of the two active thyroid hormones, thyroxine (T 4) and triiodothyronine (T 3), by the thyroid gland and the production of thyrotropin (thyroid-stimulating hormone, TSH), the hormone that regulates thyroid secretion, by the pituitary gland. The best and most widely used tests are measurements of serum thyrotropin and thyroxine. The secretion of thyrotropin changes substantially in response to very small changes in thyroxine and triiodothyronine production. For example, small decreases in thyroid hormone production result in relatively large increases in serum concentrations of thyrotropin, and, conversely, small increases in thyroxine and triiodothyronine production result in relatively large decreases in serum concentrations of thyrotropin. Therefore, patients with hypothyroidism (thyroid deficiency) almost invariably have not only low serum thyroid hormone but also high serum thyrotropin concentrations, and those with hyperthyroidism have high serum thyroid hormone and low serum thyrotropin concentrations. An exception is patients with pituitary disease and thyrotropin deficiency, who have low serum thyroid hormone but normal or low serum thyrotropin concentrations. Between the two thyroid hormones, measurements of serum thyroxine are preferred because serum triiodothyronine concentrations are abnormal in many patients with nonthyroid illnesses.

3. What is the function of thyroid stimulating hormone
    a. Controls thyroid secretion by pituitary gland
    b. Inhibit thyroid hormone by pituitary gland
    c. Prohibit the production of pituitary gland

MANJOORAN Connect

Anthrax Pathogenesis.

Anthrax is caused by the spore-forming, gram-positive bacterium Bacillus anthracis. The bacterium's major virulence factors are (a) the anthrax toxins and (b) an antiphagocytic polyglutamic capsule. These are encoded by two large plasmids, the former by pXO1 and the latter by pXO2. The expression of both is controlled by the bicarbonate-responsive transcriptional regulator, AtxA. The anthrax toxins are three polypeptides-protective antigen (PA), lethal factor (LF), and edema factor (EF)-that come together in binary combinations to form lethal toxin and edema toxin. PA binds to cellular receptors to translocate LF (a protease) and EF (an adenylate cyclase) into cells. The toxins alter cell signaling pathways in the host to interfere with innate immune responses in early stages of infection and to induce vascular collapse at late stages. This review focuses on the role of anthrax toxins in pathogenesis. Other virulence determinants, as well as vaccines and therapeutics, are briefly discussed.

4. How does the toxins affect innate immune response
    a. By modifying cell signaling pathway
    b. By interfering the pathway
    c. By combining the signals pathway

MANJOORAN Connect

Influenza Virus

Influenza viruses are significant human respiratory pathogens that cause both seasonal, endemic infections and periodic, unpredictable pandemics. The worst pandemic on record, in 1918, killed approximately 50 million people worldwide. Human infections caused by H5N1 highly pathogenic avian influenza viruses have raised concern about the emergence of another pandemic. The histopathology of fatal influenza virus pneumonias as documented over the past 120 years is reviewed here. Strikingly, the spectrum of pathologic changes described in the 1918 influenza pandemic is not significantly different from the histopathology observed in other less lethal pandemics or even in deaths occurring during seasonal influenza outbreaks.

5. The worst pandemic record of influenza virus in 1918 was
    a. Nearly 50 million people
    b. About 50 million people
    c. Over 50 million people

Myopia

In the article shown is the modern diagnostic method of progressive myopia pathogenesis. Establishment of possible myopia progress using the ultrasonographic biometry while looking into and into and down contact equator at one and the same point. The research goal is to study the residual deformative changes developed as a result of sclera distraction during reading. We have researched 150 patients. The patients were split in three groups according to the age and myopia type: I group included patients of the age of 5-12 years; the II group had patients of the age of 13-19 years; III group - patients above the 19 years. The used age differentiation is based on the eye anatomic growth criteria. The research was held by the following scheme: all the researches patients underwent the preliminary anesthesia drops instillation; the ultrasonographic biometry catheter was put using one and the same equator point. In the first position they established the front and back axis size, in the second one - the eye size was established in the medial look regime; in the third position – the eye size was established in the reading regime. The data obtained are as follows: among the first group patients, aged from 5 to 12 years, in reading regime the eye size was increased in 93% cases; among the second group patients, aged from 13 to 19 years, in reading regime the eye size was increased in 90% cases; among the third group patients, aged above 19 years - 91% cases. The reason why the sclera capsule distraction during reading regime is that of the extraocular muscles and growth of the intraocular pressure influence the process. Because the sclera is fibrotic membrane, it is characterized with well-marked elasticity, stronger accommodation and the sigh load in the near distance causes sclera capsule distraction. After the contraction the residual deformation stays there. Accumulation of such residual deformations causes the sclera weakness and formation of myopia as a disease. According to the data obtained, it is necessary to work out the certain recommendations, from the sclera nutrition and other medical preparations standpoint. Improvement of the sclera nutrition will slacken the residual changes in sclera, and this will significantly decrease the myopic disease advancing.

6. The residual changes in sclera Is reduced by
    a. Improving sclera
    b. Decreasing intraocular pressure
    c. None of the above

PART C

A Hormonal Disorder – Adrenal Insufficiency

Adrenal insufficiency is an endocrine or hormonal disorder that occurs when the adrenal glands do not produce enough of certain hormones. The adrenal glands are located just above the kidneys. Adrenal insufficiency can be primary or secondary. Primary adrenal insufficiency, also called Addison's disease, occurs when the adrenal glands are damaged and cannot produce enough of the hormone cortisol, and often the hormone aldosterone. Addison's disease affects one to four of every 100,000 people, in all age groups and both sexes.

Secondary adrenal insufficiency occurs when the pituitary gland, a bean-sized organ in the brain, fails to produce enough adrenocorticotropin (ACTH), a hormone that stimulates the adrenal glands to produce cortisol. If ACTH output is too low, cortisol production drops. Eventually, the adrenal glands can shrink due to lack of ACTH stimulation. Secondary adrenal insufficiency is much more common than Addison's disease.

Addison's disease symptoms usually develop slowly, often over several months, and may include: muscle weakness and fatigue, weight loss and decreased appetite, darkening of skin (hyperpigmentation), low blood pressure (even fainting), salt cravings, low blood sugar (hypoglycemia), nausea, diarrhea or vomiting, muscle or joint pains etc. Sometimes, however, the signs and symptoms of Addison's disease may appear suddenly. In acute adrenal failure (addisonian crisis), the signs and symptoms may also include: pain in your lower back, abdomen or legs, severe vomiting and diarrhea, leading to dehydration, low blood pressure and loss of consciousness.

Your adrenal glands are composed of two sections: the interior (medulla) produces adrenaline-like hormones; the outer layer (cortex) produces a group of hormones called corticosteroids, which include glucocorticoids, mineralocorticoids and male sex hormones (androgens).

Some of the hormones the cortex produces are essential for life (glucocorticoids and mineralocorticoids). Glucocorticoids: These hormones, which include cortisol, influence your body's ability to convert food fuels into energy, play a role in your immune system's inflammatory response and help your body respond to stress. Mineralocorticoids: These hormones, which include aldosterone, maintain your body's balance of sodium and potassium to keep your blood pressure normal.

Androgens, male sex hormones, are produced in small amounts by the adrenal glands in both men and women. They cause sexual development in men and influence muscle mass, libido and a sense of well-being in men and women.

Primary adrenal insufficiency

Addison's disease occurs when the cortex is damaged and doesn't produce its hormones in adequate quantities. Doctors refer to the condition involving damage to the adrenal glands as primary adrenal insufficiency. The failure of your adrenal glands to produce adrenocortical hormones is most commonly the result of the body attacking itself (autoimmune disease). For unknown reasons, your immune system views the adrenal cortex as foreign, something to attack and destroy. Other causes of adrenal gland failure may include: tuberculosis, other infections of the adrenal glands, spreading of cancer to the adrenal glands and bleeding into the adrenal glands.

Secondary adrenal insufficiency

Adrenal insufficiency can also occur if your pituitary gland is diseased. The pituitary gland produces a hormone called adrenocorticotropic hormone (ACTH), which

stimulates the adrenal cortex to produce its hormones. Inadequate production of ACTH can lead to insufficient production of hormones normally produced by your adrenal glands, even though your adrenal glands aren't damaged. Doctors call this condition secondary adrenal insufficiency.

Another more common cause of secondary adrenal insufficiency occurs when people who take corticosteroids for treatment of chronic conditions, such as asthma or arthritis, abruptly stop taking the corticosteroids.

If you have untreated Addison's disease, an addisonian crisis may be provoked by physical stress (such as an injury), infection or illness. All treatment for Addison's disease involves hormone replacement therapy to correct the levels of steroid hormones your body isn't producing. Some options for treatment include: oral corticosteroids, corticosteroid injections and androgen replacement therapy.

Questions:

7. Addison's disease effects

A. four of every 1,000 people

B. one of every 1,000 people, including all age-groups

C. one to four of every 1,000 people, including all age groups

D. one to four of every 100,000 people

8. Secondary adrenal insufficiency occurs due to one of these reasons

A more production of adrenocorticotropin (ACTH)

B more production of cortisol

C low production of aldosterone

D low production of ACTH

9. According to the passage(s), symptoms of Addison's disease occur

A slowly

B suddenly

C after a month

D slowly after many months

MANJOORAN Connect

10. Cortex produces

A androgens

B glucoscorticods

C mineralocorticods

D all of the above

11. Aldosterone maintains

A body balance

B balance of sodium and potassium

C high BP

D low BP

12. According to the information given in the passage, the statement that "Androgen influences muscle mass and physical and mental nature of men and women" is

A true

B false

C can't say

D not given in the passage(s)

13. Doctors refer to the damage to the cortex of the kidney as

A primary adrenal insufficiency

B secondary adrenal insufficiency

C other fatal infections

D not given

14. In a condition like secondary adrenal insufficiency

A the kidney is damaged

B adrenal glands are damaged

C the kidney is slightly damaged

D adrenal glands are not damaged

MANJOORAN Connect

Carpal Tunnel Syndrome

Carpal tunnel syndrome is a condition that may be caused by repeatedly performing stressful motions with your hand or holding your hand in the same position for long periods of time. CTS is classified as a cumulative trauma disorder, an ailment that attacks the body's musculoskeletal system. The musculoskeletal system is made up of muscles that pull on tendons and move the bones at joints. The joints are held together by ligaments. Carpal tunnel syndrome specifically affects the sensitive nerves of - and the blood supply that feeds - the hands and wrists.

Carpal tunnel syndrome has been around for a long time; meat packers began complaining of pain and loss of hand function in the 1860s. Back then, these complaints were largely attributed to poor circulation. The nature of work has changed over the years; today, more jobs are highly specialized and require use of only a small number of muscles repeatedly. With the growing numbers of people using computers and keyboards, plus the focus on better health-care for workers, carpal tunnel syndrome is of real concern to both employers and health-careprofessionals. Recent studies have shown that carpal tunnel syndrome, like all other cumulative trauma disorders, is on the rise while other workplace injuries have leveled off. Many companies are turning to physical therapists for help with designing and implementing health promotion and injury prevention programs to protect their employees from CTS.

People with CTS usually experience feelings of numbness, weakness, tingling, and burning in their fingers and hands. If not treated, the symptoms may escalate into acute, persistent pain. CTS can become so crippling that people can no longer do their work or even perform simple tasks at home. At its most extreme, carpal tunnel syndrome forces people to undergo surgery and miss many days of work, or prevents them from working at all because their hand functions are permanently impaired.

Carpal tunnel syndrome occurs in men and women of all ages, and is often found in workers whose tasks require repeating the same motion in the fingers and hand for long periods of time. CTS has surfaced among meat packers, assembly line workers, jackhammer operators, and employees who spend hours working at a computer or typewriter. Carpal tunnel syndrome shows up in athletes as well as homemakers.

The U.S. Department of Labor has cited carpal tunnel syndrome, as well as other cumulative trauma disorders, as the cause of 48 percent of all industrial workplace illnesses. The disease affects more than five million Americans.

CTS's impact on American businesses is devastating. It shows up in the workplace in the form of fatigue, poor work performance, discomfort and pain, and poor employer/employee relations. The high cost of treatment for an employee with CTS, plus the lost productivity when that employee is absent for a long period of time, strains the company's ability to operate efficiently and can lead to morale problems

when other employees have to take over the absent workers' responsibilities.

Physical therapists with specialized training in cumulative trauma disorders have been working in industrial and corporate settings for many years to meet the health-care needs of America's workforce. They work closely with employers to educate employees about CTS—what causes it and how to avoid it through proper use of the musculoskeletal system.

Physical therapists can target and correct poor work habits and improper work designs, such as tools, furniture, equipment, and work space. They can also assess the potential risks of an individual and determine if that person is physically unsuited to a particular job. Among their many responsibilities, physical therapists teach health awareness and job safety.

A typical education program includes exercises employees can do at work and at home, adjustments to the overall work environment and individual work stations, plus early detection of symptoms to avoid painful and costly surgery.

Physical therapists also work with employers and their engineering departments to design and modify the work environment, helping to remove the causal factors of CTS. If anyone has symptoms of carpal tunnel syndrome, then consulting a physical therapist or other qualified health-care practitioner for an evaluation and individualized treatment is always recommended.

MANJOORAN Connect

15. According to the passage, CTS

A is a cumulative trauma disorder

B is caused due to weakness in musculoskeletal system

C occurs due to weakness in ligaments between joints

D all of the above

16 According to the information given, CTS

A is on the rise

B is on the rise without any other cumulative trauma disorders

C is one of the common cumulative trauma disorders that is on rise

D is on the rise with many other cumulative trauma disorders

17 "In CTS, ... may become permanently impaired."

A carpels

B hands

C feet

D wrists

18 CTS often occurs in

A men

B women

C men and women of all ages

D only men of all ages

19 .....complained of pain and loss of hand function in the 1860s

A meat packers

B assembly line workers

C jackhammer operators

D employees who spend hours at a computer

20 CTS accounts for .....of all industrial workplace illnesses

A 50 %

B 48%

C 84%

D 60%

21 Physical therapists teach employees about

A proper use of the musculoskeletal system

B how to avoid CTS

C how to increase efficiency at work

D health-care needs

22 One of the impacts of CTS on American businesses

A low productivity because of absence of workers due to CTS

B absence of workers due to CTS condition

C unfair employee treatment

D none

MANJOORAN Connect

PART A

1.C

2.A

3.D

4.A

5.D

6.B

7.C

8.Internal keratin filaments

9.harsh and brittle scaly surface

10.a lipid/major lipid of sratum corneum

11.chafting or mild scaling

12.de siccation

13.water content

14.water soluable hygroscopic substances

15.smooth and supple

16.congenital ectodermal defect

17.water binding

18.ceramides

19.on extensor surfaces

20.constitutional

ANJOORAN Connect

PART B

1.A

2.A

3.A

4.A

5.A

6.A

7. C
8. D
9. D
10. A
11. B
12. B
13. A
14. D
15. A
16. D
17. D
18. C
19. A
20. B
21. B
22. A

MANJOORAN Connect

Exchange Transfusion

TEXT A

An exchange transfusion is a blood transfusion in which the patient's blood or components of it are exchanged with (replaced by) other blood or blood products.
Most simple blood transfusion involve adding blood or blood products without removing any blood. In contrast, an exchange transfusion can be via manual removal of blood followed by replacement or via a machine (apheresis).
exchange transfusion is used in the treatment of a number of diseases, including sickle-cell disease, thrombotic thrombocytopenic purpura, and hemolytic disease of the newborn. Partial exchange might be required for polycythemia.
Nearly all exchange transfusions are allogeneic (that is, the new blood or blood products come from another person or persons, via donated blood); autologus exchange transfusion is possible (using autologous blood banking), but there are not many situations in which a need for it arises, as most autologous transfusions involve no exchange.

TEXT B

An exchange transfusion reverses or counteracts the symptoms of jaundice or other blood diseases, such as sickle cell anemia.
Jaundice is a blood disease that's fairly common in newborns during the first few weeks of life. It causes a yellow discoloration of their skin and whites of their eyes. Jaundice is a result of an excess of a chemical called bilirubin in the body.
Sickle cell disease(SCD) is a group of blood disorders that cause red blood cells to stiffen and become crescent-shaped. This shape impedes their flow through the circulatory system and causes blockages in capillaries. According to the centers for Disease control and prevention, one in every 500 African-American babies in the United States is born with SCD.

MANJOORAN Connect

TEXT C

The following guidelines for exchange transfusion levels are based on the American Academy of Pediatric Guidelines and are adapted from the Department of Human Services (Victoria) Neonatal Handbook.

GUIDELINES FOR EXCHANGE TRANSFUSION IN INFANTS 35 OR MORE WEEKS OF GESTATION

Age (hrs)	**Infants at higher risk** 35-37^{+6} weeks + risk factors	**Infants at medium risk** ≥38 weeks + risk factors or 35-37^{+6} weeks and well	**Infants at lower risk** 38 weeks and well
	SBR (micromol/L)	**SBR (micromol/L)**	**SBR (micromol/L)**
Birth	200	235	270
12 hours	230	255	295
24 hours	255	280	320
48 hours	290	320	375
72 hours	315	360	405
96 hours	320	380	425
5 days	320	380	425
6 days	320	380	425
7 days	320	380	425

TEXT D

Preparation of the Infant

Medical staff should discuss the procedure with the parents/guardian and obtain consent

Advise AUM and Consultant Neonatologist on duty as soon as decision to exchange is made

Exclusively allocate at least one doctor and one nurse to care of the infant throughout the procedure

When an exchange transfusion is taking place the Consultant Neonatologist on duty should be present on the unit to provide support and to troubleshoot issues so that the Fellow or Registrar can carry out the procedure without interruption

Ensure resuscitation equipment and medications are easily accessible

Nurse infant under radiant warmer for accessibility

Ensure infant is comfortable and settled – sedation and pain relief are not usually required unless the infant

is active and likely to compromise line stability or sterile field

Ensure full cardio-respiratory monitoring is initiated and document full set of baseline observations (temperature, respiratory and heart rate, blood pressure and oxygenation)

Infant should be nil orally as soon as decision is made to perform exchange transfusion.

Pass oro/nasogastric tube and aspirate stomach contents. Leave tube in-situ and on free drainage for duration of procedure

Before commencing exchange transfusion collect blood samples for required baseline bloods and any specific testing required. Tests may include (but not be limited to) blood cultures, blood gas, serum bilirubin, blood glucose, FBC, UEC, LFT, newborn screening test, haematological, chromosome or metabolic studies

Establish vascular access for procedure if not already in-situ (see RCH Clinical Practice Guideline "Central Vascular Access Devices Insertion and Management") depending on

whether the procedure will be performed via arterial and venous access or via single venous access Check blood as per RCH Procedure "Blood Transfusion"

Part A

TIME: 15 minutes

- Look at the four texts, **A-D**, in the separate **Text Booklet.**
- For each question, **1-20**, look through the texts, **A-D**, to find the relevant information.
- Write your answers on the spaces provided in this **Question Paper**.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Exchange Transfusion

Questions 1-7

For each question, **1-7**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1. Prevalance of sickle cell disease
2. Possibility of autologous blood transfusion
3. Infant prepration details
4. Infants blood transfusion guidelines
5. Full set of baseline informations during blood transfusion
6. Aspiration of stomach content prior to procedure
7. Jaundice in newborn and the reason are fairly common during first weeks

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Questions 8-13

Answer each of the questions, **8-13**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

8. With whom the medical staff should ask for consent of an infant?

9. NBM of the baby should started at what time?

10. What is the protocol of doing blood checking?

11. What is sickle cell disease?

12. Incident rate of 5 cd in African-American babies?

13 . What is necessary to carry out the blood transfusion without interuption?

Questions 14-20

Complete each of the sentences, **15-20**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both

MANJOORAN Connect

14. An exchange transfusion is a blood transfusion where the blood and blood components are replaced with _____ or _____

15. Jaundice is fairly common in _____ during the first weeks of life

16. Crescent-shape of red blood cells impedes blood flow through _____ and causes_____.

17. Procedure consent should be obtained from infants_____

18. _____ and_____ are not usually used unless the infant is active

19. Before commencing exchange transfusion collection of _____ is necessary

20. Blood checking should be according to _____ .

Part B

In this part of the text, there are 6 short extracts relating to the work of health professionals .

For questions 1-6 choose the answer (A,B or C) which you think fits best according to the text .

1.Inception of EA2 occurs in :

A. poroxymal attack

B. 2-32 years

C. stress which is trigerred

Episodic Ataxia Type 2
Episodic ataxia type 2 (EA2) is characterized by paroxysmal attacks of ataxia, vertigo, and nausea typically lasting minutes to days in duration. Attacks can be associated with dysarthria, diplopia, tinnitus, dystonia, hemiplegia, and headache. About 50% of individuals with EA2 have migraine headaches. Onset is typically in childhood or early adolescence (age range 2-32 years). Frequency of attacks can range from once or twice a year to three or four times a week. Attacks can be triggered by stress, exertion, caffeine, alcohol, fever, heat, and phenytoin; they can be stopped or decreased in frequency and severity by administration of acetazolamide or 4-aminopyridine. Between attacks, individuals may initially be asymptomatic but commonly develop interictal findings that can include nystagmus, pursuit and saccade alterations, and ataxia

2.Usually,epithelioid sarcoma presents in the beginning as:

A. deleterious

B. wholesome

C. pernicious

Epithelioid Sarcoma
Epithelioid sarcoma is a rare, highgrade, soft tissue tumor that has a known propensity for local recurrence, regional lymph node involvement, and distant metastases. We review the clinical and histological presentations of epithelioid sarcoma. Because epithelioid sarcoma presents innocuously, it is often mistaken as a benign process, which can result in insufficient treatment. Therefore, we emphasize the need for clinicians to consider this diagnosis when a slowgrowing tumor is found on the distal extremity of a young male as the malignancy inherently portends a poor prognosis. Prognostic factors, such as local recurrence, regional metastatic disease, and tumor width, are discussed along with current treatment modalities, which include radical excision, sentinel lymph node biopsy, and radiation.

3.One of the probability factors that associated with skin to cause Erysipelas and cellulitis is

A. bacteria

B. skin damage

C. weak immune system diabetes

Erysipelas and cellulitis
Erysipelas and cellulitis are skin infections that can develop if bacteria enter the skin through cuts or sores. Both infections make your skin swell, become red and tender. Erysipelas (also known as St. Anthony's fire) usually only affects the uppermost layers of skin, while cellulitis typically reaches deeper layers of tissue. Provided the right treatment is started early enough, these infections usually clear up without any lasting effects. Left untreated, they sometimes lead to serious complications. Erysipelas is often caused by streptococcus bacteria, while cellulitis is typically caused by staphylococcus bacteria. Both types of bacteria may cause either erysipelas or cellulitis, though.Bacterial skin infections are more likely to arise if the surface of your skin is damaged, making it easier for bacteria to enter. Because of this, the risk factors include skin problems such as eczema, impetigo, fungal infections such as athlete's foot, or wounds and ulcers. Erysipelas or cellulitis can also develop following injuries, pin pricks, and insect or animal bites, or if germs get into the wound during an operation.The risk of infection is particularly high if you have a weakened immune system. The immune system may become weaker due to things like taking certain medications. These include some cancer drugs, corticosteroids and medication commonly used following organ transplants to suppress the body's immune response.The risk is also higher in people who are overweight or have diabetes, problems with the circulation of lymph or blood, or have venous insufficiency. Previous erysipelas or cellulitis is considered to be a risk factor too.

MANJOORAN Connect

4.Occurence of Esotropia is

A. is rare

B very common

C. affecting averagely

Esotropia
Esotropia is the most common type of strabismus, accounting for more than half of strabismus cases in children.[1] The incidence is about 1% over 10 years for patients under 19 years of age.[1] Risk factors for esotropia in children include premature birth, maternal smoking during pregnancy and refractive errors. If the amount of deviation is the same regardless of the direction of the patient's gaze, the esotropia is comitant. Comitant esotropia is commonly associated with refractive error, abnormal accommodation or sensory deprivation as a result of conditions such as cataract or retinoblastoma. [3] If the amount of deviation varies with the direction of the gaze, the esotropia is incomitant. These deviations may be related to mechanical restriction (orbital mass or fracture, thyroid dysfunction), or innervation abnormalities (palsy of the abducens nerve, myasthenia gravis).

5.The only treatment modality of un manageable case of this disease include

A. splenectomy

B. stem – cell transplantation

C. immune suppressive drugs

Management of Evans syndrome.
Evans syndrome is an uncommon condition defined by the combination (either simultaneously or sequentially) of immune thrombocytopenia (ITP) and autoimmune haemolytic anaemia (AIHA) with a positive direct antiglobulin test (DAT) in the absence of known underlying aetiology. This condition generally runs a chronic course and is characterised by frequent exacerbations and remissions. First-line therapy is usually corticosteroids and/or intravenous immunoglobulin, to which most patients respond; however, relapse is frequent. Options for second-line therapy include immunosuppressive drugs, especially ciclosporin or mycophenolate mofetil; vincristine; danazol or a combination of these agents. More recently a small number of patients have been treated with rituximab, which induces remission in the majority although such responses are often sustained for <12 months and the long-term effects in children are unclear. Splenectomy may also be considered although long-term remissions are less frequent than in uncomplicated ITP. For very severe and refractory cases stem cell transplantation (SCT) offers the only chance of long-term cure. The limited data available suggest that allogeneic SCT may be superior to autologous SCT but both carry risks of severe morbidity and of transplant-related mortality. Cure following reduced-intensity conditioning has now been reported and should be considered for younger patients in the context of controlled clinical trials

6.One of the important element to improve the survival of patients with this disease

A. common histogenesis

B. Defining Ewings Sarcoma

C. classification of patients for risk

Ewing's sarcoma
The identification of the non-random chromosome rearrangements between the EWS gene on chromosome 22q12 and members of the ETS gene family in Ewing's sarcoma, peripheral primitive neuroectodermal tumour, Askin tumour, and neuroepithelioma has been a key advance in understanding their common histogenesis and defining the Ewing's sarcoma family of tumours (ESFT). In addition to improvements in diagnosis and potentially the stratification of patients for risk, biological investigations of these gene fusions may define targets for much needed therapeutic strategies to eliminate minimal residual disease or metastatic disease. Insight into their relation with other oncogenic events in ESFT will advance risk group analysis and ultimately may improve clinical management and survival for patients with this disease

Fluoride

Globalization has provoked changes in many facets of human life, particularly in diet. Trends in the development of dental caries in population have traditionally followed developmental patterns where, as economies grow and populations have access to a wider variety of food products as a result of more income and trade, the rate of tooth decay begins to increase. As countries become wealthier, there is a trend to greater preference for a more "western" diet, high in carbohydrates and refined sugars. Rapid globalization of many economies has accelerated this process. These dietary changes have a substantial impact on diseases such as diabetes and dental caries.

The cariogenic potential of diet emerges in areas where fluoride supplementation is inadequate. Dental caries is a global health problem and has a significant negative impact on quality of life, economic productivity, adult and children's general health and development. Untreated dental caries in pre-school children is associated with poorer quality of life, pain and discomfort, and difficulties in ingesting food that can result in failure to gain weight and impaired cognitive development. Since low- income countries cannot afford dental restorative treatment and in general the poor are most vulnerable to the impacts of illness, they should be afforded a greater degree of protection.

By WHO estimates, one third of the world's population have inadequate access to needed medicines primarily because they cannot afford them. Despite the inclusion of sodium fluoride in the World Health Organization's Essential Medicines Model List, the global availability and accessibility of fluoride for the prevention of dental caries remains a global problem. The optimal use of fluoride is an essential and basic public health strategy in the prevention and control of dental caries, the most common non- communicable disease on

the planet. Although a whole range of effective fluoride vehicles are available for fluoride use (drinking water, salt, milk, varnish, etc.), the most widely used method for maintaining a constant low level of fluoride in the oral environment is fluoride toothpaste.

More recently, the decline in dental caries amongst school children in Nepal has been attributed to improved access to affordable fluoride toothpaste. For many low-income nations, fluoride toothpaste is probably the only realistic population strategy for the control and prevention of dental caries since cheaper alternatives such as water or salt fluoridation are not feasible due to poor infrastructure and limited financial and technological resources. The use of topical fluoride e.g. in the form of varnish or gels for dental caries prevention is similarly impractical since it relies on repeated applications of fluoride by trained personnel on an individual basis and therefore in terms of cost cannot be considered as part of a population based preventive strategy.

The use of fluoride toothpaste is largely dependent upon its socio-cultural integration in personal oral hygiene habits, availability and the ability of individuals to purchase and use it on a regular basis. The price of fluoride toothpaste is believed to be too high in some developing countries and this might impede equitable access. In a survey conducted at a hospital dental clinic in Lagos, Nigeria 32.5% of the respondents reported that the cost of toothpaste influenced their choice of brands and 54% also reported that the taste of toothpastes influenced their choice.

Taxes and tariffs on fluoride toothpaste can also significantly contribute to higher prices, lower demand and inequity since they target the poor. Toothpastes are usually classified as a cosmetic product and as such often highly taxed by governments. For example, various taxes such as excise tax, VAT, local taxes as well as taxation on the ingredients and packaging contribute to 25% of the retail cost of toothpaste in Nepal and India, and 50% of the retail price in Burkina Faso. WHO continues to recommend the removal taxes and tariffs on fluoride toothpastes. Any lost revenue can be restored by higher taxes on sugar and high sugar containing foods, which are common risk factors for dental caries, coronary heart disease, diabetes and obesity.

The production of toothpaste within a country has the potential to make fluoride toothpaste more affordable than imported products. In Nepal, fluoride toothpaste was limited to expensive imported products. However, due to successful advocacy for locally manufactured fluoride toothpaste, the least expensive locally manufactured fluoride toothpaste is now 170 times less costly than the most expensive import. In the Philippines, local manufacturers are able to satisfy consumer preferences and compete against multinationals by discounting the price of toothpaste by as much as 55% against global brands; and typically receive a 40% profit margin compared to 70% for multinational producers.

In view of the current extremely inequitable use of fluoride throughout countries and regions, all efforts to make fluoride and fluoride toothpaste affordable and accessible must be intensified. As a first step to addressing the issue of affordability of fluoride toothpaste in the

poorer countries in-depth country studies should be undertaken to analyze the price of toothpaste in the context of the country economies.

Questions

7. Which'of'the'following'would'be'the'most'appropriate'heading'for'

the'paragraph'1?

a. High sugar intake and increasing tooth decay

b. Globalisation, dietary changes and declining dental health

c. Dietary changes in developing nations

d. Negative health effects of a western diet

8. Which of the following is not mentioned as a negative effect of' untreated dental caries in pre school children?

a. Decreased mental alertness

b. Troubling chewing and swallowing food

c. Lower life quality

d. Reduced physical development

9. According'to'paragraph'3,'which'of'the'following'statements'is'

correct?

a. Dental caries is the most contagious disease on earth.

b. Fluoride in drinking water is effective but rarely used

c. Fluoride is too expensive for a large proportion of the global

population.

d. Fluoride toothpaste is widely used by 2/3 of the world's

population.

10. Fluoride'toothpaste'is'considered'the'most'effective'strategy'to'

Reduce dental caries in low income countries because.....

a. it is the most affordable.

b. topical fluoride is unavailable.

c. it does not require expensive infrastructure or training.!

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d. it was effective in Nepal.

11. Which'of'the'following'is'closest'in'meaning'to'the'word'impede?

a. stop

b. prevent

c. hinder

d. postpone

12. Regarding'the'issue'of'taxation'in'paragraph'6'which'of'the'following'

Statements is most correct?

a. Income tax rates are higher in Burkina Faso than India or Nepal.

b. WHO recommends that tax on toothpaste be reduced

c. Governments would like to reduce tax on toothpastes but can't as

it is classified as a cosmetic.

d. WHO suggests taxing products with a high sugar content instead of

toothpastes.

13. Which'of'the'following'is'closest'in'meaning'to'the'word'advocacy?

a. marketing

b. demand

c. development

d. support

14. Statistics'in'paragraph'7'indicate'that....

a. local products can't compete with global products and make a profit at the same time.

b. Philippine produced toothpaste is profitable while being less than

half the price of global brands.

c. in Nepal, fluoride toothpaste is limited to imported products which

are very expensive

d. toothpaste produced in the Philippines has a higher profit margin

than internationally produced toothpaste.

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Some Inconvenient Truths

The human predicament—climate disruption, loss of biodiversity and ecosystem services, toxification of the planet, the potential impacts of nuclear war, and social and economic inequities that impede solutions to escalating environmental problems—has been amply described. Although the steps needed to solve the predicament are clear, few have been taken—even as the situation steadily declines. The trend in greenhouse gas emissions has continued rapidly upward. The extermination of biodiversity and loss of natural services has proceeded unabated. The number of hungry people has hit an all-time high, which means that so has the number of immune-compromised individuals. That, combined with continued rapid population growth, increases the probabilities of vast epidemics. In Asia, melting of the Himalayan water tower and rising temperatures threaten the food supply of 1.6 billion people. There also have been increasing signs of great toxic peril for humanity and its life-support systems, with a growing threat from the release of hormone- disrupting chemicals that could even be shifting the human sex ratio and reducing sperm counts.

Despite the clear warnings about the predicament almost two decades ago from the scientific community precious little has been done. That's why a group of social and natural scientists and scholars in the humanities is starting the Millennium Assessment of Human Behavior (MAHB). The admittedly ambitious aim is to change human behavior to avoid a collapse of global civilization. Whereas climate change is on the political agenda, most of the other issues are not, and public understanding of what drives environmental deterioration or, indeed, of natural phenomena in general is minimal. Few non-scientists are familiar with the basic idea that environmental damage is a product of population size, per capita consumption, and the sorts of technologies and social and economic systems that supply the consumption. A vast "culture gap" has developed over the past century or so between what our society knows and what each individual knows—a gap that has proven especially troubling when elected officials and other leaders have almost no knowledge of science.

That's one reason why the devastating environmental consequences of an ever- expanding human population have been largely ignored. Governments in many struggling poor countries fail to support family planning programs adequately, whereas those in the rich countries of Europe are irrationally encouraging higher fertility. Few recognize that adding a billion people to the population in the future will cause more damage to humanity's critical life-support systems than did the most recent increment of a billion, as ever more scarce and remote resources must be tapped to support the newcomers.

Overconsumption by the rich is central to the deterioration of human life-support systems, but is ignored because most business economists, corporate executives, and politicians view it as something positive. To lead decent lives, at least two billion people are in dire need of more consumption, but extending American consumption patterns to even today's 6.8 billion people is not only unsustainable but likely a biophysical impossibility. It would, sadly, take many decades for humane actions to produce significant changes in today's population

trajectory. Yet, we know that consumption patterns can change. Following a path to reduce greenhouse gas emissions fast and deal with the rest of the

predicament would take vast political courage. Despite the current demand for research in the area of natural science, the need now is not for more natural science but rather for better understanding of human behaviors and how they can be altered to direct Homo sapiens onto a course toward a sustainable society, and to muster that courage before it's too late. Indeed, the academic focus for solving the predicament needs to shift dramatically to the social sciences and the humanities. Understanding such things as how social norms are generated and how individual actions get translated into group behavior are central to organizing a successful effort.

It is human behavior, toward one another and toward the planet that sustains us all, that requires rapid modification. The MAHB hopes to provide a basic mechanism to achieve this by exposing society to the full range of "inconvenient truths" regarding population–environment–resource–ethics–power issues, sponsoring a broad global discussion involving the greatest possible diversity of people, and trying to close crucial parts of the culture gap.

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Questions

15. Which'of'the'following'best'summarises'the'main'issue'the'writer' raises'in the'first'paragraph?

a. The ways of solving the human predicament are clear.

b. The situation regarding the human predicament is getting worse.

c. If no action is taken regarding the human predicament, there could

be a nuclear war.

d. Not enough has been done to solve the human predicament.

16. Which'of'the'following'statements'is'false?

a. The food supply to 1.6 billion people in Asia is under threat.

b. HormoneHdisrupting chemicals are reducing sperm counts.

c. The risk of vast epidemics is triggered in part by rapid population

growth.

d. Greenhouse gas emissions are increasing at a rapid rate.

17. According'to'the'writer,'which'of'the'following'statements'is'true?

a. Politicians have very little knowledge of science.

b. The general public is well informed on matters of science.

c. Politics has been affected by climate change.

d. Only scientists are familiar with the relationship between!

18. One'reason'why'the'environmental'consequences'caused'by'an' increasing'human'population'have'been'ignored'is'because....

a. poor countries have not supported family planning programs.

b. rich countries have acted irrationally.

c. there is limited public knowledge and awareness of the issue.

d. there are major cultural differences between societies.

19. Which'of'the'following'best'matches'the'writer's'view'regarding'the' issue'of'sustainable'consumption?

a. Nearly 30% of the global population consume too much.

b. It is not possible for the world population to follow American consumptions patterns.

c. Politicians!are!fighting!against!the!overconsumption!by!the!rich.

d. Only!business economists, corporate executives and politicians lead rich and decent lives.

20. Which'one'of'the'following'is'closest'in'meaning'to'humane?

a. HomoHsapiens

b. Humanitarian

c. Social

d. Global

21. Regarding'ways'to'change'consumption'patterns,'which'of'the' following'is'NOT'mentioned?

a. A need to change human behaviours

b. Continued research in natural science

c. A better understanding of human behaviours

d. Further studies in the areas of humanities

22. Which'of'the'following'is'closest'in'meaning'to'the'expression' inconvenient'truths?

a. Facts about our way of life modern society would like to ignore

b. Problems such as population, the environment and ethics

c. The current ignorance modern society has towards global problems!

d. Aspects of our way of life modern society would like to change

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Answers

1. B
2. A
3. D
4. C
5. D
6. D
7. B
8. Parents/guardian
9. as soon as the decision made for transfusion
10. RCH procedure manual
11. Group of blood disorders
12. 1 in every 500
13. Supervision of consultant neonatology
14. Blood, Blood products
15. Newborns
16. Circulating system, blockage in capillaries

17. Parents/Guardian

18. Sedation and pain relief

19. Blood samples

20. RCH Procedure.

Part B

1.B

2.B

3 B

4 A

5 B

6 C

PART C

7. b

8. a

9. c

10. c

11. c

12. d

13. d

14.b

15. d

16. b

17. a

18. c

19. b

20. b

21. b

22.a

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